



Division of Child Protection Casework Practice Requirements Manual

December 2013
(Fifth edition)

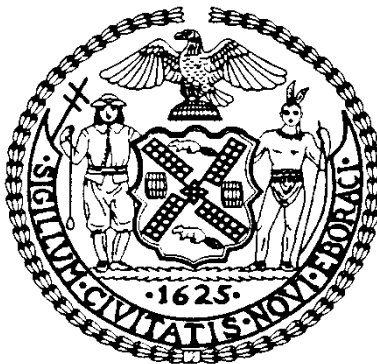


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Child Safety Alerts:

Link to review: [ACS Child Safety Alerts](#)

INTRODUCTION/OVERVIEW

Link: [Mission of Child Protective Services](#)

The mission of the Administration for Children's Services (ACS) Division of Child Protection (DCP) is to protect all children who are being abused or neglected in the City of New York, and to ensure that the appropriate services are provided to protect children and strengthen families.

THE DIVISION OF CHILD PROTECTION

The Division of Child Protection encompasses Child Protective Services and Placement Services. Child Protective Services includes the Child Protective Borough Offices, Family Preservation and Family Services Unit, Emergency Children's Services (ECS) and the Office of Special Investigations (OSI). Placement Services includes the Office of Placement Administration (OPA), Pre-placement and the Placement Stability Unit. The investigatory side of DCP is largely based out of the borough offices. These offices are made up of units of child protective specialist who investigate reported cases of abuse and neglect. DCP also administers case management services through its Family Services Unit (FSU) and Family Preservation Program (FPP). FSU is involved when a New York State Family Court Judge orders court ordered supervision for a family. The FSU worker monitors and assists the family with services while the family is under supervision. FPP is an intensive case management program designed to assist families who present an elevated risk, but who may be able to achieve safety and stability through immediate support.

DCP also links families to Preventive Services in their communities to prevent children from entering foster care. These services are designed so that children can remain safely in their homes. Services offered may include: counseling; parenting skills training; and substance abuse treatment.

Division of Child Protection Casework Practice Requirements

The DCP Casework Practice Requirements are uniform practice standards that clarify the elements of casework practice for child abuse/neglect investigations, assessment of safety and risk and service planning. It incorporates elements of various ACS procedures/policies and New York State regulations pertaining to child abuse investigations. The Practice Requirements do not represent the comprehensive collection of casework practices and procedures but does set the standard for child welfare practice and the expected tasks on child welfare investigations.

In partnership with your management team, the DCP Casework Practice Requirements will support your ability to respond to allegations of child abuse and/or neglect. It is designed to function as a living document, and to evolve alongside the changing needs of New York City's child welfare population. Accordingly, the practice requirements will automatically serve as a vehicle for implementing child welfare practices while simultaneously incorporating current policy and procedures, and relevant federal requirements.

The DCP Casework Practice Requirements are divided into five sections that represent the life of a child welfare investigation,

1. [Case Initiation](#)
2. [Interview and Assessment](#)
3. [Supervision](#)
4. [Services](#)
5. [Quality Assurance](#)

Each are linked to their corresponding folders in Docushare

Each section offers guidance on completing the necessary tasks to initiate, assess and complete a child welfare investigation. After each section is a listing of the supporting policies, procedures and forms required to meet the tasks.

The final section of the practice requirements is the Child/Parent template which is used to structure the documentation of child, parent/caretaker and household member interviews and assessments of the open investigation.

This document should be used as a tool that aids in the completion of case investigation, referral for services or case transfer to other program areas.

**The Key Commitments
Of
The Division of Child Protection:**

- 1) No child we come into contact with will be left to struggle alone with abuse and neglect.
- 2) No family who needs and wants help to keep their children safe will be left without the help they need.
- 3) Every child we come into contact with will get the help (s) he needs to be healthy and achieve his/her full educational and developmental potential.
- 4) Every team member in the division can expect guidance, respect and emotional support to achieve our goals. Every child, family, community member and foster parent we come into contact with will be treated with the same concern and respect.

Case Initiation

A CPS investigation must be initiated within 24 hours of the receipt of the SCR report. By initiating investigation and assessment activities, CPS will be able to gather sufficient information to assess whether or not the child (ren) are in immediate or impending danger of serious harm and if a safety intervention is needed immediately to provide for the safety of all children.

CASE INITIATION

CPS is responsible not only for investigating every allegation that is reported, but also for conducting a comprehensive assessment of the immediate safety and risk of future harm to each child in the family. This information gathered during the investigation about a child's safety is essential for CPS to have so they can respond as needed by case circumstances to protect the child (ren) and determine a plan of action for their investigation.

Information needed to initiate a case, can be obtained by reviewing prior case history and conducting a significant face-to-face or telephone contact with:

- The source of the report, who may provide information about whether or not safety factors exist and if the child is in immediate or impending danger of serious harm. The source may provide additional information that is not part of the SCR report.
- Another person who can provide information about whether or not safety factors exist and if the child may be in immediate or impending danger of serious harm
- Child and/or family. The contact with the child/family must be a face to face contact. CPS should consider the impact on the investigation of calling the home prior to visiting, as a telephone contact prior to visiting may compromise the integrity of the investigation.
- In- office clearances (ATS clearance, HHS Connect, criminal background checks with the assistance of the Investigative Consultant, Domestic Incident Reports (DIR's) etc.).

Contacting a source or subject of the report who does not offer enough information about whether the child is in immediate or impending danger does not satisfy the requirements for the initiation of the investigation. If a telephone contact that provides sufficient information about the safety of the child (ren) cannot occur, a home visit must ensue. Face to face contact must be made with the children, source, subject or collateral that can provide information concerning the safety of the child.

All efforts to initiate the investigation must be documented.

A face to face must take place within 24 hours for high priority cases and immediately for IRT cases. For non- high priority cases, case initiation must take place within 48 hours of the receipt of the report. The content of the report and prior history must be reviewed and evaluated to determine how soon the child/family must be seen. The seriousness of the allegations, the specificity of the allegations, the degree of vulnerability of the child due to age or other factors, any past history of abuse or neglect all must be considered when deciding how soon, with whom, and in what way the investigation must be initiated.

CPS must develop and document hypotheses to frame and limit the scope of the investigation. The hypothesis will assist CPS by attributing reasons to events and then guide CPS in looking for evidence to support or negate the hypothesis developed. Hypotheses that are aligned with child safety can support CPS response time and decrease the likelihood of inaccurate assessment of threat of harm, child vulnerability and parent protective capacity. CPS must also document their initial hypotheses in Connections and capture its changes as it evolves throughout the life of the case.

It is important to remember to Synthesize Information:

To synthesize information, CPS must use information obtained from prior family history, the source, parent/caretaker, child and collaterals. CPS must also connect the dots by using information obtained while focusing on:

- the previous and current safety concerns, needs, challenges and strengths of the parent/caretaker and the child's needs vulnerabilities and ability to self-protect
- Threat of harm to the child
- Protective Capacity of the parent/caretaker
- Level of intervention (safety or risk reduction)

CPS must discuss their assessment of the following:

Threat of Harm- situation of behaviors that impact a child's life, health and well-being. Threat of harm may range from immediate danger, impending danger, high risk, moderate risk or low risk.

Child's Vulnerability to being harmed- Factors such as age, developmental delays or other special needs increase child vulnerability.

Child's vulnerability to being rejected- Factors such as child coming out as lesbian, gay, bisexual or transgender to their parent(s)/ caretaker(s) may place the child at risk of being rejected from their homes or at risk of verbal/ physical abuse.

Parent Protective Capacity- understanding, ability and willingness to meet the child's safety and other childcare needs.

Level of Intervention- Safety Decision and Safety Planning

Safety and risk reduction plans- should specify the individual actions (individual objectives) and family actions (family objectives) that will keep the children safe.

Level of safety intervention or risk reduction services needed- consider and discuss the Threat of Harm, Child Vulnerability and Parent Protective Capacity. When safety interventions are required, they must be tested for:

- Immediacy- Must be immediate
- Specificity- Must be specific to the threat of harm
- Effectiveness- Must be effective in reducing the threat of harm
- Sustainability- Must be sustainable until the threat of harm is reduced

Required task:	Assess/describe/document:
1. Review intake and all case material. Links to Review: • High Priority Codes/Referral to DA/IRT • Child Protective Protocols • Child Safety Alert 13: IRT Process	• Whether the case is high priority • Whether case requires District Attorney (DA) referral • Whether case fits the criteria for instant response, and if so, determine the IRT type • Whether case requires any specific protocol(s): Domestic Violence, Under One Year, Sex Abuse • Consider if there is a time factor for contacting the Source (i.e., teacher and school will be closing soon) • Is there an alternate number for the source?

• HHS Connect Policy	• Is there additional information in the Miscellaneous Information section of the Intake Report? • Review all available clearances (ATS, HHS connect, DIR, etc.) • For cases forwarded to the borough office from ECS, ECS notes must be reviewed.
Required task: 2. Contact source of report immediately to obtain further clarification and information about the allegations, additional safety concerns and other persons who can provide information as to whether safety factors are present that may place the child in immediate or impending danger of harm. If the source is not immediately available, continue to contact the source until reached. Links to Review • Language/Sign Language Policies 3. Obtain and document all accounts (include dates) and obtain all information essential to the investigation. If the source is a mandated reporter, all otherwise confidential information such as diagnoses, treatment, history, etc. that are essential to the investigation must be given by the source to CPS upon request. Links to Review • 24/48hr CNNX documentation guidelines	Assess/describe/document: • Inquire as to the primary language of the family • Whether the source or other persons contacted can provide relevant information concerning the presence of safety factors • Whether or not any child in the household is in immediate or impending danger • Ask what prompted the reporter to call the SCR • Ask if the reporter has direct knowledge, or was informed by another source • Get identity and contact information for the direct source • Ask how the disclosure came about, if applicable • Ask if an injury was observed, and if so, on what date • Clarification of the allegations and/or additional information concerning the report and other possible indicators of abuse or maltreatment • Source's relationship to child/family and how long the source has known the child/family • If and when the source informed the parent/subject of this report • Ask if the parent/subject gave an explanation, and if so, what was it • How the source's account was obtained (telephone or in person) • Has the source of the report provided services in the past or currently
Required task: 4. Review prior SCR reports and service history to gather information that may assist with assessing child safety, worker safety, reveal risk elements and to prepare for home visit observations and interviews:	Assess/describe/document: • If current subject is the same as in prior reports • The number of prior indicated or unfounded reports

• It is critical to assess the new allegations in the context of all previous reports/allegations. • Review all cross-referenced reports, including Individual Report Involvement, identified by the SCR (cases that are previously indicated, unfounded reports on, or after 2/12/96, or are under investigation involving at least one of the persons named in the current report), and Out of County Reports. Links to Review • Child Safety Alert# 32 Responding to Heightened Safety Concerns in Preventive Services Cases • Obtain and review other relevant case records: closed files; clearances from all sources (CONNECTIONS, HHS Connect, Automated Case Reference System Plus (ACRS+), and WMS/CCRS) ATS, CTS and LTS informational systems. Obtain information from staff who has previously worked with the family Link to review: • HHS Connect Policy • Print a notice of existence from CONNECTIONS for each subject listed and other persons named in the CPS report. • Review of unfounded investigations Link to review: • CPS Review of Unfounded Reports Policy • Case Record Request Form	• Severity and type of current allegations compared to those in prior reports • Child's present condition, family functioning and household conditions as compared to prior reports • Results of previous safety interventions and services whether they provided for safety and appeared to address the factors that created risk • Written or verbal findings of staff who previously worked with the family. • Information about worker safety while visiting the home (vicious dogs, weapons in the home, etc.) • What do the pattern, frequency, and chronicity of past reports tell us? • Consider source of prior reports (i.e., Are there multiple reports with similar allegations called in by different sources?). • Unfounded reports are best used to assist CPS in identifying behavioral trends and patterns that may indicate abuse or maltreatment. They must never be used as the sole basis for indicating the current SCR report. • For all adults a notice of existence was printed.
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Required task:	Assess/describe/document:
5. Review family composition in and out of the home including significant others (as noted in the report as well as previous reports and case records). - Note where there is missing information that you will need to obtain during your contact with the report source and home visit. - It is especially important to obtain basic information about the parent/caretaker for each child.	- Name, address or last known address, date of birth, social security number - Physical descriptions of household members - Language spoken - Relationship to family - Marital/Partnership status - Culture/ethnicity - Resident status - Religion - Source of income - Citizenship/immigration status
Required task:	Assess/describe/document:
6. Conduct supervisory/CPS pre-investigation conference to review: current report, possible presence of safety factors/immediate or impending danger; prior history of abuse/neglect; potential worker safety concerns; information to gather from the report source; develop preliminary response/development of hypotheses and provide guidance and directives regarding required case practice. Links to review: Child Safety Alert 21: Going Out in Pairs When Conducting CPS Investigations ACS Worker Safety Video Ensuring Worker Safety	- Findings regarding previous SCR reports, service plans and/or children in placement - Whether Initial Child Safety conference, police intervention, co-CPS assistance, Family Court Division of Legal Services (FCLS) consultation and/or interpreter is necessary - Documentation of case hypotheses discussed during the supervisory pre-investigation conference

Course of Action:

- ✓ Determine the need for an immediate home visit — if it seems that the children may be in immediate danger, a home visit must be conducted immediately or within 24 hours as warranted by case circumstances and the appropriate action must be taken to protect the child. For all other cases, the home visit must take place within 48 hours.

- ✓ Follow protocols for special types of cases, i.e., High Priority, IRT, Domestic Violence, DA Referral and Under One Year, Sex Abuse.
- ✓ Arrange for any of the following, if applicable: Initial Child Safety Conference, Police intervention, Co-CPS assistance, FCLS consultation, Interpreter, clearance by the Investigative Consultants

CONNECTIONS Documentation Requirement:

Link to review:

- [Child Safety Alert #5 Case Recording and Documentation Guidelines.](#)

A CPS investigation must be initiated within 24 hours of the receipt of the report by making contact with the family, source of report, or other knowledgeable adult. The purpose of the 24- hour contact is to ensure the children are safe. For high- priority cases the 24- hour contact must be made by face to face visit to call subject children. For non- high priority cases the 24 hour contact may be initiated with the source, family or other significant party, who can attest to the safety of the children. For non- high priority cases where the 24- hour contact was made via phone, the CPS staff must within 48 hours initiate face to face interviews with all children, the subject(s), parents/caretakers and other household members and visit the home to make an assessment. Every report requires a 24/28 hour contact with subsequent documentation, even if investigations are being consolidated. The 24/48 hour contact information must be documented by the time of the Safety Assessment is submitted.

To properly document a significant 24 or 48 hour contact:

Link to review:

- [24/48hr CNNX Documentation Guidelines](#)

1. Select the Progress Notes tab on the CPRS screen, click on “New” and choose the event date & time.
2. Choose the appropriate **Type** (casework contact or collateral contact)
3. Choose appropriate **Method of Contact** (face to face, phone) and **Location of Contact**. If face to face contact is made, select check box if it was unannounced. If CPS is unable to make contact, either by phone or face-to-face within 24 hours, then the **attempted contact** must be selected and documented with an explanation and course of action. This similarly applies to the 48 hour visit.
4. Checkmark the **Participant (FP)** field of any family members is applicable. For all face- to face contacts the **Participant** field is required for all subject children for credit to be given. **If the contact was not successful, in that it was attempted, then there is no need to checkmark the individuals as participant or focus.**
5. For the non-high priority 24- hour contact by phone, the subject children must have a checkmark by their name on the **Focus** field for credit to be given.
6. Select other participants (all that apply) ex. Reporter/Source, School Staff, etc.
7. Select Author of Notes (yourself or “other”, if you do not have a role in the case)
8. In the Purpose Field you must select 24 or 48 hour, whichever applicable. You may select other choices if applicable but 24 or 48 hour must be one in order for the 24/48 hour credit to be given.
9. The following **locations** may be selected in progress notices for 24/48 hour face-to-face visits: **adoptive, home, case address, child advocacy center, court, daycare, foster home**

, hospital/health facility, LDSS office/ Borough Office, OMH facility, OPWDD facility parents' home, precinct/law enforcement, relatives home, school, service provider/contract agency, sheltered- domestic violence, and shelter- homeless. To the degree possible and when appropriate the case address is the preferred location for initial visits.

Note 1: Important Reminder on Documenting the Source Information

In order to safeguard the confidentiality of the reporter/source, only enter identifying information related to the reporter/source (name, organization/agency, telephone number, etc.) in Investigation Stage Progress Notes using the "Other Participant" data field with the value of "reporter/source". Do not include any identifying information regarding the reporter/source, including the institution, organization, etc. with which the person is affiliated, in any investigative actions. No identifying information related to the reporter/source of a CPS report may be included in narrative portion of the Family Services Intake (FSI) or Progress Note in the Family Services Stage (FSS). This will allow only those with proper security to view details of reporter/source information. In any other section the reporter/source should only be referred to as "the reporter" or "the source."

NOTE 2: Merges Link to review: [CNNX Tip Sheets on all Merges](#)

A merge combines duplicate records or persons into a single record or person. If the worker finds that they have duplicate records or persons within a case, a merge should take place. Consolidation of multiple investigations should take place when appropriate, except in cases concerning fatalities, reports involving agency staff, and Domestic Violence cases.

NOTE 3: Safety Status and Intervention

CPS needs to plan and conduct their investigation and assessment activities so that they can make a decision about the child's safety status and the need for intervention. Safety assessment is an ongoing activity throughout the case and should evolve as the case evolves.

NOTE 4: Diligent Searches

CPS should take all necessary steps to interview and assess a family and child safety based on the information reported and prior history known. CPS should utilize all resources to locate the family and/or child, which includes but is not limited to: assistance from the Investigative Consultants, visits to alternate addresses available, searches in the Department of Education Automated Tracking System (ATS) and HHS Connect.

Safety Factors

Link to review: [Expanded Safety Factors Link](#)

A Safety Decision is the CPS' conclusion of whether the child is in danger of serious harm and what is protecting the child, either through mitigating factors and family strengths, or by the interventions put in place by the CPS to control safety threats. All of the information gathered during the Safety Assessment and in the Safety Assessment Protocol is used by the CPS to support this decision. Keep these decision options in mind as you plan your investigation and assessment.

1. Based on your present assessment and review of prior history of abuse and maltreatment, the Parent(s)/caretaker(s) is unable or unwilling to protect the child (ren).

2. Parent (s)/caretaker(s) currently use alcohol to the extent that it negatively impacts his/her ability to supervise, protect and/or care for the child (ren).
3. Parent(s)/caretaker(s) currently uses illicit drugs or misuses prescription medication to the extent that it negatively impacts his/her ability to supervise, protect and/or care for the child (ren).
4. Child (ren) has experienced or is likely to experience physical or psychological harm as a result of domestic violence in the household.
5. Parent(s)/caretaker(s)' apparent or diagnosed medical or mental health status or developmental disability negatively impacts his/her ability to supervise, protect, and/or care for the child (ren).
6. Parent(s)/caretaker(s) has a recent history of violence and/or is currently violent and out of control.
7. Parent(s)/caretaker(s) is unable and/or unwilling to meet the child (ren)'s needs for food, clothing, shelter, medical or mental health care and/or control child's behavior.
8. Parent(s)/caretaker(s) is unable and/or unwilling to provide adequate supervision of the child (ren).
9. Child (ren) has experienced serious and/or repeated physical harm or inquiry and/or the Parent(s)/caretaker(s) has made plausible threat of serious harm or inquiry to the child (ren).
10. Parent(s)/caretaker(s) views, describes or acts toward the child (ren) in predominately negative terms and/or has extremely unrealistic expectations of the child (ren).
11. Child (ren)'s current whereabouts cannot be ascertained and/or there is reason to believe that the family is about to flee or refuses to access to the child (ren).
12. Child (ren) has been or is suspected of being sexually abused or exploited and the Parent(s)/caretaker(s) is unable or unwilling to provide adequate protection of the child (ren).
13. The physical condition of the home is hazardous to the safety of the children.
14. Child (ren) expresses or exhibits fear of being in the home due to current behaviors of Parent(s)/caretaker's or other persons living in, or frequenting the household.
15. Child (ren) has a positive toxicology for drugs and/or alcohol.
16. Child (ren) has significant vulnerability, is developmentally delayed, or medically fragile (e.g. on apnea monitor) and the Parent(s)/Caretaker(s) is unable and or unwilling to provide adequate care and/or protection of the child (ren).

17. Weapon noted in CPS report or found in the home and Parent(s)/Caretaker(s) is unable to protect the child (ren) from potential harm.
18. Criminal activity in the home negatively impacts parents(s)/caretaker(s) ability to supervise, protect and/or care for the child (ren).
19. No safety factors present at this time.

Referral to the District Attorney: Cases in which there are one or more of the following injuries/conditions must be referred to the DA as a crime may have been committed:

- Fatality
- Fracture
- Subdural hematoma or internal injuries
- Widespread or serious bruises, lacerations or welts or a pattern of the above, or any attempt to cause serious harm
- Sexual abuse (including attempts)
- Burns, scalding
- Attempted drowning or asphyxiation
- Malnutrition or failure to thrive
- Failure to obtain medical treatment or improper administration of medical treatment
- Abandonment, including any incident where a child is seriously injured while left unattended

High Priority Criteria: These high priority factors are usually identified at intake. Cases with codes 1 and 13 will receive managerial review and guidance at Day 7, Day 30 and at determination (Day 55).

Code 1- DOA/Fatality

Code 2- Positive Toxicology

Code 3- Serious Injury [Mandated Reporter]

Code 4- Malnutrition/Failure to Thrive [Mandated Reporter]

Code 5- Sexual Abuse

Code 6- Domestic Violence

Code 7- Child under 7, caretaker abuses drugs/alcohol [Mandated Reporter]

Code 8- Child under 7, caretaker mentally ill/developmentally disabled [Mandated Reporter]

Code 9- Child under 7, unsupervised

Code 10- Reported child under 1 year old

Code 11- Child on Sleep Apnea Monitor [Mandated Reporter]

Code 12- Weapon noted in report

Code 13- Four or more reports

***If the case is later coded 1 or 13, CPS should immediately refer the case for review to the CPM**

Instant Response Team (IRT)

Link to review:

- [IRT Manual](#)

Purpose: To minimize trauma to children and expedite information gathering, sharing and services by working collaboratively with the NYPD and the District Attorney.

Types:

- **Response Team Type I:** Instant Response on recent fatality cases by ACS (Division of Child Protection Field Offices, Emergency Children's Services (ECS), and Office of Special Investigations (OSI)) and NYPD (Patrol, Detective Squad, and Night Watch).
- **Response Team Type II:** Instant Response on severe abuse cases of children under 11, felony sex abuse of children under 18, and all sex abuse of children under 13, by ACS (Division of Child Protection Field Offices, Emergency Children's Services, and Office of Special Investigations) and NYPD (Patrol, Special Victim Squad, and Night Watch).
- **Response Team Type III:** Instant Response on severe abuse and maltreatment cases of children 11-17 and sexual abuse of children 13-17 not covered by Type II, by ACS (Division of Child Protection Field Offices, Emergency Children's Services, and Office of Special Investigations) and NYPD (Patrol, Detective Squad, and Night Watch).

Law Enforcement Coordination and Assistance:

- **Coordination/Information Sharing:** ACS and law enforcement will coordinate on severe abuse and maltreatment cases that do not require an emergency Instant Response but a multidisciplinary team response (including Domestic Violence) and share information as needed.
- **Law Enforcement Assistance:** NYPD will provide assistance when necessary, on child removals, Family Court warrant and/or entry orders, and in cases involving weapons, drug selling, incidents of violence in the home or threats to CPS. NYPD will respond not only for the purpose of ensuring the safety of all concerned but also for the purpose of investigating possible criminal activity. When this assistance is needed, contact the IRTC. However, ACS staff should continue to contact "911" in all emergency situations.

Child Advocacy Center (CAC)

Purpose: To avoid multiple interviews and reduce trauma, interview child in a child friendly center, such as the CAC.

- Because Hospital Emergency Rooms are often ill equipped to handle sexual abuse exams, team members, whenever possible, should make maximum use of Child Advocacy Centers or Child Protection Centers. These centers have staff with child abuse expertise and are equipped to handle these situations. When an IRT Coordinator/Special Victim Services (SVS) Detective is notified that a child under 13 is a victim of a sexual assault or severe physical abuse and the child has not been removed to a hospital, a determination should be made if a CAC or hospital-based CPC is the more appropriate place for the child's sexual abuse examination or interview after considering the factors in the case

Note: All IRT Cases are DA cases.

SUBSEQUENT REPORTS AND ADDITIONAL INFORMATION ON SCR REPORTS

The casework practice requirements and documentation described in this manual must be completed for all subsequent reports. Consult with a supervisor as to the extent of the information that can be incorporated from the previous investigation. Additionally, review to see if the investigations can be consolidated. * See *Initiation of Investigation section for Required Task*

Additional Information Received from the SCR on Open Cases

At times, a reporter will provide the State Central Register (SCR) with additional information on cases already active with ACS. When this information does not rise to the level of a new abuse or neglect report, it is sent to ACS from the SCR as “Additional Information.” **“Additional Information” must be fully assessed to determine the safety and well-being of the children involved.** Given that these are families who may have children already in foster care or are receiving services from ACS, the “Additional Information” should be assessed by CPS to determine if any safety or risk issues are present. Review “Additional Information Received from the SCR on Open Cases: The Need for Close Attention and Assessment” (Child Safety Alert # 16: August 17, 2006).

Subsequent Reports versus “Additional Information” for Newborns Whose Siblings Are in Foster Care:

- If CPS receives information concerning the birth, or expected birth of a new child, CPS must conduct an assessment to determine whether it would be safe for the newborn to reside in the home.
- A case conference with the family and service providers during this time should address the upcoming birth, services needed, and the family and agency’s safety plan for the baby.
- The FCLS attorney handling the sibling case must be notified to work with the case planner, case manager and CPS to notify the court and take appropriate legal action.
- The SCR must be called as soon as the child is born. If there is reasonable cause to suspect abuse or neglect of the new child, the SCR will accept this call as a new report. If the SCR does not accept the call as a new report, the SCR will take the information about the birth of a new child with a sibling already in care as “Additional Information”. Refer to “Safety Planning for Newborns Whose Siblings Are in Foster Care” (Child Safety Alert #14: June 16, 2006) and to Memo: “Additional Information on SCR Reports & Subsequent Reports” (G. Taylor: February 7, 2006).
- If the case is open in ACS for the provision of foster care or preventive services, but not open in CPS, the “Additional Information” will be assigned to a CPS worker for assessment.
- If the case is currently under investigation, the information should be integrated into the ongoing assessment of the family.

Required task:	Assess/describe/document:
1. Upon receipt of the “Additional Information” by the Applications Unit from the SCR, the Applications Unit will conduct clearances to determine where in ACS the case is active. If the case is active in the field office, the assessment of the child (ren) will be done by the CPS unit assigned to the case. If the case is active in another case management area (i.e., OCM or DFCS), the Applications Unit will assign the CPS borough field office where the family is living as the “Primary” and send an “Alert” to the case manager. The case manager must immediately go into CNNX to assign the CPS worker a role in the case (caseworker) so that CPS can do an assessment of the child (ren) and document this in CNNX. Links to review: • Applications Procedures • Assessing Additional Information Report to Ensure the Safety of Children Policy • Child Safety Alert #31 Revised Additional Information Received from SCR on Open Cases – Need for Close Attention and Assessment	• The Applications Unit must follow up with the Office of Case Management to ensure that the assignment is made to the field office worker. Any delays in the assignment must be referred to the manager or Deputy Director for Administration for follow up. • Except for demographic information that is transmitted from the SCR as “Additional Information” such as address updates or name changes, all other information received from the SCR as “Additional Information” must be carefully reviewed and assessed by the assigned CPS. Demographic information received as “Additional Information” should be forwarded by the receiving Applications Unit to the CPS or case management unit in which the case is currently active to add to the case record.
Required task:	Assess/describe/document:
2. CPS must consult with his/her unit supervisor upon receipt of such information to determine if the “Additional Information” is related to the allegations contained in previous investigated reports concerning the	• Proceed with investigative and assessment activities as per previous chapters to gather information that will help determine whether interventions are necessary to ensure the safety and well-being of the children

family.	
Required task:	Assess/describe/document:
3. Contacts with the source/reporter of the “Additional Information,” if known must be made to clarify the basis of the source/reporter’s concern and determine whether the reporter has reason to suspect child abuse or neglect. Following the contact with the source/reporter of the information, the CPS must discuss the case with the supervisor to determine whether there is a basis to make a new SCR report.	• Document all information obtained from the source of the additional information report.
Required task:	Assess/describe/document:
4. If a new report is called in to and accepted by the SCR, a complete investigation of the allegations must be conducted.	• Document all actions. • Follow investigation protocol.
Required task:	Assess/describe/document:
5. If there is insufficient basis to make a report of child abuse or neglect to the SCR, the CPS should continue his/her assessment. At any point during the assessment, if any information is uncovered that leads to a suspicion that a child may be abused or neglected, a new report must be made to the SCR.	• Document all actions. • A new report must be made to the SCR if any information is uncovered that leads to suspicion that a child may be abused or neglected.
Required task:	Assess/describe/document:
6. CPS must make contact with the foster care or preventive service provider and the ACS case management unit to discuss the open case, the additional information received, and any information the worker may have concerning the family. The CPS must consult with his/her field unit supervisor to consider the information gathered and determine if any safety interventions are necessary. All casework activities must be documented in CNNX in the open stage associated with the “Additional Information.” Supervisory review and approval is	• Document all casework activities in CNNX in the open stage associated with the “Additional Information”. • Supervisors: Review and approve the assessment of “Additional Information” consistent with the same timeframes as a CPS report.

required during the assessment of “Additional Information” consistent with the same timeframes as a CPS report. The “Additional Information” should only be closed by a supervisor after a complete review and approval of the information gathered and the assessment.	
Required task:	Assess/describe/document:
7. If the information refers to the birth of a new child or newly discovered child whose siblings are already in foster care as a result of abuse and neglect, CPS may receive information about the new child as “Additional Information”. In such cases, it is critical that the CPS and the foster care agency case planner share and discuss their information with one another as part of the assessment process. The investigation/assessment should include a review of the facts and circumstances surrounding the family’s current service needs and their ability to care for the child, coupled with the family’s history including why the siblings came into care and what progress the family has made towards addressing those safety concerns. Refer to “Safety Planning for Newborns Whose Siblings Are in Foster Care” (Child Safety Alert #14: June 16, 2006) Links to review: • Safety Planning for Newborns or Newly Discovered Children Whose Siblings are in Foster Care • Child Safety Alert# 14: Safety Planning for Newborns or Newly Discovered Children	• If newborn/newly discovered child is determined to be at imminent risk of serious harm and there is insufficient time to obtain a court order, immediate action must be taken to ensure the child’s safety. • In all cases, there must be a legal consult with the FCLS attorney on the sibling’s case regarding the newborn/newly discovered child. CPS, with the input of the FCLS attorney and the foster care agency, will need to make a decision as to whether it is appropriate to seek a remand or court ordered supervision. • If the decision is to seek court ordered supervision, it must be clearly documented in the record why the older children have not yet been reunified, while it would be safe for a more dependent and fragile newborn to remain in the home. • If the decision is made for the baby to remain in the home, a Child Safety Conference must be held as soon as possible to ensure appropriate services are in place to meet the child and family’s needs through the provision of preventive services.

Interview and Assessment

The goal of the interview is to obtain statements from the child (ren), parent/caretaker, source and collaterals that will determine credibility of allegations and the presence of any additional forms of abuse and/or maltreatment. The assessment will determine the risk elements that are present in the caretakers' history and current behaviors, conditions and abilities, and to decide if the family needs services and/or judicial intervention to change behaviors or environments that create harm to the children.

HOME ASSESSMENT

CPS staff must visit the home to continue the safety assessment and conduct face-to-face interviews with all children, the subject(s), parents/caretakers and other household members. Visiting the home also allows CPS to observe the physical environment for adequate food, clothing and sleeping arrangements for each child. If the physical environment presents safety or risk concerns, (e.g. unsafe sleeping arrangements, no window guards, no food, etc.) CPS must document these findings and take immediate actions to remedy the situation. The Information gathered from the home assessment should be used to complete the safety assessment and child/parent template.

- **If the report is designated High Priority or Immediate Danger is Suspected**, CPS staff is required to visit the home to initiate face-to-face interviews with all children, subject(s), parents/caretakers and other household members **within 24 hours**. If the only child in the home is hospitalized, then the 24 hour contact may be made face-to-face at the hospital and a home assessment may be completed before the child returns to the home.
1. **If the report is *not* designated High Priority**, CPS must make a home visit **within 48 hours** of the receipt of the report to assess the home conditions and conduct face-to-face interviews with all children, subject(s), parents/caretakers and other household members.

Required task:	Assess/describe/document:
1. Visit the home and assess the home environment for the presence of safety factors, and if any of those factors pose immediate or impending danger to the children. Links to review: • Use of Field Visit Schedule Form • Field Visit Schedule • Notice of Home Visit	Whether any of these unsafe conditions exist in the home: • no heat/hot water • unsafe space heater • no gas/electricity • no window guards • non working smoke alarm • non working carbon monoxide detector/alarm • inadequate plumbing • inadequate supply of food • inadequate/or lack of sleeping arrangements (i.e. no bed or crib; infant co-sleeping with parent, items other than the secure fitted sheet in the crib) • unsanitary conditions and/or excessive vermin • exposed wires • broken taps runs scalding water • leaking gas • broken windows and door locks • exposed radiator/exposed pipes • holes in walls • history of evictions/homelessness • heavy traffic of adults in the home • broken refrigerator • peeling paint • crumbling ceilings • medication and/or toxic chemicals within easy
2. All rooms in the home must be examined for hazardous conditions.	
3. Determine sleeping arrangements for all household members. Ensure all children have their own bed and that all infants have their own crib free of obstruction and hazardous items. Links to review:	

• Safe Sleep Resources	reach of the child
Required task:	Assess/describe/document:
4. Speak to parent and child separately about home conditions.	• Whether parent is aware of the potential safety hazards to the child • Caretaker's motivation & efforts to address unsafe home conditions • Impact of home environment on children and how children view the home • How family and community resources (neighborhood supports, transportation, stores, medical facilities, school, day care and apartment building) or lack of resources affects home conditions.
Required task:	Assess/describe/document:
5. Make diligent efforts to gain entry to the home Make referral to FCLS to obtain an entry order or warrant if needed. The entry order application requires that there is "probable cause to believe that an abused or neglected child may be found on the premises". This application must be based upon three criteria: • An SCR report as well as any additional information that we have learned in the investigation; <i>and</i> • The fact that the CPS has been denied access to the home of the child or children in order to evaluate the home environment; <i>and</i> • The fact that the CPS has advised the parent or other person legally responsible for the child or other children that ACS may consider seeking an immediate court order to gain access to the home without further notice to the parent(s) or other persons legally responsible. The judge should specify in the order which action may be taken and by whom (i.e., what action(s) to be taken by ACS and what actions by NYPD). The court application will be made without the	• Obtain police assistance if child may be in immediate danger and CPS cannot gain entry into the home. • If immediate danger is not suspected, make at least two home visits at different times of the day before seeking legal consult. • If child is school aged, attempt to make contact at the school immediately. • Refer to FCLS to obtain an entry order or warrant if needed. Link to review: • Warrant and Entry Orders Information

parents having any right to come to court to be heard and to argue against the granting of the order. CPS' obtaining this order will not be automatic and the search warrant provisions of the New York Criminal Procedure Law will apply. In deciding whether to issue an order of entry, the judge must assess which actions are necessary in light of the child or children's safety, and order only such action(s) that are the least intrusive to the family. Further, the judge must consider all relevant information, including, but not limited to: • The nature and seriousness of the allegations made in the report • The age and vulnerability of the child or children • The potential harm to the child or children if a full investigation is not completed.	
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CONNECTIONS DOCUMENTATION REQUIREMENTS:

Complete Household Condition Section of the Child/Parent Template

Link to review:

- [Child Protective Interviewing Template](#)

Course of Action:

- Obtain police assistance immediately if child may be in immediate danger and staff cannot gain entry to the home.
- Make persistent attempts to gain cooperation from the family. Repeat visits, telephone calls or explanatory letters, within 24-48 hours from CPS may be instrumental in achieving the desired access and cooperation.
- Request to Family Court Legal Services (FCLS) immediately to obtain an entry order if staff cannot gain entry to the home within 72 hours of the initial attempt.
- Refer to the Safety Assessment guidelines when considering the immediacy and seriousness of the hazardous conditions in the home and whether immediate danger is present and if the children require protection.
- If the home conditions are hazardous, address these conditions accordingly.
- **For unsanitary household conditions, Children's Service staff should contact the ACS Day Program directly at 212/361-8775 or go to the ACS Intranet (Admin Services > Day Program) and download and print a Special Handling Form and follow the Day Program directions for heavy duty cleaning.**

Links to review:

- [Day Program Services Guide](#)
- [Day Program Forms](#)
- [DOAS User Guide](#)

If you have any questions or concerns, consult with Borough Leadership.

**Special e-Bulletin from
Executive Deputy Commissioner Jacqueline O.
McKnight
Child Welfare Programs**

December 19, 2014

**-- Child Welfare Programs Practice Alert: Environmental Health
& Safety Monitoring --**

It is the responsibility of all staff to continuously monitor the safety and well-being of children on their caseloads. As a reminder, safety and risk assessments must include an evaluation of the physical condition of the home, and must note the impact, if any, of the home's condition on the child(ren)'s health.^[i]

Safety assessments of the physical condition of the home must consider:

Accessibility, including:

- The family's ability to enter, exit, and move safely within the home; and
- The ability of emergency response personnel to access the home, if needed.

Environmental safety of the home, including:

- Fire safety, including the presence of working smoke detectors^[ii] and clear, operating fire escapes, as well as the use of electric heating devices, such as space heaters;
- Carbon monoxide detectors;^[iii]
- Window guards; and
- Security of toxic or potentially toxic materials in the home, including cleaning supplies, medication, etc.

Staff must continuously assess the impact of the home's condition on the child(ren), with particular consideration given to conditions that may exacerbate the child(ren)'s health conditions or which could increase the child(ren)'s vulnerability^[iv]. Examples of home conditions that can be hazardous to a child's safety include, but are not limited to:

- Homes with excessive clutter or evidence of hoarding, which may affect accessibility as well as environmental safety;
- The presence of mold or vermin, which may contribute to or worsen chronic health conditions, such as asthma; and

- The lack of electricity, which prevents the use of medical equipment i.e., nebulizers or sleep apnea machines.

With the onset of the holiday season, staff should also be sure to address decorative items, such as candles and lighting, which can be hazardous.

Staff must address related concerns with the parent/caretaker immediately and on an ongoing basis during casework contacts. Assessments must be documented in CONNECTIONS (or hard copy case records if ADVPO). Secure supervisory guidance accordingly and contact 311 to report housing or environmental concerns/violations, contact ACS for technical assistance, convene a Family Team Conference, or call the State Central Register for Child Abuse and Maltreatment as deemed necessary.

For technical support regarding hoarding, or other clinical issues impacting child safety and well-being, please contact Assistant Commissioner Andrea Goetz in the Office of Clinical Practice, Policy and Support, at 212-442-4132 or andrea.goetz@acs.nyc.gov.

For technical support regarding asthma or other chronic health conditions and vulnerability factors, please contact Dr. Ursulina Bencosme, ACS Medical Director, at 212-676-6814 or ursulina.bencosme@acs.nyc.gov.

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Please post or distribute to staff without computer access.***

INTERVIEWS & ASSESSMENTS OF CHILDREN

The child is the primary client. It is critical to look at all the information from the point of view of the child. The family has a crucial role in keeping the child safe.

The initial interview of all children named in a new or subsequent report must be conducted separately from the parent/alleged subject within 24 hours if immediate danger is suspected or the case is high priority, and within 48 hours for all other reports. Begin obtaining information on the safety factors that will enable CPS to make the safety decision and to take all appropriate actions to manage the children's safety. CPS will also use this information to document assessment, make safety decisions, and take actions that will be reflected in the 7 Day Safety Assessment. **Children who are too young to be interviewed must be observed awake.** As the investigation progresses, more comprehensive interviews/observations (including observations of the parent and child interaction) of the children should be conducted.

Required task:	Assess/describe/document:
1. Conduct separate interviews with each of the children to obtain their account/explanation of the allegations on the report to gather information about the presence of safety factors and to assess for evidence of any abuse or maltreatment. Ensure that all children have been accounted for, including children living outside the home, in other counties, states or countries (<i>refer to ACS Interstate/ Intercounty procedures</i>). Interview the children while at school or at a Child Advocacy Center whenever possible. If the interview with the children takes place in the home, explain to the parent that the children must each be interviewed privately and separately to conduct a thorough investigation as required by the law. Ask the parent for a private location in the home to interview each child separately. Consider if the parent may retaliate against the child who is perceived to have divulged too much information to CPS. Also determine if the child feels comfortable being interviewed in the home even in a separate room. Re-interview if necessary.	• Ensure that the interview is conducted in the language that is familiar and comfortable for the child. Use a cell phone for the Telephonic Interpreter or contact the Interpretation & Translation Services to obtain a face-to-face interpreter or American Sign Language (ASL) assistance if needed. Consult with your on-site Language Service Liaison. Link to review: • Language/Sign Language Interpretation Guidance • Note the time, date and place (CAC, school, hospital, home, etc.) of the interview (i.e., on 4/2/07 at 2pm the child was interviewed at the Queens CAC by CPS Jones and Detective Smith). • In documentation of the interview, CPS must describe how the child was engaged, whether the child was interviewed alone and the child affect regarding the interview (<i>Verbal and non-verbal communication: whether the child seems unusually anxious, fearful, silent, uneasy or at conflict with the parent/caretaker, alleged subject or other household members</i>) • Note who was present in the interview room

If a child is too young to be interviewed, observe the child when awake, and the interaction between the child and the parent/caretaker. See the observation/assessment guidelines entitled <i>How to “Interview” a Baby and How to “Interview” a Toddler</i>. Links to review: • How to Interview a Baby Palm Card • Talking with Toddlers • Anti-Discriminatory Policies • Indian Child Welfare Act (ICWA)	and their role • Note who actually conducted the interview • Note if anyone else observed the interview (i.e., from Observation room at CAC) • Determine if the child is able to distinguish between the truth and a lie. • Assess the child’s version of events surrounding the alleged maltreatment. • Ask the child if they ever told anybody before and document the response. • Note how the disclosure came about • Note disclosure questions and answers verbatim. • Note who, where, when and what occurred. • Whom the child identifies as the alleged perpetrator • Inconsistencies in the stories between the child and others • Note the clarifying questions you ask the child in order to address any inconsistencies and note the child’s response verbatim. • Whether child has been previously maltreated/abused • How the child is disciplined and by whom • Is the child is disciplined and/or treated differently from others. • How the child is supervised and cared for and by whom • Ask child who watches him/her when
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	parents go out and document the response • Whether child has been threatened with serious physical harm • Whether child is subject to violent or out-of-control behavior by parent/caretaker • Whether child is exposed to drug/alcohol abuse and how it affects child's safety, care and supervision • Whether the child is exposed to violent behavior by any adult in the household, including domestic violence
Required task:	Assess/describe/document:
2. Describe the child's physical condition. Describe any physical injuries on areas of the child's body that are normally unclothed. If allegations warrant a more comprehensive body check, refer to the Link to review: OCFS CPS Program Manual for guidelines on observing areas that are normally clothed. This type of physical examination must be done in the presence of a parent/caretaker. Take photos of injuries, if appropriate, using the agency issued camera. Link to review: • Medical Issues in CPS Investigations Core Phase II Curriculum	• Size, shape, type and location of injury • Bruises, welts • Fractures • Scars, rashes • Burns • Cuts, lacerations • Bite or choke marks • Head injury/unconscious • Untreated injuries • Failure to thrive • Self injury including repetitive scratching or hair pulling/patches of missing hair • General physical condition • Medical conditions; treated or untreated • Whether any photos were taken • Obtain permission from parent prior to taking photo of child
Required task:	Assess/describe/document:
3. Seeking Medical Treatment: When there is a need for medical treatment in both non-emergency and emergency situations, obtain medical consent from the parent. With the parent(s) seek	• The precise nature of the injury requiring medical attention and explanation of how the injury was obtained • Whether the caretakers are willing to provide consent for examination and/or treatment

immediate medical treatment in emergency cases involving: fractures, head injuries, serious infections, internal injuries, severe burns, bruises, lacerations, welts, bites, severe sexual abuse, failure to thrive, malnutrition, etc. Links to review • Child Safety Alert #29 Additional Safeguards in Cases Involving Infants and	• Doctor's findings concerning the most likely cause of the injury, the probable age of the injury • Whether the injury is consistent with the explanation provided by the parent/caretaker or alleged subject • Type of medical treatment that child received & follow-up
Required task:	Assess/describe/document:
4. Describe any medical care, medical concerns, medical history, medication and immunizations. Describe any medical conditions/ concerns prescribed or non-prescribed medication, immunization status, doctor's visit, name and contact information. Describe if developmental delays/special medical needs are present Describe how the child's developmental stage (including special needs) is impacting the family. Link to review: • Medical Information • Assessment and Safety Planning with Special Medical Needs Children Policy	Minimum information if child is being removed from their home: • Immunization record • Primary care physician's name and number • Is the child taking any regularly prescribed medication? Any medications taken by child , dosage, frequency and duration • Adaptive devices • Known allergies • Where does the child regularly receive health services? • When was the child's last doctor visit? • Date child last received medical care and reason • Name, address, telephone number of Medicaid or health insurance provider • Whether immunizations are up to date • Any medical conditions and duration • Involvements with other medical facilities. • How are the special needs of the child being addressed by the parent/caretaker and extended family
Required task:	Assess/describe/document:
Identify the type of medication or medical equipment used by the child. Link to review: • Assessment and Safety Planning with Special Medical Needs Children Policy • Gathering and Assessing Medical	• Type of medication taken (document frequency and dosage) by the child or medical equipment used (also document how long the child has been using the equipment and if it is working appropriately). • If the child is removed, ensure that all medications and equipment accompany the child

Information during the Child Protective Investigation Policy • Child Safety Alert #17: Gathering and Assessing Medical Information • Hospital Contacts • HIPPA forms in various languages	• Document names and descriptions of the medications and equipment which accompanied the child into placement. • Document the special medical service received by the child and parent/caretaker (visiting nurse, occupational therapy, etc.). • Consult with the Medical Consultant on all information received from medical providers or when assessing medication and treatments
Required task: 5. Assess whether child's basic needs are being met Link to review: • Safe Sleep Information • DOAS User Guide • Sex Abuse Protocol • Sexually Exploited Youth	Assess/describe/document: • Whether child has enough appropriate food (healthy options). • Whether sleeping arrangements are appropriate (consider bed-to-child ratio, age and gender of children sharing bed, the condition of the crib/bassinet when observed: the Day Program provides cribs for children 0-3 years old and toddler beds for children over 3 years old.) • Whether child has enough weather appropriate clothing • Whether child receives routine and necessary medical care • Whether child has adequate supervision • Consider cultural factors and immigration status in the assessment
Required task: 7. Describe the overall appearance/condition of the child.	Assess/describe/document: Indicate whether any factors apply, for example: • Size or weight does not seem age-appropriate • Unexplained injuries • Excessively dirty • Untreated cradle cap • Blood or unusual discharge on diaper • Pain or difficulty sitting • Untreated lice • Radar gaze • Bald spot on back of head • Diarrhea/dehydration

	• Cannot move neck/limb • Persistent temperature • Signs of Developmental delay • Use of medical equipment for routine functions (walking, sitting etc.)
Required task:	Assess/describe/document:
8. Gather information on any mental health needs, history and treatment. Refer the case to the mental health consultant for further clarification of behaviors observed and information regarding diagnosis and/or treatment received. Link to review: • Mental Health Principles	• Psychiatric or psychological conditions • Observed behaviors • Date of last evaluation and treatment • Name, address and telephone number of provider (s) • Diagnosis /prognosis • Any medication taken by the child, dosage and frequency
Required task:	Assess/describe/document:
9 If the child or youth identifies as lesbian, gay, bisexual, transgender, or questioning, assess if the child or youth has experienced a positive, neutral or negative reaction from their parent/caretaker, sibling, and extended families members if they have come out to them. *CPS will not ask the child about his/her sexual orientation, this pertains to youth who independently disclose to CPS their sexual orientation. If they do not disclose, despite CPS assessment that they may be LBGTQ, CPS does not discuss it. Link to review: • Know Your Rights LGBTQ Pamphlet • LGBTQ ACS Policies • Improving Services for LGBTQ Children, Youth & Families	• Document if the child has come out to their families • Ensure that this information will remain confidential to maintain the child's safety. • If possible, assess and document why the child has not come out to their parent/caretaker, families. • If they have come out ask permission to have further discussion with the parent/caretaker about their feelings toward this. • If child has identified as transgender, CPS is to ask and document the child's preferred pronoun and/or name.

Required task:	Assess/describe/document:
10. Identify any developmental disabilities that the child has and gather information to assess how it affects family functioning. <i>Obtain medical documentation from parent(s), if available. If not available, document the names of medical providers and obtain details about all disabilities.</i> Using the Height and Weight chart, the Signs of Developmental Delay chart, and the Early Intervention Red Flags for Children 0 – 3 chart below, to identify any developmental lags in each of the following: motor skills, feeding/eating, toilet training, and speech. If there is a lag, conduct further assessment using medical providers and collateral contacts. It is a federal and state mandate that any child under age 3 that is the subject of an indicated case must be referred for an Early Intervention assessment. Additionally, all children age 0 – 3 suspected to have a developmental delay or disability should also be referred to Early Intervention. To refer a child to the New York City Early Intervention Program, call 311. Link to review: • Process for Referring Children for Early Intervention Services • Welcome to the NYC Administration for Children's Services website for Educational Resources. • Gathering and Assessing Medical Information during the Child Protective Investigation Policy • Child Safety Alert #17:	• Child's behavior/condition compared to age-appropriate milestones/standards. Link to review: • Handbook for Healthy Development of Children and Youth • Parent's ability to meet the needs of any children with developmental disabilities • Document the Early Intervention referral in the case record and on service plans • For all reported children under one year old, Complete the Under One Year Protocol and include in the case record. Link to refer to: • Under One Protocol

Gathering and Assessing Medical Information • Hospital Contacts • HIPPA Form Pay particular attention when assessing infants. Ensure the infant is observed <u>awake</u>. Refer to the How to Interview a Baby Palm Card guidelines in this manual Link to review: • Special Medical Need Policies and Guidance • ACS Website for Special Medical Needs Children	
11. Obtain information from child about school and attendance to assess if educational neglect may be occurring and to gather information on child's needs. Ask whether the child likes school, and if not, why not? Link to review: DOE Resources (Policies/Chancellor's Regulations and Resources)	• Name and location of school • Whether the child attends school regularly and school performance • Document the child's feelings about school and peers

CONNECTIONS DOCUMENTATION REQUIRMENTS:

Use all of the information gathered, along with information from other contacts, to do the Safety Assessment. The 7-day Safety Assessment must be submitted for approval within 5 days. The Investigation Determination Safety Assessment must be submitted for approval within 7 days of the Investigation Conclusion. CPS should also use the information obtained to complete the Child/Parent template.

Course of Action:

- ✓ If the assessment of the child warrants an IRT response or sexual abuse is suspected, arrange

for an interview of the child at a Child Advocacy Center and complete the Sexual Abuse Protocol.

- ✓ Refer to the Safety Assessment guidelines when considering whether any of the safety factors you have identified so far, alone or in combination, place the child in immediate danger of serious harm.
- ✓ If the children are in immediate danger, identify and implement the appropriate safety interventions.
- ✓ If unable to interview children and/or interview the children alone consult with supervisor.
- ✓ Obtain police assistance if there are immediate concerns about the children's safety, and CPS can not gain entry to the home or locate the child and family.
 - ✓ **Link to Review** [Child Safety Alert# 27 Locating Victims of Abuse and Neglect](#)
- ✓ Request a family meeting, or an Initial Child Safety Conference to discuss safety and risk concerns.
- ✓ Consult with Investigative Consultants, or with Clinical Consultants concerning mental health, substance abuse, domestic violence and/or medical services as needed.

Best Practice Tips for Interviewing Children

- Interview children in a safe, comfortable and neutral environment. Ensure you are at eye level with the child.
- Smile a lot, and affirm the child's contributions. (i.e., "that's very important information" or "you're very helpful")
- Never rush an interview with a child. Take as much time as the child needs to share the information comfortably.
- Use age appropriate language. Ask the child to explain what he/she means by any word they used that you do not understand.
- Ask the child if they understand the concept and importance of telling the truth.
- Gently explore discrepancies by asking for additional information. Never accuse a child of deliberately providing inconsistent information.
- If a child refuses to talk about an event, attempt to engage the child by discussing a topic of interest (i.e., a favorite cartoon). If a child still refuses to speak, comfort the child and reschedule the interview for an alternate location. Never force the child to talk if he/she is not ready.

- Always thank the child for taking the time to speak with you.
- There is no specific age at which a child is too young to be interviewed as every child develops at a different rate.
-
- When assessing infants or children with insufficient verbal skills, it is imperative that children be observed awake and in an appropriate context to evaluate any possibility of developmental delays.
- Make efforts to engage adolescents/teens in the home.

Tips on Concluding an Interview

from *NYS Children's Justice Task Force Forensic Interviewing Best Practices*

- Ask the child if there is anything else you need to know.
- Ask the child if he has any questions for the investigators. Questions that can be answered should be answered.
- Explain to the child what will happen next in the investigation. Refrain from making any promises that cannot be kept.
- No matter what the outcome of the interview, thank the child for participating.
- Give the child a card with a name and phone number of an appropriate contact person.
- In the event of a disclosure, address the potential “fall out” from the disclosure. This may be dealt with by saying, “Some children have an easy time after telling, and some kids have a hard time. Some families are glad that the child talked, and others are not. Some of the kids are happy they told, and others wish they had not. If you have any mixed up feeling about telling, it is OK to call. I’ll be glad to talk to you.”
- Prior to leaving the interview room, the interviewer could engage in a neutral topic of conversation or activity with the child. The purpose of this transition is to allow the child to leave the interview with as little anxiety as possible.

Weight and Height of Children from 0 to 20 Years of Age Boys

AGE	WEIGHT IN POUNDS			HEIGHT IN INCHES		
	3 rd Percentile	50 th Percentile	97 th Percentile	3 rd Percentile	50 th Percentile	97 th Percentile
Birth	5.5	8	10	18	20	22
3 months	10	12.5	16.5	22.5	24	26
6 months	14	17.5	22	24.5	26.5	28.5
9 month	16.5	21	25	26.5	28.5	30.5
12 months	18	22.5	28	28	30	32
15 months	20	24.5	30	29	31	33.5
18 months	21	26	32	30	32.5	34.5
21 months	22	27	33	31	33.5	36
24 months	23	28	34	32	34	37
27 months	23.5	29	35.5	33	35	38
30 months	24.5	30	37	33.5	36	39
33 months	25	30.5	38	34	37	40
36 months	26	31.5	39.5	35	38	41
4 years old	29	35	46	37	42	43.5
5 years old	34	40	54	39.5	43	46
6 years old	36	46	62	41.5	45.5	49.5
7 years old	40	50	70	44	48	52
8 years old	44	56	81	46	52	55
9 years old	49	64	94	48	52.5	57
10 years old	51.5	70	98	50	54.5	59.5
11 years old	58	80	122	51.5	56.5	62
12 years old	64	88	138	53	59	64.5
13 years old	72	100	154	56	61.5	67.5
14 years old	81	110	168	58.5	64.5	70.5
15 years old	90	125	182	61.5	67	72.5
16 years old	100	134	194	62.5	68.5	73.5
17 years old	108	142	205	63	69	74
18 years old	114	148	213	63.5	69.5	74.5
19 years old	117	152	218	64	69.7	74.7
20 years old	118.5	155	221	64.5	69.8	75

If child's weight/height is in 3rd Percentile or lower; if child's weight/height is in 97th Percentile or higher — please consult with your supervisor and medical consultation team.

Source: National Center for Health Statistics (NCHS)

Weight and Height of Children from 0 to 20 Years of Age

Girls

AGE	WEIGHT IN POUNDS			HEIGHT IN INCHES		
	3 rd Percentile	50 th Percentile	97 th Percentile	3 rd Percentile	50 th Percentile	97 th Percentile
Birth	5.5	7.5	9.5	18	19.5	21.5
3 months	9	12	15	21.5	23.5	25
6 months	12.5	16	19.5	24	26	27.5
9 month	15	19	23	25.5	27.5	29.5
12 months	17	21	26	27	29	31
15 months	18.5	22.5	28	28	30.5	32.5
18 months	20	24	30	29	31.5	34
21 months	21	25.5	31.5	30	32.5	35
24 months	22	26.5	33	31	33.5	36
27 months	23	27.5	34.5	32	35	37.5
30 months	23.5	28.5	36	33	36	38.5
33 months	24	29.5	38	33.5	36.5	39.5
36 months	25	30.5	40	34	37	40
4 years old	28	35	46	36.5	40	43
5 years old	31	40	54	39	42.5	46
6 years old	35	45	63	41.5	45	49
7 years old	39	50	73	44	48	52
8 years old	42	57	84	46	50.5	55
9 years old	48	64	97	48	52.5	57
10 years old	53	72	110	49.5	54.5	59.5
11 years old	59	80	130	51.5	56.5	62
12 years old	66	92	144	54	59.5	65
13 years old	73	101	158	56.5	62	67
14 years old	80	109	170	58	63	68
15 years old	87	115	179	58.5	63.5	68.5
16 years old	92	118	185	59	64	68.7
17 years old	96	121	189	59.2	64.2	68.9
18 years old	97	123	192	59.3	64.3	69
19 years old	99	125	195	59.4	64.4	69.1
20 years old	100	128	196	59.5	64.5	69.2

If child's weight/height is in 3rd Percentile or lower; if child's weight/height is in 97th Percentile or higher — please consult with your supervisor and medical consultation team.

Source: National Center for Health Statistics (NCHS)

Signs of Developmental Delay

This list is not comprehensive. Any one concern may not indicate a delay. CPS should document all concerns observed, statements made by the parent/caretaker, medical practitioners and sources when assessing for developmental delay. CPS should also seek the assistance of the Medical Consultation team when making final assessment of information and statements obtained.

If a child is school age, or attends preschool or a child care center, CPS should ask the educational provider if developmental delays have been suspected or tested for by the school.

Age	Signs of Developmental Delay
Newborn-12 months	Exhibits signs of Failure to Thrive including: unusually small and thin, lethargic, apathetic to eye contact/attention, does not meet developmental milestones (below). At 3 months does not smile or coo At 5-6 months does not hold head or reach At 7-8 months does not roll over/sit At 9-10 months does not creep or grasp with thumb and second finger Unusually small/thin
1-2 years	Does not say words in native language Cannot drink from cup Unusually small/thin Cannot walk without help
2-3 years	Unusually quiet Cannot run/jump Poor speech in native language Does not feed self Unusually thin/small
3-4 years	Cannot respond to simple questions in native language No toilet training or toilet trained child is incontinent Depressed/withdrawn Unusually thin/small
4-6 years	Cannot respond to simple questions in native language Sits still most of the time Has not completed toilet training or toilet trained child is incontinent Unusually thin/small
6-12	Cannot respond to simple questions, read or write letters in native language Cannot write numbers Cannot drink from a glass Cannot eat with a spoon or fork

	Has not completed toilet training or toilet trained child is incontinent Unusually thin/small
12-18	Cannot respond to two-part questions in native language Cannot drink from a glass Cannot eat with a spoon or fork Has not completed toilet training or toilet trained child is incontinent Unusually thin/small

“How to Interview a Baby” Developed by The Babies Can’t Wait Advisory Committee 2005	What To Look For When Providing a Quality Assessment of an Infant
1. The infant must be observed <u>awake</u>, without any external distractions. It is never sufficient to observe a sleeping infant. 2. Developmental Milestones: ▪ Is the infant on target with his/her developmental milestones? 3. Physical Appearance: ▪ Physical condition of the baby: Skin, eyes, etc. ▪ Is the infant of average size for his/her age? ▪ Does the baby move all four limbs? ▪ How alert is the baby? ▪ Any obvious disabilities? ▪ How does the baby smell? ▪ Are his/her clothes clean? ▪ Is the sleeping area clean, safe and free of any obstruction? ▪ Does the baby have a crib? 4. Interaction Between Baby, Parent, and Others in the Home: ▪ Does the parent/others respond to the baby's interactions? ▪ How does the baby respond to the parent? ▪ Do the parent and the baby look at each other? ▪ Does the baby appear comforted when the parent picks him/her up? ▪ Does the parent/others make positive statements about the baby? ▪ Does the parent smile when speaking about the baby? ▪ Does the baby cry, smile, or laugh? ▪ How does the parent respond to the cries? ▪ Can the parent differentiate between	Early Intervention Red Flags For Children Age 0 - 3 1. Physical: • Child does not use both sides of body equally , or favors one side • Child is awkward and trips a lot • Child seems unable to hold onto things past 12 months • Child is does not walk confidently past 18 months 2. Speech: • By <u>7 months</u>, no signs of cooing or babbling • By <u>12 months</u>, doesn’t have a few single words • By <u>2 years</u>, Not verbalizing “dat” or pointing at what he/she wants • By <u>2 years</u>, doesn’t have any short sentences or word combinations • By <u>2 years</u>,, can say words, but doesn’t use them to talk to people 3. Adaptive: • Difficulty eating: Losing fluid when sucking, gagging, refusing certain textures • Difficulty sleeping (excessive sleep, lethargy, night terrors, rigidity, tenseness) • Extreme sensitivity to certain experiences, like bathing, dressing (sensory integration problems) 4. Cognitive: • By <u>6 months</u>, does not shake, bang, drop, and/or mouth objects • By <u>9 months</u>, does not turn when name is called or to locate a sound • By <u>12 months</u>, does not containerize or put 2 things together

the types of cries and recognize what each cry expresses? ▪ Does the baby react to toys and other objects? ▪ Is he/she socially responsive? ▪ Does the baby have a regular eating and sleeping schedule? All children age 0 – 3 who are the subject of an indicated child protective report <u>MUST</u> be referred to Early Intervention. To refer a child for Early Intervention, call: 1-800-577-BABY (2229) or 311	• <u>By 24 months</u>, does not use 2 word phrases • By <u>24-30 months</u>, no simple pretend play • By <u>30 months</u>, not be able to match shapes: circle, triangle, etc. • <u>By 36 months</u>, not able to feed him/herself 4. Social/Emotional: • Cannot be consoled • Doesn't make eye contact • Appears expressionless, or is inappropriately friendly • Injures self (scratching/biting,/headbanging)
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“Talking with Toddlers” Developed by The Babies Can’t Wait Advisory Committee 2006-2007	Developmental Red Flags for Toddlers
Toddlerhood is a formative stage during which the infant continues to develop cognitively and social-emotionally, as well as expands receptive and expressive language skills, and gross motor skills. Children do not proceed at the developmental pace, and should be assessed individually in reference to their particular circumstances. Impacts upon developmental pace can include: pre-mature birth, developmental delay, positive toxicology at birth and traumatic experience. Concerns about delays should be expressed to the appropriate professional. Toddler Communication Abilities Ages 1-2 Toddlers, ages 1-2 have limited, yet emerging, expressive and receptive language abilities. By age 2, a child should be speaking in short sentences. Toddlers should be assessed by utilizing developmental milestone achievement, physical appearance, and infant- parent/caretaker interaction. (Refer to How to “interview” a Baby Palm Card, 2006). General Linguistic Ability for Children ages 1-2: <i>Note: Since linguistic development is rapidly ongoing, the age 1-2 ability has been provided in comparison to the age 0 to 1 ability.</i> By age 1: • Can vocalize or babble spontaneously • Can recognize familiar objects • Can point to objects • Expresses needs though gestures and/or vocalization • May recognize pictures • Recognizes and expresses use of familiar objects through play • Can remember and retrieve out of sight	Note: In accordance with NYS law, all children, ages 0-3, who are the subject of an indicated child protective report, must be referred to the Early Intervention program. Additionally, any other child for whom a developmental delay is suspected should also be referred to the Early Intervention Program. To refer a child to the Early Intervention Program, call 311. By 1 Year: • Does not respond to Pat-a-cake, Peek- a- Boo or other baby games • Does not imitate sounds • Does not say 2-3 words • Is not pulling up to a standing position • Does not point • Does not make eye contact By 18 months: <i>Any of the 1 year criteria or</i> • Does not imitate words • Does not attempt to feed self with a spoon • Does not imitate speech • Does not move around and explore • Does not squat when picking up objects • Does not recognize a few body parts • Does not express his/her own intentions By 2 Years: <i>Any of the 1 year or 18 month criteria, or</i> • Does not name a few familiar objects • Does not use a few two- to three word phrases in any language

objects • Imitates words and gestures • Attempts to label objects • Smiles at “no” and repeats “no” By age 2: • Should be speaking in short sentences with word combinations • Can name familiar objects • Combines gestures and speech • Asks to have needs met • Recognizes pictures • Can verbalize use of familiar objects • Says “mine”, says own name, and begins to use pronouns:” you”, “me”, etc. • Repeats new words • Understands action words • May experience anxiety when separated from parent/caretaker for interview • Seeks adult approval By ages 2-3: By age 3, toddlers have greater expressive and receptive language ability. However, children with Limited English Speaking Ability (LESA) or who communicate solely in an alternate language may appear to lag behind the ability of primary English speaking children when interviewed in English. Again, all children do not proceed at the same developmental pace. All children should be assessed individually in reference to their particular circumstances. Generally Linguistic Ability for Children Ages 2 to 3: • Understanding of language is far superior to verbal expression ability. • Imitates other’s language (repeats new words or accents.) • May not answer open ended questions • Memory is evidenced through demonstration, rather than through verbalization • Answers in 1-3 word sentences • Can generally identify people with whom	• Does not notice animals, cars and other moving objects • Has not yet begun to play symbolically with objects such as cars, housekeeping toys, dolls • Engages in rocking or head banging for extensive periods of time • Does not walk up stairs • Does not have a 50 word vocabulary • Does not recognize clothing and other familiar objects By 3 years: <i>Any of the 1 year, 18 month or 2 year criteria or</i> • Does not appear to be aware of other people • Uses little or no speech • Does not use play to imitate adult activities • Avoids looking in mirrors and at pictures • Does not point to familiar objects • Does not follow simple directions • Engages in repetitive behavior for extended periods of time • Does not express needs verbally • Is not toilet trained, or attempting to toilet train *Note: Children of a particular age may not respond to the lower developmental tasks due to delay, or alternately, because they may have outgrown them. Only consider ability for prior age task if the child is not on target for current age tasks.
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he/she has had significant relationship-
with tendency to use nicknames

- Expect “I don’t know” or “because” or “why” responses
- Can provide his/her own gender
- Beginning ability to draw/scribble. Can draw a circle. May require demonstration. Object drawn may not be recognizable item before age 4.
- Lacks ability to sequence events or label time. Limited understanding of time concepts, and of words such as “before”, “after”, “yesterday”, “tomorrow”.
- Short attention span- varies from child to child.

Developing an Awareness of the Stage

- Many children of this age experience stranger anxiety. Do not mistake stranger anxiety for general fright or fear, or mistake a shy temperament for withdrawal. Alternately, consider whether a child appears to be inappropriately friendly.
- Letting a child play with toys allows a relaxed environment. Do not attempt to interpret the child’s play psychodynamically. Any concerns about play themes should be referred to an appropriate professional.
- Be prepared to sit on the floor and engage with the child in his/her play. Interact with the child at his/her developmental level.
- Do not take it personally if the child does not appear to cooperate with the interview.
- Young children who have an overly heightened state of arousal will often look away to regulate themselves. Do not attempt to maintain engagement or the child’s anxiety level will increase. Allow the child to pause. Wait until the child resumes the interaction.

Interview Suggestions • Ask/ identify the child's primary language, and maintain an awareness of any cultural differences and idioms • Do not interview when a child is hungry or tired • Use words that are 1-2 syllables and easy phrases • Use specific names instead of pronouns • Do not ask complex, multi- part questions- Aim for one idea per sentence • Only introduce one idea at a time	
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Tips on Interviewing Adolescents and Older Teens

*Marty Beyer, Ph.D.; Suggestions for Interviewing Teenagers
Strengths/Needs- Based Support for Children, Youth & Families*

Successful interviewing relies on the interviewer taking responsibility for reducing the teenager's anxiety and for building trust. Since adolescents tend to be egocentric, they doubt that other can really understand their unique experience. In addition, it is normal for teenagers to process questions differently than older individuals, feel defensive when identity- related issues are raised, and mistrust adults. Teenagers with attention problems and/or trauma- related reactivity are even more challenging to interview.

Interviews with young people are more likely to be successful if you:

- **Follow the adolescent's lead**
 - Encourage the young person to tell his/her own story without interruption. If you are patient, many of your questions will be answered without putting the teenager on the spot. Be an active listener, nod in agreement and make encouraging comments. Body language that is calm and conveys openness increases relaxation, which helps teenagers tell his/her story. Avoid being controlling.
- **Recognize the young person's strengths**
 - Talking about what he/she (and the family) does well usually gets the teenager more involved in the interview
- **Do not take the teenager's behavior personally**
 - Do not assume an adolescent is being uncooperative. He/she may not communicate easily with adults. He/she may be responding with fear to an unfamiliar and threatening situation. His/her initial lack of trust may be the result of past victimization.
- **Listen to the young person's interest**
 - Many youth in family and juvenile court are different in race, culture and class (and often gender) from the interviewer. Avoid putting your frame on the interview and instead learn about what the young person is an expert on: video games, social networking, sports, music, etc. You don't have to share this interest to be attentive, and it will make the young person less anxious to talk about what they know. Don't talk too much about yourself.

INTERVIEWS OF PARENTS/CARETAKERS, ALLEGED SUBJECTS AND HOUSEHOLD MEMBERS

A face-to-face interview with all subjects of the report, parents/caretakers, and all reported and non-reported household members must be conducted within 24 hours if immediate danger is suspected or case is high priority, and within 48 hours for all other reports. In every SCR report received, Child Protective Services is responsible not only for investigating every **allegation** that is reported, but also for conducting a comprehensive **assessment** of the immediate safety and risk of future harm to each child in the family. The best way for the CPS to begin the initial interview is by engaging the subject, parent, family member, or caretaker. The goal is to first gather the necessary information in order to assess whether a child is abused or neglected and second, to take the steps necessary to protect that child from further harm. **This can best be accomplished if the CPS opens up a conversation with the parent/caretaker in a way designed to elicit accurate information. This cannot be done by opening the conversation by listing the allegations in the report.**

Link to review:

- [Child Safety Alert #20 Initiating an Investigation: The First Interview with the Subject](#)
- [Use of telephone calls to children, subjects, Parents/Caretakers, Family and/or other Household Members](#)

Engaging Parents/Caretakers

During the course of an investigation, it is important to inform the subject about each of the allegations and provide the subject with an opportunity to respond. However, opening an investigation in this way will naturally lead to denials. It is not good practice to list specific allegations at the very outset of the conversations with the subject. The CPS should state that ACS has been notified of some concerns about the care of the family's children and wants to help the family keep its children safe. This introduction can be followed by general questions that narrow down to the more specific ones related to the allegations.

Before ACS makes a final determination, every subject should have the chance to hear each allegation and respond to it. Furthermore, in making a determination, the CPS must document each allegation separately, along with the finding for that allegation. **However, reading or paraphrasing the allegations to the subject or family at the outset of the investigation is not acceptable practice.**

Should the subject demand to know the specific allegations and refuse to proceed, the CPS may respond generally: "We've received a report that your children may not be going to school," or "We're looking to see if your children are receiving the medical attention they need," or "We are checking to see if your children have been injured." If families insist on more specific information, CPS should refer them to the contact information for the State Central Register to be found in the Notice of Existence that must be given to the subject at the initiation of an investigation. Parents or other subjects of a report have a legal right to request information (other than the name of the reporter) from the SCR.

In addition, because the law protects the confidentiality of SCR reporters and subjects, **CPS must never bring the intake report or a copy of this or any other part of the case record with them on field visits or disclose the source of the report.**

During the pre-conference at the initiation of the investigation, Child Protective Supervisors should discuss with CPS how best to begin each investigation based upon an assessment of the intake report, prior history and contact with the source. Part of this supervision should involve the development of a hypothesis and suggested questions for the worker to use to initiate the investigation. The initial interview should focus on gathering information on the presence of safety factors, on the family composition, on engaging the family and beginning the investigation process. More comprehensive interviews should be conducted throughout the investigation to gather sufficient information to make decisions about safety, risk of future harm, the credibility of the allegations and family needs and strengths.

Quality investigative work demands that a parent's explanation of a child's injuries, school absences, or other relevant behaviors not be accepted at face value. Instead, it is incumbent on the investigator to look for information from collateral interviews and records to establish whether the parent's explanations are credible. Though ACS protocol requires that parents' explanations be documented within the record, practice also demands that those explanations be questioned and verified, every time. In particular, discrepancies in stories among the families and household members of child abuse victims should always be identified and addressed. Failure to do this may lead to our missing critical evidence and making poor assessments.

The following are guidelines to assist CPS staff in interviewing the subject of a report:

1. Introduction:

CPS can start the initial interview by asking general questions about the family. It is best to start broadly before narrowing in on the allegations. You should begin the conversation with the parent/subject by introducing yourself and explaining the reasons that you are interviewing them. For example:

- "I am from the New York City Administration for Children's Services. I am here because there are some concerns about the safety of your children. The reason for my visit is to determine if they are safe and to assist you and your family in keeping them safe, if necessary. To do this, I need to ask you some questions about your children and family. I also want to listen to what you have to say."

2. Engagement:

After you have introduced yourself and explained the reason for the interview, you have an opportunity to ask general questions about the family that will help you to build rapport with the family member and gather information that you will need to assess the safety of the children. Prior to even exploring the allegation with the parent/caretaker and/or subject you should first try to obtain information on the family composition, sleeping arrangements, times when the subject may be alone with the child, how children are disciplined, etc. It is important to interview each household member separately and to ask children these same questions, separate from their parents/caretakers, and compare the answers of the family members. Information that must be

obtained during this interview is listed in interview related subsections and in the section entitled “Case Initiation” in the Casework Practice Requirements.

3. Free Narrative:

When it is time to discuss the allegations, do not begin by accusing the subject or reading the allegations described in the intake report. At this time it may be helpful to start by asking questions that will allow the subject to have an opportunity to provide his or her own version of the events, like:

- “As I said earlier, I am here to discuss concerns that have been brought to our attention regarding your children. I am interested in hearing how things are going with you and your family and whether you are in need of any assistance.”

You should allow the parent/caretaker and/or subject to respond with minimum interruption at this stage of the interview.

4. Open-ended Questions:

After the subject has had the opportunity to share his or her own thoughts on how the family is functioning, you can begin exploring the allegations by asking open-ended questions. It may be most effective to start with questions that do not directly identify the allegations as described in the intake report. For example:

- “Do you know how your daughter got injured?” Instead of: “Did you punch your daughter in the eye?”
- “How is your child doing in school?” Does your son/daughter enjoy school?” Instead of: “Why is your child in danger of being left back?”
- “How is your child’s attendance?” Instead of: “This allegation says you do not send your child to school.”
- “Who takes care of your child when you are not home?” Instead of: “Why are you leaving your child home alone?”
- “Do you sometimes drink alcohol? How often? How many drinks do you have?” Instead of: “Do you have a drinking problem?”

5. Specific Questions:

At some point in the interview, it may be necessary to ask the subject specific questions that will allow him/her to respond to the allegations that were presented in the intake report. As stated earlier, **these allegations should not be read word for word** in order to protect the reporter and the child, and to enhance your ability to conduct an appropriate investigation. You can describe the nature of the allegations as they were reported in the intake report and give the subject an opportunity to explain his/her point of view. It is important to remember that most people do not readily admit to mistreating their child. **Denial of an allegation does not automatically mean that the information is not true or that there are not safety concerns for the child.** How and when you discuss or describe the nature of the allegations will be determined based on the circumstances of the case, the family and your own judgment.

6. Concluding the Interview:

Concluding the interview respectfully is an important opportunity to facilitate continued engagement with the subject and the building of trust. The subject will appreciate being given the opportunity to ask questions. You should always thank the subject for taking the time to speak to you and provide your contact information. This is also an opportunity to introduce the Notice of Existence and explain the next steps of the investigation.

Since every person is different, you may need to use the engagement and interpersonal skills you have learned to calm down a concerned parent. If the subject of the report asks specific questions during the interview process about what the allegations in the report are, you can describe the nature of the allegations and emphasize that investigating the information described in the report is only one part of what you are responsible for. You can explain that you are also responsible for determining whether the family needs any assistance. Efforts to focus the conversation on the questions that you have for the subject should be made before describing the nature of the allegations. However at times sharing this information will be an important step in engaging the parent/subject.

During your interviews you can expect denial, but you **should not assume the neglect or abuse did not occur solely because the subject, family members or children denied the allegations. A statement of denial does not end the investigation.** All CPS investigations must include contacting collateral resources who may have information about the safety of the children, such as the source, schools, pediatricians, other family members, the police, parole officer, the building superintendent, and the neighbors. Information gathered by various contacts must be compared and assessed when determining whether there is “some credible evidence” to substantiate an allegation.

It is important to remember that by the conclusion of the investigation you must gather all of the necessary information to determine whether there is credible evidence to substantiate or unsubstantiate each allegation, and whether any interventions are necessary to ensure the safety of, and minimize the future risk to, the children.

For investigations relating fatalities and/or serious injuries relating to unsafe sleeping conditions review links:

- [Guidance for CPS Investigations on Infant Fatalities and Injuries Involving Unsafe Sleeping](#)
 - [Guidance for CPS Investigations of Infant Fatalities and Injuries Involving Unsafe Sleeping](#)
 - [Tip Sheet for Infant Sleep Related Injuries and Fatalities](#)
 - [OCFS Fatality Report Format](#)
 - [Securing Records in High Profile and Fatality Cases](#)

Required task:	Assess/describe/document:
1. Provide appropriate notifications (Notice of Existence) and explain your role and purpose. Link to review: OCFS Translated Versions of NOE All subjects and other persons named in a CPS report are entitled to a Notice of Existence of a CPS report. This notification must be printed from CONNECTIONS and mailed or hand delivered. *In the event that CPS arrives at the case address and no one is home. CPS is to leave a notice of visit in a secure location. Link to review: • Notice of Home Visit	• Method of notification • Names of persons notified and dates of notification
Required task:	Assess/describe/document:
2. Interview the adults in the home, establish their relationship to the child (ren) and to each other, determine who are the primary and secondary caretakers, who else has substantial contact with the child, and if there is more than one household. Identify adults who frequently visit the home. Begin to assess who and what may be a threat to the child and who may be a support for the child's safety. Obtain basic facts about the parent/caretaker for each child, regardless of where they reside. *If information about absent parents is not shared during the interview, review the following sources for information about absent parents/caretakers: past case records, DIR's, child's birth certificate, social security records, income maintenance, Medicaid, immunization records, and child support information	• Ensure that the interview is conducted in the language that is familiar and comfortable for the family. Use your cell phone for the Telephonic Interpreter or contact the Interpretation & Translation Services to obtain a face-to-face interpreter or American Sign Language (ASL) assistance if needed. Consult with your on-site Language Service Liaison. Record the following for all parents, and for each household member, boarder, and adult who frequents the home: • Request proof of name, address, date of birth • Race and ethnicity • Religion • Source of income, if SSI/SSD is identified document the nature of the disability • Primary language/sign interpretation; • Citizenship status

Interview everyone residing in the home and obtain basic facts about the household and verify household composition Links to review: • Procedure for Handling Cases with Diplomats, Consular Officials, and any Foreign Government Officials • Indian Child Welfare Act (ICWA) • Language/Sign Language Policies • Anti-Discrimination Policies • ACS Immigrant Resources Obtain information regarding the family's strength and support systems	• Marital Status • Physical description (height, approx. weight, body shape, facial features, complexion, any special features, injuries and general physical condition). • Document the role each adult plays in the caretaking of the children • Document whether members of the household are a source of support or if applicable can be kinship resources or know of possible kinship resources • Identify supports to the household (Including but not limited to, other family members, friends, a church, PTA, Tenant Patrol/Neighborhood Watch/Community Centers/Community Based programs) • Document any refusals to give information
Required task:	Assess/describe/document:
3. Obtain medical information from the child's parent/caretaker Health insurance and carrier Link to review: Health Care Coverage Information Request medical releases be signed Links to review: • Gathering and Assessing Medical Information during the Child Protective Investigation Policy • Child Safety Alert #17: Gathering and Assessing Medical Information • Hospital Contacts • HIPPA forms in various languages	• Immunization record and whether the parent believes immunizations are up to date. • Location of usual source of health care. • Primary care provider's name, phone number and address. • Date of child's last doctor visit and reason for the visit. • Date of child's last full medical examination. • Significant medical history, including diagnoses, medications and/or medical equipment used. • Observe medication and document names and recommended dosage • Type of coverage • If coverage meets family's needs

Required task:	Assess/describe/document:
4. A face to face interview with all alleged subjects of the report, parents and caretakers. Obtain accounts of and address each allegation. NEVER read the allegations to the parents/caretaker. • Let the parent know that there has been a report • Communicate to the parent that there are safety concerns that the CPS is here to investigate • Use words like “concerns” rather than “allegations” • Begin by asking questions before discussing/explaining what the specific concerns/reasons/allegations are • NEVER divulge the source	Account of each alleged subject, parent/caretaker concerning each allegation: • What, when, where, how • Who was present • Explanations for the incident and each allegation • How does the subject, parent/caretaker feel about and react to the incident/allegations • Their response to the incident reported (both the verbal and non-verbal response) Link to review: • Interviewing Tip Sheets

Required task:	Assess/describe/document:
5. Assess parent/caretaker’s ability to care for children, protect against the threat of harm and to meet child’s basic needs for food, clothing, shelter and/or medical care (immunizations, treatment for illness or injuries). Explore child care arrangements. Protective Capacity: • Cognitive- does the parent/caretaker recognize and understand the safety concerns that are present and is able/willing to do the task that protects the child; • Behavioral: what were the actions that the parent/caretaker took to mitigate the safety concerns; • Emotional: what relationship does the parent have with the child; does she	• Whether parents provide sufficient supervision/child care arrangements and who else cares for children. If older children care for younger siblings obtain the parent/caretakers assessment of their ability to do so. • Explanation of any situation in which parent did not meet child’s basic needs • Methods of discipline (Assess whether suitable and age appropriate forms of discipline are utilized) • Type of support parent receives from friends, relatives, neighbors, significant others and those who help the family • Caretaker’s recognition of threats to children’s safety

blame the child for the actions and/or inappropriate reactions of the subject parent/caretaker. Factors to consider when assessing for protective capacity: • Will the parent/caretaker protect the child during times of crisis? • Assess the parent's pattern of behavior in emotionally charged situations to know if they can be relied upon to protect the child. • What is the plan of the parent/caretaker to protect the child? • Discuss the factors that are influencing the parent/caretaker protective capacity. <i>For example: mental health, DV, substance use, developmental disabilities.</i> • Discuss how these factors increase or reduce the parent / caretaker protective capacity. • Discuss your assessment of the protective capacity (understanding, ability and willingness to protect) of the other family members.	• Efforts made to remedy threats to the children's safety • Whether caretaker exposes child to criminal acts • Whether caretaker targets one particular child • Whether caretaker views child in negative terms that result in rejection, hostility, resentment, indifference towards the child • Whether caretaker has realistic expectations of the child(ren) • Strengths- communication, energy level, self-awareness, organizational skills, nurturing attitude towards child • Finances and its impact on family functioning. Identify all sources of income and expenditures and note any discrepancies between living circumstances and known income • Opinion of prior services/placement received
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Required task:	Assess/describe/document:
6. Obtain information from parent/caretaker on children's school history to assess if educational neglect may be occurring as well as to gather information on the safety of the child and his/her needs. Parents' explanations for protracted absences must not be accepted without further investigation. Request School Information Release and if applicable, School Based Support Team/Committee on Special Education/Pre School Special Education Release be signed by parent(s)	• Name and location of school • Whether the child attends school regularly and knowledge of school performance • Any special education and/or supportive needs • Pattern of absenteeism/truancy • Efforts made by parent/caretaker to address absenteeism/truancy • Prolonged absences from school • Academic and school related strengths

Links to review: • DOE Policies and Resources Folder ○ Joint Policy Statement on the Reporting and Investigating of Ed. Neglect ○ Chancellors Regulation's ○ ACS Educational Website • Child Safety Alert #18: Educational Neglect Cases, Coordinating with the Department of Education	• Assess any prior allegations of educational neglect
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Required task:	Assess/describe/document:
7. Ask the parent/caretaker about any prior maltreatment history Link to review: • Chronic Neglect: Identifying and Assessing	• Severity of prior maltreatment/abuse • Efforts and willingness to prevent further maltreatment/abuse • Efforts made to eliminate threat of harm • Whether caretaker's response to previous incidents was not commensurate with the severity of the abuse or maltreatment • Severity and immediacy of threat to harm child

Required task:	Assess/describe/document:
8. Describe parent/caretaker's overall mental and physical health and how it affects his/her ability to care for children Consult and share information with Mental Health and/or Medical Consultant for assistance with screening, assessment and service planning, or to make home visits with you as needed Link to review: • Principles to inform child welfare decision making regarding mental	• Current health, medical history, special health needs, diagnosis, required care, medication, name and number of health care provider. Identify any chronic health problems or chronic pain conditions. • Whether parent/caretaker's apparent or diagnosed mental health status or developmental disability seriously affects his/her ability to supervise, protect or care for the child(ren) • Whether parent/caretaker is receiving mental

health issues	health services and/or attends or has attended any support groups. • Whether the parent/caretaker is receiving services for developmental disabilities, or attends a support group(s). If so, does he/she feel that the services are effective? • Any potential negative effects of health problems on children? • Whether behavior is violent or appears out of control • Identification of sources of stress- who or what upsets parent • How parent handles upsetting situations and what they do when they are angry • Recent changes or losses that have added to the parent's stress • Any symptoms such as loss of appetite and inability to sleep • Any problem with communication or memory, or cognitive skills • Whether there is any evidence of psychotic or delusional thinking • Whether parent/caretaker appears to be cognitively limited or intellectually challenged (If he/she attended special education or is currently receiving services) • Whether parent/caretaker was abused/maltreated as a child and has the parent been diagnosed with, or disclose a history of emotional disturbance, alcohol or drug dependence (including prescription medication), physical scars, disability and/or impairment • Whether parent/caretaker was raised in foster care, how he/she feels about that experience
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	and if he/she received any counseling or services
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Required task:	Assess/describe/document:
9. Domestic violence screening is required for all CPS investigations, regardless of type of allegations. Links to review: • DV Screening Tool • DV Protocol • DV Resources for CPS Investigations Interview household members separately. Do not confront alleged or suspected abusive partner in front of the survivor or children. The safety of the children and survivor is paramount. Engage survivor in safety planning and explore options including holding the abusive partner accountable. Consult with supervisor, Domestic Violence Consultant, Investigative Consultant or FCLS to determine necessary actions to hold abusive partner accountable and reduce threat to children and survivor. DV consultants are available for assistance with screening, assessment and service planning, or to make home visits with you as needed. Request Domestic Incident Reports (DIRs) from the Investigative Consultant on site. Review DIRs obtained and document pertinent information.	DV Screening conducted with parents/caretakers concerning his/her partner (spouse, companion, significant other, etc.): • Whether partner is controlling or jealous • Whether partner was ever made to feel afraid or unsafe • Whether partner interferes with parenting • Whether partner ever threatened, hit or hurt parent • History of police involvement in domestic violence incidents, orders of protection or prior DV allegations on CPS reports If DV is alleged, suspected or uncovered, conduct a comprehensive assessment to determine: • Substantial risk of serious physical harm, as indicated by: • extreme imbalance of power • isolation • threats of harm to survivor or children • escalating violence • history of serious violence/injury • use of/access to weapons • abusive partner's substance abuse or mental illness • threats of suicide by abusive partner • stalking behavior by abusive partner • threats of homicide or suicide, or threats to harm the survivor, children or pets by the abusive partner • Any bruises or injury to the survivor or children • Survivor's ability to participate in safety planning to protect self and children. Refer to the DV Protocol and DV Specialist for safety planning strategies.

Link to review: • Accessing NYPD DIR • Child Safety Alert #26: Domestic Incident Reports • CPS Access to Criminal History • Reporting of Criminal History to Outside Agencies Complete Screening for Domestic Violence. Refer to and complete the Domestic Violence Protocol if DV is alleged, suspected or uncovered during the interview. Refer to the ACS DV Guiding Principles. ACS believes that one of the most effective ways to enhance children's safety after the detection of domestic violence is to support and help non-abusive parents to protect themselves and their children while engaging abusive partners in services and holding them accountable for their actions. Link to review: • New York City Administration for Children's Services Principle Guidelines for Addressing Domestic Violence • Case Practice Guidelines for Cases Involving DV • OCFS Guide for Identifying Domestic Violence Do not disclose confidential address of survivors of domestic violence.	• Abusive partner willingness to take responsibility to end the violence and engage in services to stop the abuse • Other actions or interventions necessary to hold the abusive partner accountable and reduce the threat to the survivor and children – including referral to police or legal action where appropriate.
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Required Task:	Assess/describe/document:
10. Once a CPS has some verification that they believe a family is in a DV shelter, they must contact the ACS Domestic Violence Policy and Planning Unit (DVPP)	• Document information that leads CPS to believe the family is residing in a DV shelter. • To locate persons known to ACS who may be residing in a Domestic Violence Shelter, please email the ACS Domestic Violence Policy and Planning Unit (DVPP) both B. Indira Ramsaroop brahaspa.ramsaroop@dfa.state.ny.us and Denise Walden Greene denise.waldengreene@dfa.state.ny.us Include the following information in your email: a. Name and date of birth of the parent/guardian and child; b. Social Security number of parent/guardian and/or child, if known; c. Aliases the parent/guardian may use; d. A brief summary explaining why the CPS knows or believes the family is in a domestic violence shelter; an e. The CPS' and supervisor's name, telephone number, office and unit number *Workers should be informed this procedure is <u>not</u> for diligent searches. The specificity of the information regarding the names and dates of birth of the client and the client's children ensures that the correct client is located in the event that there is

	more than one person in the DV shelter system with the same name. If any of the above information is missing the request will be returned to the worker asking for the additional information
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Domestic Violence Safety Planning

A Domestic Violence Safety Plan is an immediate response to ensure safety for the survivor and children. The CPS will assist the survivor in developing a plan for safety. It is vital to consider the survivor's past experience with help-seeking, and to acknowledge that ending the relationship or setting limits with the abuser may **Increase Danger** to the survivor and the children in the short-term. It may be unsafe for the survivor to write down the safety plan, or to take literature or information with her if she lives with the abuser. If it is dangerous for her to take the safety plan with her, then it is suggested that CPS develop an alternate method to enact the safety plan (e.g.: leaving copies of important documents with a trusted friend or family member, having the survivor

memorize a phone number where the plan can be initiated or having the survivor memorize the steps to action when initiating the plan.

Important Factors to Explore

- Ask about cues that precipitated past domestic violence incidents (alcohol, drugs, stress, arguments, weekends, nights, etc.)
- Ask about what has worked in the past to protect the survivor and children?
- Identify where the survivor can go for help if there is an emergency, including NYPD and hospitals (number, address).
- Identify who the survivor can call or go to for help in an emergency (number, address)
- Discuss what can be done legally (Orders of Protection, calling police and or parole/probation officer, meeting with District Attorney if charges are pending).
- Ask if there is a current Order of Protection, and if so, make sure school, day care, neighbors, baby sitters are aware/have a copy. Ask neighbors to call police if the abusive partner is seen. Contact local D.V. P.O. to discuss other strategies.
- Access to important items for survivor and children (keys, birth certificates, medication, money, passports, social security information, physician's records, eyeglasses, etc.)
- Offer to help survivor obtain a cell-phone programmed to dial 911 (contact the Deputy of Administration for your office).
- Offer to help the survivor have their home locks changed. Each Borough has designated personnel to initiate this request through an ACS approved vendor.
- Discuss available services (Safe Horizon, Family Violence Prevention Program, PPRS, Alternatives to Shelter Program, Non-Residential, Community, Residential, etc.) (Refer to CPS DV Protocol). Provide the survivor with the **1-800-621-HOPE (4673)** hotline number.
- **Consider history of domestic violence when planning Child Safety Conferences. Joint attendance at conferences may endanger survivors or violate Orders of Protection.**
- Whenever possible, place responsibility for abusive behavior on the abusive partner in documenting findings of investigation and planning interventions.

Assessing Allegations of Mutual Abuse

- By definition, domestic violence typically involves a **pattern of behavior** in which one partner—the abusive partner—uses violence and other forms of abuse to gain and maintain **power and control** over the other partner—the survivor. However, some domestic violence cases present as “mutual abuse.” There may be allegations or criminal complaints against both partners, and dual Orders of Protection.

The following information will assist CPS in identifying patterns of abusive behavior and the primary aggressor in the relationship. Efforts should always be made to identify the primary aggressor. CPS should also consult with the Domestic Violence Consultant on all patterns of abuse identified.

A thorough assessment would include:

- Identifying which party exercises power and control over the other; Careful interviewing and observation of both partners and children
- Reviewing history of domestic violence incidents in the household
- Reviewing existing police reports, arrest history, criminal cases, Domestic Violence Incident Reports (DIRs) or temporary or final orders of protection
- Conversations with police or domestic violence service providers who have had contact with parties
- Interviewing all parties separately and assessing through investigation who is placing the child(ren) at risk; who is using controlling or coercive tactics in the relationship; and who is afraid of whom
- Doing collateral interviews with neighbors, school personnel, responding police officers
- Looking at physical evidence such as bloody or torn clothing, broken furniture, etc.
- Examining each person’s role and actions and asking: who has been focused on keeping the children safe and whose behavior has placed them at risk? Sometimes actions may seem puzzling but when we do a deeper analysis, we often uncover ways in which survivors strategize to protect themselves and their family members
- Identifying nature of physical injuries, if any, slashes or bruises on hands or forearms may be defensive injuries incurred while blocking blows. The survivor may have used force against the abuser in self-defense
- Factoring in that it is not mutual abuse when a survivor resorts to violence in self-defense
- Keeping in mind that the comparative size of the parties can be important in making an assessment, however, it is not always the determining factor in assessing who is the primary aggressor
- Using the Suspected Batterer’s Interview in the DV Protocol as a preliminary assessment for the suspected abusive partner

- Consulting with the Clinical Consultation Team and/or the Office of Domestic Violence Policy and Planning (DVPP) after going through the above analysis

Characteristic Information for Interview Awareness

- ☐ **Characteristics of abusive partners:** May be charming, manipulative, or threatening in interactions with authority figures. Often deny, minimize, or excuse their abusive behavior. Often present as survivors. Communicate sense of “ownership” of partner, children. Hold primary control in family. Isolate partner and children from support systems. Seek to control interactions with outside systems. Have been abusive in previous relationships.
- ☐ **Characteristics of survivors:** Self-blame. Fear of reprisals for revealing abuse or ending relationship. May use physical violence in self-defense. May be isolated from support systems. More likely to be injured.
- ☐ **Children’s responses:** Often, children identify accurately which adult is “scary” or “hurts people” in the family, and which adult is in danger. Children often worry about the survivor’s safety, and fear separations from the survivor. However, many children are loyal to an abusive parent, and some children identify with the abusive partner, and may blame the survivor for the abuse or repeat the abusive partner’s excuses.

Required task:	Assess/describe/document:
11. Conduct separate interview with companion/significant other and describe the relationship between parent/caretaker and companion/significant other. Domestic violence in the home can complicate investigations. Adult and child survivors will in some circumstances try to mislead the investigator, either from fear or an unwillingness to speak ill of one’s family. Adult companions who are loosely connected to the home are often the source of such family tension. Investigators must be vigilant about determining who these individuals are, their role in the home, and as much of their background as can be gathered. Each person must be evaluated for his/her potential to harm each child in the home.	• Relationship between each child and the significant other • Whether the companion/significant other is a source of stress to the family • Whether the companion/significant other threatens the children’s safety • Whether the significant other was abused/maltreated as a child. If so, whether significant other has children of his/her own and if so who is caring for them • Whether the significant other has a severe mental illness that would impact on his/her appropriate interaction with the children.

Required task:	Assess/describe/document:
12. Interview Other Persons Named in the Report (adults listed on Intake Report Summary or the CPS Investigation Summary who are not identified as subjects). Interview all persons designated as Alleged Subject on the List of Principles of the Intake Report Summary and all persons designated as Confirmed Subject or Non-Confirmed Subject on the CPS Investigation Summary. Conduct separate interviews of all household members and adults who frequent the home.	• His/her explanation of the incident and allegations compared to the parent's and child's accounts • How he/she is involved with the family • Interaction with parent/caretaker • Interaction with each of the children in the household • Whether he/she can be a resource to the family • If applicable, whether he/she can be a kinship resource or know of possible kinship resources • Whether he/she is a source of stress to the family • Whether he/she threatens the safety of the children • Any knowledge of caretaker or other household member's current or past involvement with child welfare
Required task:	Assess/describe/document:
13. Assess whether or not there is substance abuse (alcohol or drug) by parent/caretaker or other household members. If so, evaluate impact on child's safety and family functioning. Link to review: • Drug Assessment Check List Substance/drug screens must be conducted within 24 hours of the request. Do not request the parent/caretaker or household member to undergo the screen until lab access is ensured within 24 hours. A substance/drug screen alone is NOT sufficient for an assessment. The parent/caretaker or other household member must be assessed at a substance abuse program, or by the Substance Abuse Clinical Consultant. Consult and share information with Substance Abuse Consultant for assistance with screening, assessment and service planning, or to make home	• Results of drug screen and/or substance abuse assessment • Impact of substance abuse on parent/caretaker's ability to supervise, protect or care for the children • Whether parent/caretaker or household members abusing drugs/alcohol is willing to receive treatment, if he or she has received treatment in the past, and what was the result of the treatment. If applicable, describe if any of the following symptoms: • Alcohol on breath • Client or home smells of marijuana, crack, methamphetamine, or other drugs • Drug paraphernalia, empty beer cans or liquor bottles are present in the home • Pupils constricted, "pinned" or dilated in well-lit room (indicates stimulant use) • Head drops, eyes close—"nods out"

visits with you as needed. *A positive test for substance abuse is not enough to indicate substance abuse. CPS must document and correlate the impact of the parent/caretakers drug use to their ability to care for the subject children and protect against a threat of harm.	• Anxious and moves about quickly • Swollen “boxing glove” hands • Pattern of small circular scars or scabs over veins on backs of hands, inner arms or ankles- “tracks” • Lips and fingers burned (from crack pipe) • Eyes tear, nose runs, sweating (withdrawal symptoms) Irritable, mood swings, poor personal hygiene or appearance • History of prior substance abuse but denies current abuse and refuses drug testing • Attention is impaired • Appears apathetic or acts unusually calm to unsettling events/situations • Hands shake, is unable to sit still • Gives misleading information about self and social functioning • History of frequent income support case closing • Unexplained source of income • Appears hyper, rapid speech, inability to sit still
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CONNECTIONS Documentation Requirement:

- Link to review: [Use of Child/Parent Template to Assess Parent/Caretaker Protective Capacity and Child Vulnerability](#); [Child Parent Templates](#), [DV Screening Tool](#), [DV Protocol](#), [Drug Assessment Check List](#), and any safety planning initiated during this time.
- Link to review:
 - [Court Authority to Order Court Ordered Investigations](#)

Course of Action:

- ✓ Conduct or refer for formal substance abuse assessment
- ✓ **Without delay**, arrange for a drug screen test, if client is willing or ordered by the court.
- ✓ If domestic violence is suspected, complete the Domestic Violence Protocol. Consult with the Investigative Consultant who obtain and interpret Domestic Incident Report (DIR's).
- ✓ Assist in safety planning and needed services for survivor and children, including legal action to hold the abusive partner accountable.
- ✓ Refer to the Safety Assessment guidelines when considering the immediacy and seriousness of the unsafe conditions and whether the children require protection.
- ✓ If the children are in immediate or impending danger, identify and arrange for the appropriate safety interventions.
- ✓ A Family Meeting or Initial Child Safety Conference (ICSC) may be requested. (A Child Safety Conference must occur when imminent danger to a child exists.
- ✓ If you have any questions or concerns, consult with your supervisor.

INTERVIEWS WITH COLLATERAL CONTACTS

Child Protective Specialists are responsible for interviewing collateral contacts as part of every child protective investigation. CPS will interview collateral contacts to obtain additional information concerning the allegations and the possible existence of any abuse or maltreatment from those who are familiar with the child and family. The information obtained will assist in the assessment of whether the children are in immediate danger or if there are risks of future abuse or maltreatment.

Appropriate collateral sources include but are not limited to: individuals who know the family, family members, neighbors, medical, mental health, service providers and other agencies providing services to the family, schools, police, child care providers, religious or community institutions and other persons who may have information relevant to the allegations in the report and to the safety of the children.

Required task:	Assess/describe/document:
1. Contact and obtain needed information from all appropriate collateral sources of information. Link to review: • Child Safety Alert #23 Interviewing Neighbors and Superintendents as part of CPS Investigations Mandated reporters (e.g.: teachers) who make reports must be contacted immediately, unless emergency action to protect a child needs to be taken first. In instances when mandated reporters can not be reached on first attempt, be sure to leave messages specifying times and telephone numbers they can use to contact CPS. Contacting a teacher or guidance counselor weeks after she has reported abuse or neglect is not acceptable case practice. Link to review: • Revised SCR Reporting Requirements for Mandated Reporters and Expansion of Social Service Workers Classified as Mandated Reporters	For each interview with a collateral source: Ensure that the interview is conducted in the language that is familiar and comfortable for the collateral contact. Use your cell phone for the Telephonic Interpreter or contact the Interpretation & Translation Services to obtain a face-to-face interpreter or American Sign Language (ASL) assistance if needed. Consult with your on-site Language Service Liaison. Links to review: • Language/Sign Language Policies • Inquire as to the primary language of the family • How long each collateral has known the child and/or family • Collateral's assessment of the child's condition • Collateral's assessment of family functioning, including their knowledge of any safety factors or risk elements that may be operating in the family • Collateral's knowledge of the incident/allegations, and any other signs of abuse or maltreatment

	• Collateral's knowledge of parents efforts to protect against threat of harm
Required task:	Assess/describe/document:
2. Contact medical provider and obtain medical information in all CPS investigations. Links to review: • Child Safety Alert #17: Gathering and Assessing Medical Information When physical abuse is alleged or suspected, CPS must contact the child's medical provider at any and all medical institutions that treated the child. Consult and share information with Medical Consultant as appropriate. Cases involving allegations of physical or sexual abuse should be assessed by a multidisciplinary team, preferably at a Child Advocacy Center or Child Protection Center. Obtain x-ray films, records of physical examination, and photos of visible trauma.	• Date of the child's last visit and the reason for the visit • The name of the person who accompanied the child (gave permission for treatment). • Any incidents in which the parent/caretaker delayed treatment or failed to follow medical advice. • Any concerns about the parent's ability to care for the child and the reasons for the concerns. • The dates of immunizations and whether there are missing immunizations. • The medications and/or medical equipment that have been prescribed, indications for their use compliance, and whether the parent has been trained to provide/use them with the child. • In any case where there is a new baby (regardless of whether the new baby is a subject of the allegations), CPS must make a medical collateral contact to make sure the baby is going to well-baby visits and to find out if the new baby has any special needs. • If CPS learns of a child's specialized medical condition, confirmation must be made with a medical professional. An assessment of whether the parent understands the condition and their ability to manage the child's necessary care must also be made. • If CPS learns from the medical provider that the child is being seen by a mental health professional, CPS should follow up with that mental health professional for information (diagnosis, medication prescribed, compliance with treatment, parent/caretaker understanding of treatment, etc.) • Medical professional's description of the injury or injuries. • Medical professional's opinion of whether

	the injury is consistent with the explanation provided by any caretakers, including the findings supporting that opinion. • Medical professional’s opinion of whether the injury is consistent with the explanation provided by the subject child, or any other child or collateral contact that has provided an explanation. • Other medical findings or observations that support a suspicion of abuse. • Current or prior history of multiple visits to various health care providers/facilities for a variety of injuries (as part of an assessment of whether the caretaker is trying to avoid having a single provider know how many injuries a child has experienced and to rule out certain parental mental health diagnoses). • Severity, frequency or extent of injuries has escalated over time. • Conflicting, inconsistent and evolving (changes over time) reports regarding the mechanism, timing and circumstances surrounding the injury. • Caretaker delay in seeking care for the reported injury or “under reporting” the seriousness of the injury.
Required task:	Assess/describe/document:
3. Interview school staff/day care staff/educational provider. Link to review: • DOE Policies/Chancellor's Regulations and Resources • Early Care and Education (aka Child Care and Head Start) Regardless of whether the source of the report is a school official or the allegation is educational neglect, the child’s school must also be contacted as soon as possible	• Name and location of school • Whether child’s basic needs appear to be met • Collateral’s interaction with child and family • Date the 700B form was submitted to school to obtain reports. • Document the results of Automated Schools (ATS) database search, including absences, lateness, test scores, etc. • Behavior and physical appearance of child when attending school • Whether the child attends school regularly and school performance (current and historical) and any unusual changes noted by school staff

to obtain all information about safety and risk, child's attendance, school performance and overall functioning. CPS is required to visit the school and to conduct interviews with school personnel face-to-face in order to gain a better understanding of the child and fully assess the situation. It is often necessary to speak with school personnel at different points in the investigation in order to make a thorough assessment. CPS must document if, and why a school visit did not occur.	• Pattern of absenteeism/truancy • Any known or suspected developmental delays and any completed assessments or testing results • Date of contact and persons contacted at the school not limited to Source of the report, Guidance Counselor, Teacher, School Social Worker, Principal • Assess any prior allegations of educational neglect • If a child received Special Education Services, obtain copies of the Individualized Education Program (IEP) and supportive documents, (psychosocial, psychological testing, educational testing).
Required task:	Assess/describe/document:
4. Interview other collateral sources such as other family members, baby sitters, after school program staff, service providers, District Attorney (if applicable) and others who are familiar with the child and family. CPS may use collaterals listed in prior records. Like to review: • Sharing information with District Attorney's Office Department of Probation and Department of Parole	• Unusual changes • Whether child's basic needs appear to be met • Collateral's interaction with child • Any concerns they may have (i.e. weapons in the house, excessive use of alcohol or drugs, domestic violence, etc.)
Required task:	Assess/describe/document:
5. For cases involving domestic violence, obtain any prior domestic incident reports from the Investigative Consultant. CPS should also consult with the Investigative Consultant on cases where there is current or prior criminal activity, missing children or alleged subjects. Links to review: • Accessing the NYPD DIR System • CPS Access to Criminal History Reports	• Frequency and nature of police incidents, prior arrests and convictions involving domestic violence. • CPS is to seek the assistance of the Investigative Consultant to interpret all report(s).
Required task:	Assess/describe/document:

6. Re-interview caretakers and children as necessary to explore inconsistencies in information from collaterals and the family. Compare all of the collateral accounts to the information gathered from interviews with children, parents/caretakers, alleged subjects and household members. Integrate information obtained from collaterals into the safety assessment, risk assessment and allegation determination decision making processes.	• Discrepancies in accounts and corroborating information • Quality of information- accuracy, objectivity and reliability.
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CONNECTIONS Documentation Requirement:

Complete Interviews of Collateral Contacts. The medical professional's name, professional licensure (i.e., MD, RN, and NP), affiliation (hospital or clinic) and field of practice or expertise must be documented in the record.

Course of Action

- ✓ When information gathered during the investigation meets the criteria for referral to the DA's Office, refer the case to the Criminal Justice Coordinator who will forward the report to the DA's office.
- ✓ If the case meets the criteria for an Instant Response Team (IRT), contact the Instant Response Team Coordinator for a possible IRT.
- ✓ Refer to the Safety Assessment guidelines when considering the immediacy and seriousness of the unsafe conditions and whether the children require protection.
- ✓ A Family Meeting or an Initial Child Safety Conference, when imminent danger to a child exists.
- ✓ If the children are in immediate danger, identify and arrange for the appropriate safety interventions.
- ✓ If you have any questions or concerns, consult with your supervisor.

SAFETY ASSESSMENT

SAFETY vs. RISK

Links to review:

- [Safety Assessment Policies/Guidance](#)

To support the child welfare goal of safety, permanency and well-being for children and their families, two assessments are completed during the course of every CPS Investigation: (1) Safety and (2) Risk (which will be discussed later in this manual). The first priority after receiving a report from the State Central Register (SCR) is to conduct a thorough investigation to assess the safety of each child. The assessment of safety is ongoing throughout the investigation. Although CPS are required to document safety at Day 7 and Day 55 of the investigation, **CPS should ALWAYS assess safety during every contact with family members and collaterals to determine whether a Safety Modification and/or appropriate intervention action are necessary.**

Safety

- Safety deals with **present or impending danger**.
- Safety factors pose a threat of immediate or impending harm.
- Safety factors pose a threat of serious harm.
- In order to control for safety, there must be an assessment of the need for immediate interventions, as well as strengths, resources or protection factors present in the home.

Risk

- Risk is the likelihood that a child may be abused or maltreated **in the future**.
- Risk Elements identify significant behaviors and circumstances within a family unit that create different levels of risk to the child.
- Risk may be very high even if the report is unfounded; conversely, risk may be low even if the report is indicated.
- Risk may be reduced with appropriate services, changes in the caretaker's behavior, and family or community support.

Utilize all the information gathered in home visits and contacts with children, parents/caretakers and collaterals to complete your safety assessment and decision.

SAFETY ASSESSMENTS ARE REQUIRED TO BE CONDUCTED, DOCUMENTED AND SUBMITTED FOR APPROVAL WITHIN THE FOLLOWING TIMEFRAMES:

- **Seven Day Safety Assessment:** Sufficient information gathered, decision made, documented, and submitted within five (5) days and approved within seven (7) days from receipt of a report of suspected child abuse/ maltreatment.
- In response to any **registered 'subsequent' reports**, and must also be completed within five (5) days from receipt of the subsequent report.
- Within twenty-four (24) hours from **receipt of a fatality report**, if there are

surviving siblings and when completing a 30-day child fatality report.

- Within seven (7) days of the completion and submission of the **Investigation Conclusion**.
- As part of the **Family Assessment and Service Plan (FASP)**, completed at each Initial, Comprehensive and Reassessment FASP for all cases with a program choice of “protective”.
- **Safety Modification:** At any point during the investigation, the CPS should submit a Safety Modification if any of the family’s circumstances has changed.

CPS must comment on all of the Safety Factors selected. Comments must describe the behaviors and/or conditions observed by CPS and/or by reliable collaterals that support the selection of the safety factor.

Even if “No Safety Factors Identified” is selected on the Safety Factors tab, comments should be recorded in the Comment field. The CNNX progress notes should also support the safety assessment and selected decision.

For each safety factor that applies, assess the following:

- **Immediacy** (whether a dangerous situation is already present or likely to occur in the immediate future)
- **Seriousness** (whether there is a dangerous situation that presents likelihood of serious harm to a child’s life or health)
- Whether **protection** is required to protect child from dangerous behaviors or conditions
- **Child’s vulnerability**

Links to review:

- [Interviewing Template Tip Sheets to include Protective Capacity and Child Vulnerability](#)

THE FIVE SAFETY DECISIONS

A Safety Decision is CPS’ conclusion of whether the child is safe in the home. All of the other components of the Safety Assessment are used by CPS to support this decision.

1. No safety factors were identified at this time. Based on currently available information, there is no child (ren) likely to be in immediate or impending danger of serious harm. NO safety plan/controlling interventions are necessary at this time.
2. Safety Factors exist, but do not rise to the level of immediate or impending danger of serious harm. No safety plan/controlling interventions are necessary at this time. However, identified safety factors have been/will be addressed with the Parent(s)/Caretaker(s) and reassessed.
3. One or more safety factors are present that place the child (ren) in immediate

or impending danger of serious harm. A Safety Plan is necessary and has been implemented/maintained through the actions of the Parent(s)/Caretaker(s) and/or either CPS or Child Welfare Staff. The child (ren) will remain in the care of the Parent(s)/Caretaker(s).

4. One or more safety factors are present that place the children in immediate or impending danger of serious harm. Removal to, or continued placement in foster care or an alternative placement setting is necessary as a controlling intervention to protect the child (ren).
5. One or more safety factors are present that place or may place the child(ren) in immediate or impending danger of serious harm, but Parent(s)/Caretaker(s) has refused access to the child(ren) or fled or the child(ren)'s whereabouts are unknown.

SAFETY INTERVENTIONS

If CPS determines that the child (ren) is/are not safe in the current home environment, he/she must develop a safety plan wherein interventions are clearly matched to the present/impending danger. The plan/interventions must be reliable and sufficiently strong to control for safety until more detailed assessment and service planning can take place. To document your Safety Plan and Interventions, use the Safety Interventions tab to select interventions that control for the immediate health and safety of the child. One or more Safety Interventions may be selected from the list. Select which intervening action has been taken to support the child safety based on the Safety Factors and Safety Decisions the CPS has selected. Each safety intervention the CPS selects must be supported by comments.

- Intensive Home Based Family Preservation Services
- Preventive services
- Emergency Shelter
- Domestic Violence Shelter
- Non-offending parent/caretaker has moved to a safe environment with the child (ren)
- Authorization of emergency food/cash/goods
- Judicial Intervention
- Order of Protection
- Law Enforcement Involvement
- Emergency Medical Services
- Crisis Mental Health Services
- Emergency in-patient mental health services
- Immediate supervision/monitoring
- Emergency Alcohol abuse services
- Emergency Drug abuse services
- Correction or removal of hazardous/unsafe living conditions
- Placement- Foster Care
- Placement- Alternative Caregiver

- Supervised Visitation
- Use of family, neighbors or other individuals in the community as safety resources (specify)
- Alleged perpetrator has left the home voluntarily, current caretaker will appropriately protect the survivor with CPS monitoring
- Alleged perpetrator has left the home voluntarily, current caretaker will appropriately protect the victim with CPS monitoring
- Alleged perpetrator has left the home in response to legal action
- Follow- up to verify child (ren)'s whereabouts/ gained access to child (ren)
- Other (comment required)

CHECKING FOR COMPLETION OF THE SAFETY ASSESSMENT

CONNECTIONS will verify that all parts of the Safety Assessment have been completed. In order to complete the Safety Assessment, the following conditions apply:

- Comments must be recorded for ALL safety selections
- If “No Safety Factors Identified” is selected on the Safety factors tab, then “No Safety Factors Selected” must be the selected Safety Decision and explain why no safety factors were identified in the Comments that provide information on what behaviors or conditions were observed to be present or reported to be present by reliable collaterals or by the family.
- If Safety Decision #2 is selected, the Parent/Caretaker tab is enabled; however, the system will not require you to record a narrative. The Controlling Interventions/Safety Plan tab will be disabled
- If Safety Decision #3 is selected, both the Parent/Caretaker Actions/Safety Plan and Controlling Interventions/Safety Plan tabs will be enabled and require documentation.
- If Safety Decision #4 is selected, the placement window will be enabled and you will be required to document your reason for this selection. The Parent/Caretaker actions/safety plan and the controlling interventions/safety plan tabs will also be enabled and will require documentation.
- If Safety Decision #5 is selected, both the controlling interventions/safety plan and the parent/caretaker actions/safety plan tabs will be enabled and require documentation.
- A Safety Decision must be recorded.
- The Date and Type fields must be completed.

NOTE: A Safety Assessment must ALWAYS be completed unless the *Fatality – No Surviving Children* check box is selected on the CPS Investigation Conclusion window to confirm there are no surviving children in the stage.

RISK ASSESSMENT AND THE RISK ASSESSMENT PROFILE (RAP)

Link to review:

- [Risk Assessment \(RAP\)](#)

In addition to assessing for safety, CPS must decide if the child (ren) is/are at risk of future abuse or maltreatment. The assessment of risk differs from a safety assessment in that it is **focused on the likelihood that a child (ren) will be abused or maltreated in the future**. The assessment includes the identification of risk elements present in a family and a system-supported calculation of the “weight” of those elements. This system-supported calculation produces a risk score that helps the CPS determine if a family’s case should be opened for services in order to reduce the likelihood that the child (ren) will be abused or maltreated in the future.

Risk

- Risk is the likelihood that a child may be abused or maltreated **in the future**.
- Risk elements identify significant behaviors and circumstances within a family unit that create different levels of risk to the child.
- Risk may be very high even if the report is unfounded.
- Risk may be reduced with appropriate services, changes in caretaker’s behavior and family or community support.

Utilize all the information gathered in home visits and contacts with children, parents/caretakers and collaterals to complete your Risk Assessment and complete necessary documentation via the Risk Assessment Profile (RAP).

The Risk Assessment Profile (RAP)

CPS is responsible for thoroughly and continually assessing the safety and level of risk of every child in the household throughout the life of the CPS case and documenting this information in a timely and accurate manner in the case record.

Link to review:

- [DCP Guidelines for Completing the RAP in CNNX](#)

The RAP is designed to help CPS and supervisors make informed decisions regarding whether to open a case for services. This tool supports the ability of CPS and supervisors to classify and measure more accurately the level or severity of risk in a family. The RAP uses various scales to promote and support a structured and rational decision making approach to case practice without replacing the professional judgment of CPS and their supervisors. CPS uses the RAP in CONNECTIONS to document information the RAP then uses to calculate the level or risk. The RAP is to be completed during the Investigation stage in order to assist CPS in making informed decisions regarding whether to open a case for services. The RAP should be completed no later than forty days (40) after the initiation of a case or before a case is submitted for closure if less than 40 days. CPS staff is directed to complete and submit the RAP for approval when sufficient information has been gathered to inform the completion of the RAP. In circumstances where the

CPS staff must complete the RAP before sufficient information is available or the investigation is ongoing, CPS should leave the RAP in draft and alert their supervisor of the status. The RAP should remain in draft status until the additional information is gathered and documented, when the case has reached the 40th day or when CPS is ready to submit for investigation closure. In instances where an earlier service referral for the family is being made but the investigation is ongoing, CPS should complete the RAP but also keep this in draft status until additional information is received and available to include in the RAP. CPS should alert his/her supervisor of the draft status. The final RAP must be submitted by the 40th day of the investigation.

The RAP is intended to support the development of the service plan targeting resources to High and Very High risk cases. Once CPS decides to open the case for services and the family is either mandated to accept or agrees to accept services, a complete analysis of safety factors, risk elements, family strengths, family view and needs is necessary in order to develop of a service plan.

The RAP also supports the ability of CPS to accurately classify and measure the level of severity or risk in a family. The response to each Risk Element in the RAP will have an associated point value (or “weight”). The point value as determined by examining the strength of the correlation between the Risk Element and the incidence of subsequent indicated abuse and neglect reports for a sample of actual cases in New York State. The higher an individual Risk Element’s score is, the higher its contribution is to the Preliminary Risk Score. The Risk Assessment Profile (RAP) produces a numerical score that is calculated after all 15 risk elements are answered. Scores may range from 1-19.

Risk Elements Definition

1. Total prior reports for adults and children in RAP family unit.

Displays the number of prior indicated reports in which:

- An adult in the RAP family unit was a confirmed subject (regardless of report type); or
- A child in the RAP family unit was a confirmed victim of abuse or maltreatment I a familial report type.

This calculation includes prior indicated reports where an adult in the RAP family unit was a subject, regardless of whether or not the children who were abused /maltreated in the prior report are members of the current RAP family unit. This calculation also includes prior indicated reports where a child in the RAP family unit was abused or maltreated by an adult who is not part of the current RAP family unit. All persons in the caretakers of the child (ren) alleged to be maltreated.

This calculation does not include the following:

- Duplicate (DUP) stages
- Reports where all of the RAP family unit members had “No Role”

The response to this Risk Element displays as follows:

- No prior determined reports
- Prior unfounded reports only
- One to two prior indicated reports

- Three to four prior indicated reports
- Five or more prior indicated reports

2. Child (ren) in RAP family unit was in the care or custody of substitute caregivers or foster parent(s) at any time prior to the current report.

Indicates whether any child in the RAP family unit previously resided (or currently reside) with a foster parent or substitute caregiver, either informally or formally, for a significant period of time. The Placement does not need to have been due to child protective concerns; it could have been an informal family arrangement for one of many reasons.

3. Child (ren) under one year old in RAP family unit

Indicates whether there are any children younger than one year of age residing in the home, in foster care with a permanency planning goal of returning home, or temporarily in another living situation (such as living with a relative or in a hospital) but expected to return home. If the **DOB** field on the person detail window is blank for any person whose Rel / Int field signifies that the person is a child. CONNECTIONS include that person as a child younger than one year old in this calculation.

4. Inadequate housing with serious health or safety hazards, extreme overcrowding, or no housing

Evidence of inadequate or hazardous housing may include, but is not limited to, the following:

- Serious overcrowding
- Serious inadequate furnishings to meet the family's needs
- Inadequate heat, plumbing, electricity or water
- Lack or inoperability of essential kitchen appliances or bathroom facilities
- Multiple serious health hazards, such as rodent or vermin infestation, garbage and junk piled up; perishable food found spoiled; evidence of human or animal waste; and walls, floors, doors and furnishings thick with dirt and debris
- Multiple serious safety hazards, such as leaking gas from a stove or heating unit; dangerous substances or objects stored in unlocked lower shelves or cabinets, under sink, or out in the open; peeling lead-based paint; hot water or steam leaks from a radiator; broken or missing windows; and no guards on open windows.

In some cases, one or two isolated hazardous conditions will be corrected prior to the determination of the report, such as restoring heat or installing window bars. In these cases, the response to this Risk Element would be "No. "However, if the hazardous situation has been an ongoing concern, such as a filthy house with multiple hazards, and, based on past experience, the condition is likely to reoccur even if it has been cleaned up by the time of the determination; the response to this Risk Element would be "Yes."

5. Financial resources are severely limited or mismanaged to the degree basic family needs are chronically unmet.

This Risk Element is present if either of the following conditions exist

- The family does not have enough financial resources to meet the basic needs of the family for shelter, food, clothing and health. Benefits such as public

assistance, SSI, food stamps, public housing or housing vouchers, HEAP, etc. should be considered as financial resources that help meet the family's basic needs.

- The financial resources should be sufficient to meet the family's basic needs, but are not sufficient due to mismanagement or inappropriate abuse of funds.

6. Caretaker(s) in primary household has reliable and useful social support from extended family, friends or neighbors.

Indicates whether the caretaker(s) living in the primary household with the child (ren) has reliable and useful social support from informal sources, such as extended family, friends or neighbors. Reliable and useful social support is present when the adult caretaker(s) has a *network* of relatives, friends or neighbors to call upon for assistance in any area where the family may need help, such as child care, transportation, emergency financial or housing help, or emotional support. In addition, the informal social support network is nearby and readily available when needed.

7. Caretaker is a perpetrator of, or victim, of domestic violence, or has serious conflicts with other adults.

Domestic violence is defined as a pattern of coercive tactics that can include physical, psychological, social, economic or emotional abuse perpetrated by one adult against another adult. Examples of domestic violence include: grabbing, pushing, hitting, punching, kicking, choking, biting and restraining; attacking with weapons; threatening to harm the partner or the children; stalking and harassment; intimidation; forced sex; berating and belittling; denying access to family assets, etc. This includes:

- a caretaker who is a victim or perpetration of domestic violence involving a partner, former partner or other adult;
- a caretaker who continues to maintain any type of relationship with a abusive/abused adult and domestic violence remains a threat (the presumption would be that domestic violence remains a threat)
- an order of protection is in effect against the abusive adult; or
- A caretaker who is involved in serious conflicts (volatile arguments or physical fighting with other adults within the RAP family unit.

8. Caretaker(s) with alcohol abuse problems with the past two years, with risk of not meeting responsibilities.

Alcohol abuse means regular or periodic use of alcohol, with the risk of not meeting responsibilities or having adverse effects pm daily living (e.g. danger of job loss, financial problems, partner threatens to leave, child care suffers, criminal justice system involvement)

- If the caretaker is participating in a non-professional support group, such as Alcoholics Anonymous (AA), without any other evidence of continuing alcohol use within the last two years, do not consider this by itself, as a current alcohol abuse problem.
- If the caretaker was in treatment more than two years ago, but there is evidence that the person has resumed using alcohol, consider this as a current alcohol abuse problem.

- Respond “Yes” to this Risk Element if the caretaker is currently participating in an alcohol treatment program.
- Respond “No” to this Risk Element if the caretaker had an alcohol problem in the past, but has completed treatment and has remained alcohol-free for at least two years.

9. Caretaker(s) with drug abuse problem within the past two years, with risk of not meeting responsibilities.

Drug abuse means regular or periodic abuse of one or more drugs, with the risk of not meeting responsibilities or having adverse effects on daily living (e.g., danger of job loss, financial problems, partner threatens to leave, child care suffers, criminal justice system involvement).

- If the caretaker is participating in a non-professional support group, such as Narcotics Anonymous (NA), without any other evidence of continuing drug abuse, do not consider this, by itself, as a current alcohol abuse problem.
- If the caretaker was in treatment more than two years ago, but there is evidence that the person has resumed using drugs, consider this as a current drug abuse problem.
- Select “Yes for this Risk Element if the caretaker is currently participating in a drug abuse treatment program.
- Select “No” for this Risk Element if the caretaker had a drug abuse problem in the past, but has completed treatment and has remained substance-free for at least two years.

10. Caretaker(s) has a serious mental health problem

The caretaker should be considered as having a mental health problem if he or she:

- Exhibits symptoms, such as bizarre behavior or delusions, of an undiagnosed mental illness;
- Has recent repeated referrals for mental health evaluation or treatment;
- Has been prescribed medication for an ongoing or recurring serious mental health problem;
- Is currently experiencing depression of an ongoing or recurring nature, or suicidal behavior;
- Has a current diagnosed serious mental illness; or
- Has attempted suicide in the past

11. Caretaker(s) has very limited cognitive skills

Very limited cognitive skills could include mental retardation, brain injury or some type of cognitive disability that limits the caretaker’s ability in major life activities, such as child care, capacity to form positive relationships with others, self-care, self direction, receptive and expressive language, learning, capacity for independent living and economic self-sufficiency.

12. Caretaker(s) has a debilitating physical illness or physical disability

Indicates whether or not the caretaker has a serious physical disability or debilitating

illness that limits his / her ability to perform any major life activities, such as child care, capacity to form positive relationships with family members or others, self-care, self-direction, receptive and expressive language, learning, mobility, capacity for independent activities and economic self-sufficiency.

13. Caretaker(s) has and applies realistic expectation of all the children

“Realistic expectations” is defined as having an understanding of age-appropriate behavior, setting consistent, realistic standards, as well as safe and reasonable limits with appropriate consequences. In addition, the caretaker provides the child (ren) with options, encourages and helps the child (ren) with tasks when needed, and adapts parenting practices to the needs of the child (ren) and circumstances. Select “Yes” for this Risk Element *only* if the caretaker has realistic expectations of *all* of the children.

14. Caretaker(s) always or usually recognizes and attends to the needs of all the children.

Indicates whether the caretaker has a history of recognizing and attending to the daily needs of all the children. This strength would be present if the caretaker:

- Has demonstrated competence in meeting the basic and unique needs of all of the children;
- Is resourceful in making attempts to meet child(ren) needs despite adverse circumstances; and
- Has demonstrated the ability to prioritize the children needs above the caretaker’s.

The word “always” is used because some caretakers always meet the needs of all of their children (i.e., a false report). The word “Usually” is included because some caretakers may meet all of the needs of their children, but may have an isolated or temporary instance of not meeting a child’s needs.

15. Caretaker(s) views the abuse / maltreatment situation as seriously as the caseworker.

This Risk Element refers to whether the caretaker shares the caseworker’s assessment of the seriousness of the child abuse/maltreatment situation.

- If the caretaker views the situation as less serious than the caseworker does, select “No” for this Risk Element.
- If both the caseworker and the caretaker view the situation as not serious (e.g., a patently false report? Or both see the situation as serious, select “Yes.”

The RAP also calculates **Risk Ratings** (both Preliminary and Final), which refer to the probability that a subsequent child abuse/maltreatment report will be received and indicated on the RAP family unit within the next two years.

Case Risk Rating Guidelines

(4) Very High Risk

- Cases assigned a very high-risk rating reflect situations which pose the most dangerous and highest likelihood of future abuse or maltreatment to a child
- It is expected that these cases will be open for services unless clear justification can be provided. Services are essential to decrease identified risk.
- It is likely that most of the family conditions/behaviors match the risk element descriptors on the upper range of the scales, or that one or more of the elements are significant enough to warrant a very high risk rating.
- If the case is to be closed with a “Very High” risk rating without services, CPS must have a conversation with the Child Protective Manager (CPM).

(3) High Risk

- Cases assigned a high-risk rating reflect situations where there is substantial risk of future abuse or maltreatment to a child.
- It is expected that their cases will be open for services unless clear justification can be provided. There is a high likelihood that these cases need services to decrease identified risk.
- It is likely that many of the family conditions/behaviors match the risk element descriptors in the middle to upper range of the scales, or that one or more of the elements are significant enough to warrant a high risk rating.

Note on Engaging Families with High or Very High Risk

Engaging the family so that they will be receptive to services will be an important case consideration regardless of whether the report is “indicated” or “unfounded”. Some unfounded cases will have a level of risk that is high or very high. While CPS does not provide ongoing services to families with only unfounded reports, the CPS worker can refer a family to a service provider within DSS or in the community. Good practice requires CPS to address all of the needs of the family.

(2) Moderate Risk

- Cases assigned a moderate risk rating reflect situations where some risk elements are present at a moderate level of severity. These elements may be off-set by individual, family or community strengths.
- It is likely that the family conditions/behaviors match the risk elements descriptions on the middle to lower range of the scales.
- Some cases may have needs that can appropriately be met through local district or agency services. The family’s needs for risk reduction may be able to be met in some cases through services within the community.

(1) Low Risk

- Cases assigned a low case risk rating reflects situations where the risk of future abuse or maltreatment to a child is low or minimal.
- Few of these cases are likely to have family or child needs which are appropriate for

local district and /or agency services.

- It is likely that most of the family conditions/behaviors match the risk element descriptors on the lower end of the scale. If there are any risk elements rated higher, they are likely to be offset by family or individual strengths.

Final Risk Rating

The Final Risk Rating will be calculated once the caseworker has responded to all of the **Elevated Risk Elements**. If the caseworker later changes the response to any Elevated Risk Element, the Final Risk Rating will recalculate automatically.

Elevated Risk Elements:

In addition to the 15 risk elements, there are 8 additional items described as elevated risk elements. The presence of any of these elevated risk elements automatically raises the risk rating to Very High. The elevated risk elements are not included among the previously mentioned 15 risk elements, because the elevated risk elements refer to events that are relatively rare, and thus would not affect the likelihood of subsequent abuse and neglect in the majority of cases. Although they occur infrequently, the elevated risk elements indicate that there is a heightened risk of serious abuse or neglect in the future. For this reason, a policy decision has been made to override the Final Risk Rating to Very High if any of the elevated risk elements are present.

- If “No” has been selected as the response for every Elevated Risk Element, the Final Risk Rating will be calculated as equal to the Preliminary Risk Rating.
- If “Yes” has been selected as the response for at least one Elevated Risk Element, the **Final Risk Rating** will be calculated as Very High and the following text will display below the **Final Risk Rating** field: “*Elevated Risk Exists.*”

Final Risk Rating Generation

After the elevated risk elements are taken into account, the system generates the Final Risk Rating; either Low, Moderate, High or Very High.

High Risk and Very High Risk Cases

- Services are deemed essential for cases with high or very high risk in order to decrease the risk of subsequent abuse or maltreatment.
- If protective or preventive services are not provided to high or very high risk cases, an explanation or why services are not being provided is required.

Moderate Risk and Low Risk Cases

- Families with moderate or low risk may either have no services needs, or their needs may be appropriately served by other services in the community.
- The RAP does not replace caseworker judgment and there may be valid reasons why a services case should be opened for a family with low or moderate risk.

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CONSULTATIONS

The **Clinical Consultation Team (CCT)**, a partnership between the Division of Family Support Services and DCP, was implemented to support case work decision making through the use of consultant teams with specialized knowledge and skills in four targeted areas: Domestic Violence, Mental Health, Substance Abuse and Medical Concerns (including but not limited to Special Medical Needs cases). Clinical Consultants enhance the assessment of children and families in these areas by assisting with case specific consultation, direct services (CASAC) and offer service referrals to meet the service needs of the family.

Investigative Consultants (ICs) with extensive law enforcement investigative backgrounds are available in each Borough Office to assist in CPS investigations. CPS are to consult with the Investigative Consultants on all cases where there is domestic violence, sexual abuse, physical injury to a child, fatality, missing person, IRT, criminal activity or concerns for criminal activity, HP 13 cases and kinship resource clearances.

CPS should confer with a consultant whenever a family is experiencing one or more of the aforementioned conditions.

Types of Consults:

Link to review Consultants:

- [Consultants](#)
- **Domestic Violence:**
CPS must receive a DV consultation on all cases where a family is experiencing domestic violence, has a history of domestic violence or where domestic violence is suspected. DV consultants are available for assistance in DV screenings, interviews, service assessment and planning, home visits with CPS and attend child safety conferences or family meetings.
- **Mental Health:**
CPS must consult and share information with the mental health consultant on cases where there is a history of mental health, a confirmed mental health diagnosis or suspicion of. CPS can receive assistance in screening, interpretation of medical documentation, assessment and service planning, home visits with CPS, attend child safety conferences or family meetings.
- **Substance Abuse:**
CPS must obtain a consultation with the CASAC for assistance in substance abuse screening interpretation, referral for services or direct services. The CASAC is also available for participation in child safety conferences, family meetings or home visits with CPS.
- **Medical Concerns:**
CPS must confer with medical consultant whenever there are medical concerns or interpretation of medical documents needed. The medical consultant can provide service referrals, make home visits with CPS and attend child safety conferences as needed, or provide relevant information to assist with the family's service needs.

- **Investigative Consultant (ICs):**

CPS must request a case consultation with an Investigative Consultant in the following circumstances:

- Current report alleges child has a suspicious injury and there is a history of prior injury to a child in the home. (Whether indicated or unfounded)
- Current report alleges domestic violence (obtain Domestic Incident Reports and any criminal history)
- Parent/Caretaker is suspected to be involved in illegal activity
- Sexual Abuse cases not assigned to the Hospital/Sexual Abuse Unit
- If CPS is unable to locate the family or child after home visits have been attempted
- Other cases where the Supervisor-II or above deems the advice of the Investigative Consultant could benefit the investigation

Required task:	Assess/describe/document:
1. Refer case for clinical consultation based on information received in the SCR report or information obtained from the Link to review: • Referral for Clinical Consultant *CPS should make the referral for the clinical consult as early in the case as possible or when information is made known.	• Document reasoning for obtaining the clinical consultation • Complete clinical consultation request for and submit for referral • Document outcome of referral • Document follow up and actions completed as a result of the referral outcome
Required Tasks:	Assess/describe/document:
2. Referral to the Investigative Consultant Link to review: • Investigative Consultant Referral Form	• Document reasoning for obtaining the consultation • Complete the Investigative Consultant request form • Document outcome of consultation and any follow-up given ○ Including the interpretation of any criminal background checks and domestic incident reports

CHILD SAFETY CONFERENCES

A **Child Safety Conference (CSC)** is a gathering of family members, their natural supports, including relatives friends, neighbors, etc. who have responsibility and interest in protecting the child and who, together with ACS staff, service providers and community representatives, meet to thoroughly discuss the safety concerns, family strengths and resources in order to make the best safety decision for a child. The group works together to create a safety plan that is tailored to the individual needs of the child and the resources of the family, exploring in-home options if they address the safety concerns, or out- of- home placement, where necessary. The conference takes place in real time and the decision made therein must be effected immediately in the safety interest of the child (ren). While it is not a formal legal process, the conference gives value to the families and children's right to privacy. All participants, other than the family and child, are expected to sign an agreement not to discuss the content of the meeting with others.

Child Safety conferences are crucial tools and triggered by (1) when protective custody is being considered due to concerns for a child's safety, (2) when legal intervention is being considered (3) after an emergency removal has taken place to explore options (4) when a parent/caretaker expresses interest in voluntarily placing the child (4) on behalf of a newborn whose parent/caretaker has a child who is currently placed in ACS custody or if the parent test positive for illegal substances during the 3rd trimester of pregnancy or when there are other indicators that the mother may not be able to care for the child at birth and (5) when a fatality of a child has occurred and there are surviving siblings.

The child safety conference is led by a trained facilitator, who works with the group to consider everyone's perspective and who helps the group to reach the best safety decision. It begins with the brief history of what led to the CSC and then to a discussion of the family's and child's strengths and challenges that serve as a means to developing a recommended safety plan. The group then comes together to finalize the details of the plan. The CSC is typically 1/12 to 2 hours in length.

There are two types of child safety conferences, the initial child safety conference where decisions about safety are made and the follow- up child safety conference where the service plan is reviewed and the family's engagement is assessed.

Child Safety Conferences:

- **Initial Child Safety Conference (ICSC)**
 - Link to review: [Revised ICSC Protocol](#)

The ICSC participants focus on safety concerns, family strengths and available resources. The participants brainstorm to develop and agree upon a final safety plan that is least restrictive and least intrusive for the child. To support the bio-parent in their understanding of the child welfare process and to advocate on their behalf during the conference, Parent Advocates are invited to all ICSC's.

- **Follow- up Child Safety Conference (FCSC)**

The FCSC takes place approximately 20 days after the ICSC. The FCSC brings the groups back together (minus the Parent Advocate) to share information on the implementation of the service plan and on the child's adjustment to foster care if the ICSC decision was placement.

Family Team Meetings:

Link to review:

- [Meeting Guidelines: Finding Solutions with Families](#)

While the Child Safety Conference must be held prior to finalizing any safety plan that involves removal or legal intervention to ensure all alternatives are explored, a Family meeting is convened by the CPS unit as a family- focused intervention in response to an identified risk concern to work in partnership with the family to develop a service plan that addresses the identified risk elements when they do not present immediate or impending danger of serious harm to the child (ren) and court intervention is not being considered. The participants are family members, their supports, service providers and the assigned CPS. The primary focus is to address the identified risks while considering the safety, well-being and permanency of the child (ren).

If an assessment of danger is made at the family team meeting and it appears that a removal may be necessary, the CPS will ask for an immediate CSC.

Types of Family Involved Meetings

Office Meetings: These meetings can occur at any time, with or without a previous appointment, when the family or the CPS feels they need to discuss or clarify a matter for any given reason, or for the CPS to gather additional information needed for the investigation. These meetings usually take place in an ACS Borough Office. This meeting will result in verbal agreements on next steps and should be documented in CNNX.

Home Meetings: These meetings can occur at any time during a home or community based visit for similar reasons and purposes as the Office meetings. Outcomes of this meeting should be documented in CNNX.

Family Meetings: These are meetings that the CPS convenes as a “family-focused intervention”, held in response to an identified risk concern. The participants are family members, their supports, service providers and the assigned CPS. The primary focus is to address the identified risks, while considering the safety, well-being and permanency of the child (ren). This meeting will result in a written safety plan that involves resources and supports and must be documented in CNNX.

Required task:	Assess/describe/document:
1.CPS is to request an Initial Child Safety Conference when: ○ When CPS has determined a removal or legal intervention is necessary to keep a child (ren) safe. ○ 24 hours after an emergency removal has been done (before filing the Article X petition) ○ To determine if voluntary	• Document reasoning for Initial Child Safety Conference and why the subject child (ren) would not be safe with the parent/caretaker • Document reasoning for the emergency removal

placement on a child should be accepted ○ On behalf of a newborn when there are safety concerns about the parent/caretaker Link to review: • Initial Child Safety Conference Policies and Forms • ICSC Brochure in Various Languages • Parent Handbook for Children in Foster Care	
Required Task:	Assess/describe/document:
2. Family Team Meeting Link to review: • Family Meeting Policies and Forms	• Document the reasoning for the family meeting and the type of meeting that occurred. • Document the outcomes of the meeting and any safety planning that was discussed in CNNX.

DETERMINATION

Link to review

- [CNNX Step by Step Guide: Appendix C and J](#)

For each report of suspected child abuse or maltreatment received, the local CPS must determine whether the report is indicated or unfounded within 60 days. This section provides a general understanding of legal definitions of child abuse and maltreatment, and the factors child protective services must consider in determinations. If there is confusion about a legal issue or concept, consult with FCLS.

If the investigation reveals that there is some credible evidence that child abuse or maltreatment exists, the report must be indicated. Some credible evidence is defined as evidence which is worthy or capable of belief. If no credible evidence exists, the report must be unfounded and sealed.

Note: Although the language for report determination is “indicated” or “unfounded”, the language for allegation determination is “substantiated” or “unsubstantiated”.

Each allegation of abuse or maltreatment must be specifically addressed in the case record, and determined to be substantiated or unsubstantiated in reference to the role of the subject parent, other person legally responsible for the child and for each child. **Note: Denials of an allegation DOES NOT warrant unsubstantiation.** Further investigation is always required to make a determination.

Any new evidence or allegations of abuse or maltreatment revealed during the investigation also must be added, addressed and determined.

If the investigation reveals that some credible evidence of child abuse or maltreatment exists, the report must be indicated (even if the concerns have been remedied prior to the closure of the investigation). If no credible evidence exists, the report must be unfounded. An unfounded case is available to CPS when investigating a subsequent report of a child abuse or maltreatment involving a subject of the prior unfounded report, or a child named in the unfounded report, or a child’s sibling named in the unfounded report. CPS may not indicate a subsequent report based solely on the existence of a prior unfounded report(s).

In fatality investigations, CPS must check the *Fatality-No Surviving Children* in the check box on the CPS Investigation Conclusion window to confirm that there are no surviving children in the stage. This check box automatically appears if the investigation finds all of the following: a DOA/Fatality allegation, a date of death for the subject child associated with the fatality, and no other persons younger than 18 years with a role of maltreatment, abuse or no role.

Statutory Definitions to Guide Determinations

CPS must gather sufficient information within 60 days from receipt of a report to assess the child's and family's circumstances and the evidence to make a decision to "indicate" or "unfound" a report of suspected child abuse or maltreatment. This section provides legal definitions of child abuse and maltreatment, and the factors child protective services must consider in determinations. If there is confusion about a legal issue or concept, FCLS should be consulted. The following legal definitions and considerations from the **Family Court Act and Social Service Law** should guide determinations to indicate or unfound a report. Using the evidentiary standard, the CPS addresses whether a subject parent through acts of omission or commission has caused abuse or maltreatment of a child, as defined by the statute (below).

Parents and Other Persons Legally Responsible (FCA Section 1012(g))

New York State's child abuse and maltreatment definitions apply to acts or omissions of parents and other "persons legally responsible." Persons legally responsible include the child's custodian, guardian, and any other person responsible for the child's care at the relevant time. The term custodian includes any person continually or at regular intervals found in the same household as the child when the conduct of such person causes or contributes to the abuse or neglect of the child.

Abused Child (FCA Section 1012(e); SSL Section 412(1))

An abused child means a child less than 18 years of age (youth in residential care with certain handicapping conditions are included up to age 21 years), who, through the conduct of the parent or person legally responsible for the child's care, experiences:

- Serious injury inflicted or allowed to be inflicted upon the child by other than accidental means. Serious injury includes a disfigurement or impairment that is extensive and lasting, or loss or impairment of a function of a bodily organ, or other impairment of physical or emotional health that is extended in duration or an injury that causes or creates a substantial risk of death.
- Danger of serious injury that is created or allowed to be created, by other than accidental means. This includes a substantial risk of physical injury to the child which would be likely to cause death, or serious or extended disfigurement, impairment or loss of the function of a bodily organ; or
- Sex Offense – Parent committed or allows to be committed a sex offense against a child. This includes sex felonies and misdemeanors such as rape, sodomy, sexual molestation, and other acts that allow, permit or encourage the child to engage in prostitution or pornographic sexual performances.

Factors to Consider In Determining Whether a Child Is Abused

In the Investigation Conclusion Narrative, CPS must address each allegation for each identified child, allegations that are part of the original report, and those that arose during the investigation, and distinguish the role of each of the subject parents or other person legally responsible. To determine whether the child is abused, CPS must evaluate all the facts gathered, assess the total

circumstances, and determine whether there is “some credible evidence,” that is, evidence worthy or capable of belief, to find that the child is abused, based on the following factors:

- Assess the severity of the injury or offense against the child or the danger or substantial risk of serious injury and its impact on the child’s physical, mental and emotional condition;
- Assess whether the serious injury or danger of serious injury occurred accidentally;
- Assess the degree of each parent or person legally responsible participated in the non-accidental serious injury, danger of serious injury or sex offense against the child;
- Assess whether the parent or person legally responsible created or allowed circumstances to be created that resulted in serious injury or danger of serious injury or a sex offense against the child;
- Determine whether there is a causal connection between the parent’s or other person legally responsible, acts or omissions and the circumstances that allegedly produced the serious injury, danger of serious injury, or sex offense against the child;
- Upon identification of a person who is responsible for the child’s serious injury, danger of serious injury, or sex offense, CPS must clearly document that the person is the offender and take steps to hold the person accountable through criminal court or family court intervention and an individual service and treatment plan.

Maltreated Child

Child maltreatment encompasses the legal definitions of **Abandoned Child, Maltreated Child and Neglected Child.**

- **Abandoned Child** The subject parent or other person legally responsible for the care of the child demonstrates intent to forego his or her parental rights and obligations as manifested by his or her failure to visit or communicate with the child less than 18 years although able to do so.

It is an affirmative defense to a criminal prosecution for abandonment of a child under Penal Law Section 260.00, that the parent, guardian, or other legally responsible person who intended to abandon a child who is not more than five days old, providing that the parent, guardian, or other legally responsible person: intended that the child be safe from physical injury; and cared for in an appropriate manner; left the child with an appropriate person or in a suitable location; and promptly notified an appropriate person of the child's location. Note: This is not a bar to appropriate child protective action, including family court proceedings.

- **Maltreated Child (SSL Section 412(2))**
A maltreated child means a child less than 18 years of age (youth in residential care with certain handicapping conditions are included up to age 21 years) who has had serious physical injury inflicted upon him or her by other than accidental means or whom the Family Court defines as a neglected child.
 - **Neglected Child (FCA Section 1012 (f))**
Neglected child means a child less than 18 years of age whose physical, mental, or emotional condition has been impaired or is in imminent danger

of becoming impaired as a result of the failure of his or her parent or other person legally responsible to exercise a minimum degree of care. Failure to exercise a minimum degree of care concerns the basic duty of parents or caretakers to take reasonable action in regard to the following areas:

- In supplying adequate food, clothing, shelter, education, medical, dental, optometrist, or surgical care, though financially able to do so or offered financial or other reasonable means to do so; or
- In providing proper supervision and guardianship; or
- By unreasonably inflicting or allowing to be inflicted harm or creating a substantial risk of harm, including the infliction of excessive corporal punishment; or
- By misusing a drug or drugs or alcoholic beverages to the extent self-control of actions is lost. Where the parent or other caretaker is voluntarily and regularly participating in a rehabilitation program, evidence of loss of self-control through drugs or alcohol shall not establish that the child is a neglected child in the absence of evidence establishing that the child's physical, mental or emotional condition has been impaired or is in imminent danger of becoming impaired because of the caregiver's failure to exercise a MINIMUM DEGREE OF CARE.

Factors to consider in determining whether a child is neglected:

In the Investigation Conclusion Narrative, CPS must address each allegation for each identified child, allegations that are part of the original report, and those that arose during the investigation, and distinguish the role of each of the subject parents or other person legally responsible. To determine whether the child is neglected, CPS must document a link or Causal Connection between the child's impaired condition, or imminent danger of impairment, the parent's or other person legally responsible acts or omissions, by evaluating all the facts gathered, assessing the total circumstances, and determining whether there is "some credible evidence," that is, evidence worthy or capable of belief, to find that the child is neglected. These factors must be considered:

- **Actual Impairment or Imminent Danger of Impairment of a Child's Physical, Emotional or Mental Condition**

The focus must be on serious harm or potential harm to the child, in view of their age and developmental stage, not just what might be deemed undesirable parental behavior; however, a finding of neglect may be appropriate even when a child has not been actually harmed but is in imminent danger of impairment. Imminent danger of impairment must be near or impending, not merely possible.

Impairment of a child's emotional health or impairment of mental or emotional condition means states of substantially diminished psychological or intellectual functioning in relation to, but not limited to, such factors as:

- Failure to thrive

- Control of aggressive or self-destructive impulses
 - Ability to think and reason
 - Acting out or misbehavior, including incorrigibility, ungovernability or habitual truancy;
- Impairment of the child’s condition or imminent danger of impairment must be clearly attributable to the unwillingness or inability of the parent or other person legally responsible to exercise a minimum degree of care toward the child; a causal link or connection must be established; and
 - Actual Failure by Parent to Exercise a Minimum Degree of Care: A minimum degree of care is a baseline of proper care for children that all parents, regardless of lifestyle or socio-economic status must meet. The law requires an objective evaluation of the parent or caretaker as follows:
 - Would a reasonable and prudent parent or caretaker have acted, or failed to act under the circumstances then and there existing.
 - Was the “minimum” degree of care “good enough,” that is **not** best or ideal, but adequate, basic
 - Was the failure actual, not threatened
 - What was the impact on the child, in view of the child’s age and developmental stage and special needs, if any

Required task:	Assess/describe/document:
1. Considering all of the information gathered from the investigation, address each allegation and apply information collected against the statutory definitions for: ❖ Person legally responsible ❖ Child abuse ❖ Child maltreatment ❖ Child neglect.	● Whether each allegation is substantiated or unsubstantiated and the findings that led to each determination ● Whether source’s, children’s, parent’s, alleged subject’s, collateral and all other accounts corroborate information or whether there are discrepancies ● Whether the determination is consistent with the investigative findings, as documented in the progress notes and other information sources, and the statutory definitions and standards ● Justification for the determination as follows: ● Analyze the collected facts concerning each incident or occurrence associated with the allegations on the original report, the consolidated report, or any other allegations that arose during the course of investigation

	• Distinguish the role of each of the subject parents or other persons legally responsible for the child in the incidents or occurrences associated with the allegations of child abuse and maltreatment on the original report, a consolidated investigation, as well as any other allegations, which arose during the course of the investigation; • Analyze the collected facts concerning the physical, mental and emotional condition of all children in the household, in view of their age and developmental stage; • Analyze the collected facts regarding whether the physical, mental or emotional condition of the child was harmed or in imminent danger of harm; • Analyze the collected facts to determine whether there is a causal connection between the harm or imminent danger of harm to the child and the subject parent's or other legally responsible person's acts, omissions, or role in the incident or occurrence related to the allegations under investigation • Whether source's account, children, parent's, alleged subject's, collateral and all other accounts corroborate
Required task:	Assess/describe/document:
2. Determine whether allegations involving child abuse such as serious physical injury or substantial risk of serious physical injury by other than accidental means, or certain sex offenses are substantiated, and the findings and reasons the allegation is substantiated.	All elements listed below must be present to substantiate an allegation of child abuse. • Assess the severity of the injury or offense against the child or substantial risk of serious injury and its impact on the child's physical, mental and emotional condition. • Whether the serious injury or substantial risk of serious injury occurred accidentally; and the degree each parent did, or did not participate, in the injury, or the substantial risk or

	serious injury, or sex offense against the child, or, the degree the parent created or allowed circumstances to be created that resulted in serious injury or substantial risk of serious injury or a sex offense against the child. • Whether there is a causal connection between a parent's acts or omissions and the circumstances that allegedly produced the serious injury, substantial risk of serious injury, or sex offense against the child. • Upon identification of a person who is responsible for the child's serious injury or, substantial risk of serious injury, or sex offense, CPS must document that the parent or other person is the offender and take steps to hold the person accountable through criminal court or family court intervention and an individual safety and service plans.
Required task:	Assess/describe/document:
3. Determine whether each of the allegations that the child was maltreated or neglected such as educational neglect, emotional neglect, excessive corporal punishment, inadequate food, clothing shelter, inappropriate custodial conduct, inadequate guardianship, inappropriate isolation/restraint, lack of medical care, lack of supervision, parent's drug/alcohol misuse are substantiated, and the findings and reasons the allegations are substantiated In many reports of educational neglect, the child will return to school once ACS begins its investigation. The return of the child to school does not necessarily mean that the allegation is to be unsubstantiated. Every allegation must be fully investigated and the determination based on the facts present in the case. If the investigation reveals that some credible evidence of child abuse or maltreatment exists, the report must be indicated. It must be determined	All elements listed below must be present to substantiate an allegation of child maltreatment or neglect. • Whether there is actual impairment or imminent danger of impairment of a child's physical mental or emotional condition of the child. Impairment of a child's emotional condition means a state of substantially diminished psychological functioning in relation to, but not limited to, such factors as failure to thrive, control of aggressive or self-destructive impulses; ability to think of reason; acting out misbehavior. ○ Impairment of the child's condition or imminent danger of impairment must be clearly attributable to the unwillingness or inability of the parent to exercise a minimum degree of care toward the child. Immediate danger must be near or

that the child's unexcused absences from school are the result of the actions of the subject of the report or the failure to act by the subject of the report.	impending, not merely possible. ○ Whether there is an actual failure by the parent to exercise a minimum degree of proper care for the child based on an objective assessment regardless of lifestyle or social economic condition. ○ Whether a causal connection exists between the actual harm and imminent danger of harm to the child and the parent's acts, omissions, or failure to exercise a minimum degree of care.
Required task:	Assess/describe/document:
4. The CPS determines, within 60 days of receipt of the report, whether there is "some credible evidence" abuse or maltreatment by each of the parents or other persons who are subject of the report, and makes a determination of "indicated" or "unfounded."	• Whether the determination is consistent with the investigative findings and safety assessments • Whether the justification for the determination is stated • ALL allegations must be addressed in the documentation
Required task:	Assess/describe/document:
5. If the report is indicated, ALL subjects and any other persons named in the report must receive written notification of the determination ("Notification of Determination") as well as of the subjects' rights to amendment and to a fair hearing if such an amendment is not granted. The notice must be given by CPS within seven days of the indicated determination.	• Who was provided notification and date that notification was provided
Required task:	Assess/describe/document:
6. If the report is unfounded, the CPS should notify any individual, unit or organization or agency which received a copy of the report of this determination. CPS may notify the mandated reporting source of the report outcome if report was unfounded.	• Who was notified of the determination, how and when.

CONNECTIONS Documentation Requirement:

Complete the Investigation Conclusion Narrative section in CONNECTIONS. The Investigative Narrative must address each allegation for each child and subject in the report. Using the standard of "Some Credible Evidence", explain how the evidence complies with the

Elements of Abuse and Maltreatment definitions. Each element in the definition must be satisfied. Evidence must also comply with jurisdictional requirements and who can be considered a subject of a report. If the evidence does not support the definitions, then the report must be unfounded.

Submit determination to Supervisor for approval by **Day 50**. High Priority cases may require a higher level of approval.

If you have not completed your investigation within 60 days, do not to continue the investigation off line or in progress notes. Discuss with your supervisor the obstacles preventing case closure. Develop and document your plans to bring the investigation to a close.

Course of Action:

- If the report is indicated and does not warrant a protective removal, provide appropriate services/refer family to preventive services (refer to Services and Risk Assessment sections)
- If the report is unfounded and the family requests services, refer family to preventive services
- If the report is unfounded and no services are requested or needed, close the case (see Closing Case section)
- If a report of child abuse and neglect is suspected to be false and knowingly made, report the case to a supervisor and the Criminal Justice Coordinator.

Guidelines for Case Closing

Once a CPS case has been opened for an Investigation and/or Services, it is important to work towards the time when the case can be appropriately closed. The following list of general guidelines for case closing may be useful in conjunction with your specific evaluation of a particular CPS service case.

Consider closing a CPS service case when:

- Based on currently available information, there are no children likely to be in immediate danger of serious harm and there are no controlling safety interventions required or in place at this time.
- The Final Risk Rating (RAP) or the Risk Rating (risk scale) has reached an acceptable level. Cases determined to be at “Low Risk” are unlikely to have children abused or maltreated in the future and generally can be closed. Families with “Moderate Risk” may either have limited service needs, or their needs may appropriately be served by other services in the community and therefore the case may be able to be closed. Cases with “High Risk” or “Very High Risk” are considerably more likely to have children abused or maltreatment in the future and therefore services are deemed essential to decrease identified risk. If CPS is closing a case with “High Risk” or “Very High Risk” an explanation of why services are not being provided is required at the investigation conclusion.
- The service plan review shows an acceptable level of outcome achievement for the most significant identified problems.

Then additionally consider:

- Whether the family’s progress has been consistent over a long enough period of time.
- Whether improvements can likely be maintained despite the withdrawal of services.

Whether additional or on-going services could further reduce risk, whether these services are available, and whether the family has a reasonably strong ability to benefit if services were maintained or provided.

CPS must use the correct Closure Reason Choice when Progressing an INV Stage either off CNNX or to an FSS stage.

Expanded Definitions of Investigation Conclusion Reasons

Unfounded; Case Open- Services

The worker has assessed that no serious safety factors or risk issues currently exist which warrant or require the provisions of child protective services. However, preventive services are warranted and have been accepted by the family. Additionally, court ordered preventive services, court ordered preventive services or voluntary placement may exist.

Unfounded: Closed, No Services Required

The worker has assessed that no serious safety factors or risk issues currently exist which warrant or require the provisions of child protective services. Additionally, none of the following services are required or provided:

- Preventive Services

- Court Ordered Services (FSU)
- Court Ordered Supervision
- Court-Ordered Placement
- Voluntary Placement

Unfounded; Closed – Refused Services

The worker has assessed that no serious safety factors or risk issues currently exist which warrant or require the provisions of child protective services. Preventive Services were offered to the family and refused and there is not sufficient evidence to initiate or continue a Family Court action to compel involvement.

Unfounded; Closed – Unable to contact/moved out of jurisdiction

The worker has assessed that no serious safety factors or risk issues currently exist which warrant or require the provisions of child protective services and services cannot be provided, due to one or more of the following circumstances:

- The current whereabouts of the family are unknown and the family cannot be located.
- The family has moved out of the current CPS jurisdiction and cannot be located.
- The family has moved out of New York State.

Unfounded; Closed – Referred to Community Based Services Only

The worker has assessed that no serious safety factors or risk issues currently exist which warrant or require the provisions of child protective services. The family has been referred to community based services only. No agency direct or purchase provided services or court ordered services are required or being provided at this time.

Indicated; Case Open- CPS required

The worker has assessed that serious safety factors and/or risk issues exist which require the on-going assessment of safety and risk and/or court ordered CPS placement, court ordered services or supervision currently exist. In addition; preventive services, voluntary placement or non-LDSS custody-relative/resource placement may be requested and provided to the family and/or a PINS/JD placement may exist.

Indicated; Case open- CPS not required

Current serious safety factors or risk issues may exist, however, do not place the child(ren) in immediate danger. The family has been offered and subsequently refused appropriate services. Additionally, the CPS worker has assessed that:

- It would not be in the child's best interest or that there is insufficient evidence to initiate or continue a Family Court action to compel involvement
- OR-
- CPS sought a court order to compel the subject(s) of an indicated abuse and/or maltreatment report(s) to receive such services, but the court has dismissed the petition and it is not in the child(s) best interest to continue additional Family Court action.

Indicated; Closed – No services required

The worker has assessed that no serious safety factors and/or risk issues exist that require the on-going assessment of safety and risk. Additionally, none of the following services are required or being provided.

- Preventive Services
- Court Ordered Services (FSU)
- Court Ordered Supervision
- Court-Ordered Placement
- Voluntary Placement

Indicated; Closed – Unable to contact/move out of jurisdiction.

Serious safety factors and/or risk issues exist that require the on-going assessment of safety and risk. However, services cannot be provided, as one of the following circumstances:

- The current whereabouts of the family are unknown and the family cannot be located.
- The family has moved out of the current CPS jurisdiction and cannot be located.
- The family has moved out of New York State.

Indicated; Closed – No Surviving children

All of the following circumstances must exist:

- A DOA/Fatality allegation exists and is substantiated
- A DOD was entered for the AB child(ren) associated to the DOPA/Fatality allegation
- There are no other persons younger than 18 years of age with a role of MA, AB, or No Role in the case composition.

Indicated; Closed – Referred to Community Based Services Only

The worker has assessed that no serious safety factors and /or risk factors exist that require the on-going assessment of the safety and risk. The family has been referred to community-based services only. No agency direct or purchase provided services or court ordered services are required or being provided at this time.

FILING ARTICLE 10 PETITIONS IN COURT & TAKING CHILDREN INTO PROTECTIVE CUSTODY

Article 10 of the Family Court Act states; that court action is warranted when the state, through its Family Court, may intervene against the wishes of the parent(s) to ensure that a child's needs are properly met. Court action may be necessary to assist CPS in ensuring that all reasonable efforts are made to protect children while maintaining children in their homes, and preventing foster care placement. Court-ordered interventions include orders authorizing services, or directing a parent to cooperate with services, requiring an offending parent to stay away from the child, or other family members, and abstain from conduct causing harm or threatened harm to the child. When a child's life or health is in imminent danger, and safety interventions are not sufficient to maintain the child at home, CPS is required to obtain a court order prior to taking the child into protective custody, if there is enough time to apply for a court order without compromising the safety of the child.

Prior to the Article X petition being filed, as mentioned in the earlier section Conferences, the CPS will hold an Initial Child Safety Conference, the outcome of this conference will have an impact on what is requested of the court's consideration in the petition filed (i.e. continued remand, vs. court ordered services/monitoring). Also prior to the Article X petition being filed, the CPS can apply for the following types of court orders:

Links to review:

- [Communication/Collaboration during CPS Investigation and Family Court Proceedings](#)
- [Guidelines for Working with Attorneys Representing Parents and Children](#)
- [Family Court Legal Services Policies](#)
- [Family Court Descriptions \(1027/1028/Fact Finding/Disposition/ROS, etc\)](#)

Pre-Petition Order to Produce

CPS can request an order that: the parent or other persons legally responsible for the child or children produce the child or children at a particular location which may include a child advocacy center, or to a particular person for an interview of the child or children, and for observation of the condition of the child, outside of the presence of the parent or other person responsible.

Before CPS can make this application, he/she must have a "reasonable cause to suspect that a child or children's life or health may be in danger". This belief must be based on:

- an SCR report as well as any additional information learned by CPS during the investigation; **and**
- the fact that the CPS has been unable to locate the child named in the report or any other children in the household; **or**
- the fact that the CPS has been denied access to the child or children in the household sufficient to determine their safety; **and** the fact that CPS has advised the parent or other

persons legally responsible that ACS may consider seeking an immediate court order to gain access to the child or children without further notice to the parent or other persons legally responsible.

Pre-Petition Order of Entry

As opposed to the “reasonable cause to suspect that a child or children's life or health may be in danger” standard for an order to produce, the entry order application still requires that there is “probable cause to believe that an abused or neglected child may be found on the premises.” The law requires that an application for an order of entry be based upon three criteria:

- an SCR report as well as any additional information that we have learned in the investigation;
and
- the fact that the CPS has been denied access to the home of the child or children in order to evaluate the home environment; *and*
- the fact that the CPS has advised the parent or other person legally responsible for the child or children that ACS may consider seeking an immediate court order to gain access to the home environment without further notice to the parent or other person legally responsible.

The judge must specify in the order which action may be taken and by whom (i.e., what action(s) to be taken by ACS, and what action(s) by NYPD). CPS’ ability to obtain this order will not be automatic and the search warrant provisions of the New York Criminal Procedure Law will apply.

For each of these orders, the court application will be made without the parents’ having any right to come to court to be heard and to argue against the granting of the order. The judge has the discretion whether to make such an order. In deciding whether to issue either of these orders, the judge must assess which actions are necessary in light of the child or children's safety, and order only such action(s) that are the least intrusive to the family. Further, the judge must consider all relevant information, including but not limited to:

- the nature and seriousness of the allegations made in the report;
- the age and vulnerability of the child or children;
- the potential harm to the child or children if a full investigation is not completed;
- the relationship of the source of the report to the family, including the source's ability to observe that which has been alleged; and
- the child protective or criminal history, if any, of the family and any other relevant information that the investigation has already obtained.

Link to review:

- [Warrants and Entry Orders](#)

Removals

CPS must take all necessary measures to protect a child’s life or health, including when appropriate, the provision of emergency medical services, in-home services and other safety interventions to ensure the health and safety of the child, and prevent or eliminate the need for foster care.

CPS should consider a broad range of safety-oriented interventions to keep children safely at home and out of foster care. **The law requires that reasonable efforts be made to prevent or eliminate the need for foster care placements, where appropriate.** If it is concluded that the child is unsafe at his/her present location then the CPS must take immediate action to protect the child.

CPS must be proactively expedient in notifying the Child and Family Specialist of the need for an Initial Child Safety Conference (ICSC) when conditions in the home indicate that the level of danger to the child (ren) has increased, and a conference is needed to help decide if a protective removal is necessary or if an alternative safety plan can be developed. The purpose of the Child Safety Conference is to make the best safety decision for the child through in-home options if such can fully address the safety concern or out of home placement if necessary.

Court-Ordered Removal

Where temporary removal of a child from home is necessary to avoid imminent danger to the child's life or health, and the parent is absent and if there is not enough time to file an Article 10 petition alleging abuse and neglect and for the court to hold a preliminary (1027) hearing but there is sufficient time to seek a court order prior to the removal without compromising the safety of the child CPS may seek an order from the court authorizing the removal. Inform the parent of the intent to apply for an order of removal. The CPS and supervisor must then make contact with FCLS to obtain a court order.

Examples of factors that may allow sufficient time to obtain a court order prior to removal:

- The child is temporarily safe because he or she is already staying with an appropriate resource for a time sufficient to allow ACS to obtain a court order.
- The child is temporarily safe because he or she is hospitalized or is in another safe location for a time sufficient to allow ACS to obtain a court order.
- The child is temporarily safe because the perpetrator of the harm or risk of harm to the child is in the custody of law enforcement authorities and has not access to the child for a time sufficient to allow ACS to obtain a court order.

Emergency Removal

CPS is authorized to conduct an **emergency removal** without parental consent or court order **only** in circumstances of impending or imminent danger to the child's life or health, when there is not enough time to apply for a court order. In this case, an Article 10 petition **must** be filed as soon as possible, but **no later than the next court day**. CPS is required to give parents/caretakers, schools, hospitals or other facilities from which children are removed, written notice. Parents must be informed of their right to apply to the Family Court for the return of the child, their right to legal counsel, as well as information on how to contact CPS and the agency to which the child will be taken. **(Form DCP- 005 – Notice of Temporary Removal of Child/Notice of Right to a Hearing or Form 701C - Notice of Removal from a School or Hospital)**

The information regarding these mandates must be documented in CNNX. The progress note must document whether the filing of a pre-removal petition was considered and/or the reasons a removal was necessary prior to filing for a court order, whether the removal order was obtained, and, if not, the reasons why the order was not sought or was not granted by the court.

If a child has been taken into protective custody by CPS, the police or other authorized person, CPS must determine whether the child can be returned home once the emergency circumstance is resolved. CPS must make an immediate referral to the Child and Family Specialist Unit requesting an Initial Child Safety Conference (ICSC) to be convened.

In cases of child neglect, CPS does not need prior approval of the court to return children in protective custody to their parents or to persons designated by the child's parents, following resolution of the emergency. In cases of child abuse, CPS cannot return the child without prior court approval.

Link to review:

- [Conducting Emergency Removals](#)

Custody Petitions: Only in Exceptional Circumstances

A relative or family friend's willingness to file for custody of a child who is the subject of a child protective investigation **should not be the determining factor for CPS or FCLS when they are deciding whether to file an abuse or neglect petition in Family Court.** Encouraging family members or friends to file a custody petition instead of filing a neglect petition is an appropriate option for CPS staff to recommend **only in exceptional circumstances.** CPS staff may not recommend that a relative or family friend file for custody during the course of a child protective investigation without obtaining the express approval of the Child Protective Manager.

Link to Review:

- [Child Safety Alert# 9: Foster Care Placement: Taking Care Relative Custody is Recommended](#)

Filing the Article Ten Petition

Once CPS determines that a child is in need of protective custody prior to obtaining a court order, the law requires CPS to file an Article 10 petition as soon as possible but by no later than the next court day. The fact that the child is in protective custody at a hospital or other non-foster care placement is not a basis to delay filing the Article 10 petition or the referral to the Child and Family Specialist, who will evaluate the child's placement and service needs, and convene the Initial Child Safety Conference. Additionally, ACS cannot request or authorize a hospital to hold onto a child once there is no longer a medical necessity for hospitalization.

The court must hold a hearing as soon as possible to determine whether the child's interests require protection pending a final order of disposition. A hearing need not be held if a child taken into protective custody for neglect has been returned to the family. However, Children's Services or the Law Guardian may apply for such a hearing, or the Court may order such a hearing any time after the petition is filed.

Links to review:

- [Child Safety Alert# 34 Submission of Court Reports](#)
- [Criminal Record Information from eJusticeNY](#)

Legal Consultation

DCP may seek a consultation as needed with FCLS to ask basic, focused, case related legal questions. **DCP may seek a consultation any time prior to the commencement of a legal proceeding. After commencing a legal proceeding, DCP should confer with the assigned FCLS Attorney for guidance on legal issues.**

Topic discussed in consultation may include:

- Legal status of a father
- Legal status of a custodian or guardian
- Obtaining information related to a newborn or newly discovered child whose siblings are in foster care or under an order of supervision from the Family Court
- Clarification of whether a particular person on a case is a “person legally responsible” (PLR)
- Res Ipsa Physical Abuse Cases: cases in which a child is injured and the parents fail to provide explanation or the explanation is not consistent with the injuries or there is a question as to whether the injuries were accidental or inflicted.
- Jurisdictional or venue- related questions: e.g. where a petition should be filed if parents or children do not reside in New York City
- Clarification of whether a particular child qualifies as “destitute”
- Clarification of whether a voluntary placement can be considered for a particular child
- What action to take when children or parents are missing

REMOVAL FLOWCHART

The Constitution of the United States of America and the laws of the State of New York prohibit the interference with parental rights absent the consent of the parents or a court order except in exigent circumstances. When considering whether it is appropriate to remove a child from his or her home, a child protective agency must employ the following analysis:

Is There Imminent Danger To the Life or Health of the Child?
(If Not, No Removal of Any Kind Can Be Made)



Can Reasonable Efforts Be Made
To Prevent or Eliminate the Need for Removal?
(E.g.: provision of services, removal of perpetrator from the home,
alternative living arrangements, additional safety measures)



If Not, Is there Time to File A Petition and Hold a 1027 Hearing?
(Notice to Parent Required)



If Not, Is There Time to Make an Application for a Pre-Petition Removal Order?
FCA 1022
(Notice to Parent Required Pursuant to 1023)



If Not, Is This an Emergency?
If so, And There Is No Time for Any of the Above, Emergency Removal Can Be Made.
(Petition To Be Filed Forthwith)
FCA 1024

(Note: Children's Services must keep in mind that, in every case, the court must determine whether a removal, even if justified, would be in the best interests of the child, balancing the protective need against any trauma the child might suffer as a consequence of the removal. See NY Court of Appeals decision in Nicholson.)

Courtesy: Chris Guardo, Family Court Legal Services

Required task:	Assess/describe/document:
1. Consider the broad range of safety-oriented interventions. Seek review and consultation from an available supervisor/manager, CCT, CFS, and the Family Court Supervising Attorney regarding the safety assessment and response and determine whether removal is the only safety intervention possible for the child. Consider whether there is enough time to obtain a court order without compromising the safety of the child.	• Whether in-home forms of safety interventions are sufficient to ensure the safety of the children • Whether an Order of Protection or other court assistance would be sufficient to ensure the safety of the child. • Whether all appropriate reasonable efforts to avoid removal have been made or considered • Consult with Supervisor/CPM • Contact Clinical Consultants • Contact CFS to convene an Initial Child Safety Conference • Contact Supervising Attorney.
Required Task:	Assess/describe/document:
2. If there is reasonable cause to believe that the circumstances or the condition of the child are such that continuing in his/her place of residence or in the care and custody of the parent/caretaker presents an immediate danger to the child's life or health, consult with the supervisor and CPM to review the removal decision and obtain their approval, and determination regarding preliminary or pre-removal court orders, when it is possible to do so without compromising the safety of the child. For Emergency or Court Ordered removals, obtain police assistance and initiate Instant Response as appropriate. Notify the Family Court Attorney within the next business day.	• Complete W-865D, CPS referral to court. FCLS accepts referrals for Article 10 petitions between 9 AM and 10 AM each day that the Family Court is in session. After 10 AM, the Child Protective Manager, must contact the borough supervising attorney or assistant supervising attorney. CPS should notify FCLS as soon as it is determined that a removal is deemed necessary so that it can be determined if it would be feasible to file an Article Ten petition or pre-removal application before 5 PM. If removal is made without a court order, an Article Ten petition must be filed as soon as possible but not later than the next court day. • In cases where the child has been removed, the abuse or neglect petition must state the date and time of the removal and the reasons or circumstances why the child was removed, why there was not sufficient time to obtain a court order, and reasonable efforts to return the child home safely, or the justification for lack of such efforts. • All FCLS contacts and court activity date of remand hearing, intake hearing, fact-finding hearing and results of these hearings.
Required Task:	Assess/describe/document:
3. During the removal, all possible efforts should be made to engage the parent and the children in identifying any possible kinship resources for placement. (See: <i>Trauma Reduction Guidelines</i>). Kinship resources are	• Name and contact information of kinship resources. Efforts made to place children with relatives or people with a close relationship with the family or children.

not limited to relatives. They can be any person with whom the family or child has a significant relationship. Attempt to obtain information on as many potential resources as possible. ◦ Kinship Placements ◦ Emergency Home Studies for Youth Placed in Foster Care as a Result of Voluntary Placement Agreements, PINS Cases and JD Cases ◦ Recent Statutory Changes to Grandparent's Rights ◦ Seeking Extended Family and Other Caring Adults with Positive Connections to the Child or Youth ◦ Certification/Approval of Foster Boarding Homes: Procedure 105 ◦ Approval/Certification of Kinship Foster Homes: Questions and Answers Memo	
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Required Task:	Assess/describe/document:
4. Upon the removal of the child, the CPS must provide the parent with the Notice of Temporary Removal and Right to a Hearing, Form (DCP 005), and resource materials that will aid in their understanding the removal and placement into foster care process. Complete and file an affidavit of service of notice with the court. Link to obtain: • Notice of Temporary Removal • Parent Handbook for Children in Foster Care The parent(s) should arrive at the Intake Part no later than 2 p.m., at which time a lawyer will be assigned and an initial hearing will be held. Contact parent as soon as child is placed to provide the name, location and phone number of the foster care agency. Also provide information on the parent to parent meeting and first visit.	• Whether notice was provided to the parent • Whether parent/caretaker were contacted with placement and visitation information • Date and time of the 20 day Child Safety Conference (CSC) • Date and time of the Parent to Parent meeting

Required Tasks:	Assess/describe/document:
6. All children taken into protective custody should be medically cleared by a nurse prior to placement at the field office or at the Children's Center. However, children requiring immediate medical attention must be taken directly to the hospital (Refer to procedure for providing medical consent). If circumstances warrant (especially in sex abuse situations where there is no medical emergency), CPS should take children to Child Advocacy Centers (CAC).	• Where the children were medically cleared and name of physician/nurse who cleared the child • Results of the medical clearance and any medical treatment or medication needed or provided and who provided consent for the treatment. • What circumstances led to the decision to take the child to a CAC. • What took place at the CAC and a summary of the interview and/or examination.

Course of Action:

- ✓ Make all reasonable efforts to ensure the safety of the child (ren) while maintaining child (ren) safely in their homes, preventing foster care placement.
- ✓ Obtain parental consent or a court order prior to removing a child, when it is possible to do so without compromising the health or safety of the child.
- ✓ Obtain parental consent or a court order prior to an invasive medical examination or treatment, when it is possible to do so without compromising the health or safety of the child.
- ✓ Make all possible efforts to reduce the trauma children experience during a removal.
- ✓ If you have any questions or concerns, consult with your Borough Leadership.

Services

Service referrals can support and rehabilitate children and families for the purpose of averting placement of children into foster care. Services can also enable children placed in foster care to reunite with their parents/legal guardians or primary discharge resource and also reduce the likelihood that a child (ren) could re-enter foster.

SERVICES

CPS are to assess the level of safety and risk to a child (ren) throughout the life of the case. In every case, whether indicated or unfounded, CPS must consider a plan for services. The decision on which service referral to make will be made based upon the safety/risk concerns for each family, taking into account the prior history of child welfare involvement.

Low/Moderate Risk

When a families functioning does not pose any immediate/impending safety/risk concerns to the child (ren) CPS can make an assessment to refer a family to a Community Based Organization and/or a Purchased Preventive Service (PPRS).

Moderate/High/Very High Risk

When there are concerns that the likelihood a child will continue to be maltreated due to risk concerns, CPS must make an assessment for Preventive Services (PPRS) to be put into place. If the family refuses services and/or does not engage with a PPRS agency, CPS must assess the need for an ICSC to make a recommendation for Court Ordered Services (Family Service Unit) where a family can be mandated by Family Court to engage with a service plan to meet the needs of the child/family.

High/Very High Risk/Safety Concerns

When safety concerns are discovered that rise to the level of immediate/impending danger of harm to the child (ren), CPS can either support a safety plan that has been put in place by the parent/caretaker and/or initiate a safety plan that controls for the immediate health and safety of the child. Some interventions in this category are the Family Preservation Program, Removal and Parole to a Non-LDSS Resource, Multi-Systemic Therapy for Child Abuse and Neglect PPRS and other high end evidence based PPRS programs and the most restrictive, placement into Foster Care.

Link to review:

- [Permanency Principles](#)

The development of a family service plan must be based on a comprehensive assessment which includes: the safety assessment and decision, the risk assessment rating, the assessment of risk elements, information gathered on family functioning and family strengths and needs.

CPS must engage the family in the joint development of a family and individualized safety and risk reduction service plan that identifies the specific behaviors that need to be modified.

Plans should be communicated with family members in clear and understandable language, with goals and timeframes. Documentation of the service plan and the families understanding of the plan should be captured in CNIX progress notes and if applicable the FASP, and for non ICSC's, the Family Meeting Service Plan Agreement.

Note: Family Team Meetings and Child Safety Conferences can be requested and used to support or initiate safety or service plans.

CPS is to utilize the Clinical Consultants and Investigative Consultants on cases where there is Domestic Violence, Mental Health and Substance Use to help support the assessment of these underlying issues and recommendations for appropriate service referrals tailored to meet the needs of the family.

The following are the ACS referral for services from the least restrictive (CBO) to the most restrictive (Foster Care):

Referral to Community Based Organization (CBO)

Community Based Organizations (CBO) are referral resources for families within their own communities where CPS has assessed there to be no or very low risk of a child(ren) being repeatedly maltreated and/or can be referred to in combination of the other service referrals. Some examples of a CBO are: Mental Health Services, Substance Abuse Counseling, Education Services (such as Early Intervention, Committee on Special Education); Medical, Immigration and Office of Persons with Developmental Disabilities.

Required task:	Assess/describe/document:
1. Locate a community based organization that will address the family's needs: Links to review: • http://www.hitesite.org/ • Resources and Information for Families • Educational Resources • Resources for Parents of Children with Special Medical Needs • Desk Reference Of Frequently Used Services Provided by ACS	• Contact the organization and determine if it meets the needs of the family: including language capacity. • Contact the family to discuss what the agency has to offer. • Follow up that the family is engaged with services. • Document engagement and linkage with the CBO in progress notes prior to closing the case. Link to review • Principles to inform child welfare decision making regarding mental health issues
Required task:	Assess/describe/document:
2. Referral to Early Care and Education Services (Head Start) Link to review • Early Care and Education	• CPS to complete the Referral for Early Care and Education Services Sections 1-8 • Submit the Referral for Early Care and Education Services Sections 1-8 to the CPSSII • CPSSII to complete the Certification Section • CPS to submit the completed referral to the Office of Family Support via a password protected email at

	ECE.ProtectiveReferral@acs.nyc.gov .
Required task:	Assess/describe/document:
3. Referral for early intervention services: - Any child between 0-3 must be referred to early intervention if the following conditions apply: Link to review: Early Intervention and Child Find Referral Process	- For a referral for <u>Early Intervention</u>: Child with a suspected or known developmental delay or disability. - CPS to complete an Early Intervention Program Referral Form. Link to review: Early Intervention Form - Fax to the EIP Regional Office in the child's Borough of residence. - For a referral for Developmental Monitoring: Child is developing typically but may be "at risk" for atypical development, <i>or</i> child missed or failed newborn hearing screening. - CPS to complete an Early Intervention Program Referral Form. Early Intervention Form - Fax to the Child Find Office:

CONNECTIONS Documentation Requirement:
- If INV is still active, all progress notes are entered into the INV stage - If INV is coming due and can be closed, CPS to progress to FSS and enter engagement between the family and CBO in FSS progress notes, this needs to be done quickly as CPS must decide within 14 days to close the FSS or mandate services. Link to review: CPS Responsibilities in Investigation and Post Investigation Cases - If the case is Indicated, there must be documented follow up that the family is engaged with the CBO; if not a re-assessment should be made to whether a referral for a PPRS agency is necessary. - If the case is unfounded, it is best practice for follow-up that the family is engaged with the CBO.

Referral to Purchased Preventive Services (PPRS)

Purchase Preventive Referrals (PPRS) are made for supportive and rehabilitative services to children and their families for the purpose of averting a disruption of a family which could result in the placement of a child in foster care; enabling a child who has been placed in foster care to return to his family at an earlier time than would otherwise be possible; or reducing the likelihood that a child who has been discharged from foster care would return to such care.

The level of service need will be assessed through the completion of the Service Connect Instrument (SCI). Once completed, it will provide CPS with recommendations of specific treatment modalities that fall within the range of the identified level of service need. The level of service need is broken into four categories: low, moderate, high and very high.

CPS should use the Link to review: [Preventive Desk Guide](#) to review the below listed modalities of treatment and the PPRS Agencies that fall within those categories.

1. General Preventive
2. Evidence Based Preventive Programs
3. Special Medical Needs
4. Deaf and Hearing Impaired
5. Sexually Exploited Youth

Required task:	Assess/describe/document:
1. Open Family Service Intake (FSI) and progress to Family Service Stage (FSS) within 24 hours of opening the FSI Link to review: • Creating an FSI from an INV Tip Sheet • Determining a Families Eligibility for Services CPS must ensure that all PIDS are merged so an FSI/FSS is opened correctly.	• Merge all PIDs. Link to review: ○ How to merge Person ID's PIDS\ • Review and update the fields of the FSI. • Complete the decision summary to progress to the FSS • Determine Families eligibility for services • Complete the CS 2921 to open the case in WMS/CCRS
Required task:	Assess/describe/document:
2. Completion of the Service Connect Instrument (SCI) Link to review: • DCP Procedure for Referring Families to PPRS	• Go to ACS Intranet http://nycacs/ • Go to Tools http://nycacs/tools • Go into SCI Instrument • Complete the SCI questionnaire • CPS will document in Connections the results of the SCI from the SCI recommendations page. • Discuss the outcome of the SCI with CPSSII and/or CPM using Preventive Desk Guide. • CPS will complete and submit the SCI referral request form. • The CPS will then take the completed SCI packet and discuss with the Purchase Preventive Services Liaison (PPRS)

Required task:	Assess/describe/document:
3. Develop the initial Family Assessment and Service Plan (FASP) Link to review: • FASP Resources	• Select the most appropriate program choice and permanency planning goal (PPG) • Complete the needed fields (dependent upon case specifics)
Required task:	Assess/describe/document:
4. Transfer to the PPRS agency by following the policy. Link to review: • DCP Procedure for Referring Families to PPRS • Transitional Meeting Guidelines When referring cases to PPRS • CNNX Transfer CM to SSO Tip Sheet	• PPRS liaison will provide the CPS the assigned agency • CPS must assign a caseworker role to the assigned agency in CNNX within 1 business day of referral notification. • CPS to have phone conference with the assigned caseworker from the agency. During this conversation there must be a date set for the transitional meeting, within 5 days of the referral date. • Complete the transitional meeting , review the Transitional Meeting Policy • Once the case is accepted (within 10 business days of case referral), CPS is to send case planning to the agency and obtain the acceptance letter form (PROMIS, 842 form). • CPS to forward case management in CNNX to Support Systems Office (SSO)
Required task:	Assess/describe/document:
5. For children being discharged from a hospital/medical/mental health care setting	• Request discharge plan which should include the following: • Follow Up Appointments • Medications regime • Services that must be in place prior to return home or to follow discharge to community • CPS must ensure that infants/children are visited regularly while in the hospital and that their individual need for clothing, toiletries are addressed. • CPS as well as all other case planning staff should be involved with the discharge plan from the time of admission.

Required task:	Assess/describe/document:
6. Family Team Meetings and Child Safety Conferences can be requested and used to support or initiate safety or service plans.	• CPS must facilitate family meetings and/or ICSC when the safety/risk concerns to a family are high and they are not engaging in a service plan that has been recommended to meet the needs of the children,

Connections Documentation Requirement:
• Document outcome of the transitional meeting • Copy and paste the SCI recommendations page which identifies the model of service selected and reasons for referral. • For children discharged from Hospital/Medical/Mental Health facilities all information must be captured in CNNX including but not limited to: hospital name, physician name and contact information, referred doctors' name and contact information, medication, discharge place, discharge resource and emergency plans.

Course of Action
✓ Ensure occurrence of the transitional meeting with the PPRS Agency. ✓ CPS to have conversation on the recommended services based on the SCI with the Supervisor II. ✓ If the report is indicated and opened for services, open a CWS type FSS and complete the Initial Family Assessment and Service Plan and provide the appropriate services for the family by consulting the PPRS Liaison, or through community based service providers as per the individual needs. ✓ If the children are in imminent danger, arrange for appropriate safety interventions/safety plan. ✓ In cases where a removal is not warranted, but it is clear that services are needed to stabilize the family and the parents reject these services, CPS must submit an ICSC referral for the recommendation of Court Ordered Supervision (FSU).

FASP Tip Sheet

How to choose the Program Choice and Permanency Planning Goal

For children on your caseload, you must select the appropriate Program Choice and Permanency Planning Goal to create a FASP with a question set customized to the child's circumstance or situation.

There are five Program Choices available for selection in a Child Welfare Services (CWS) stage; however, no more than three may be selected at a time:

Program Choice	Definition
Preventive Non-Mandated	Service provided to a family to prevent the placement (or replacement) of a child into foster care when the need for foster care is not imminent
Preventive Mandated	Those supportive and rehabilitative services provided to children and their families for the purpose of averting a disruption of a daily which will or could result in placement of a child in foster care, enabling a child who has been placed in foster care to return to his family at an earlier time than would otherwise be possible, or reducing the like hood that a child who has been discharged from foster care would return to such care.
Placement:	The child was removed from the home and placed in the care and custody of the local district Commissioner of Social Services or the Commissioner of OCFS.
Protective	Service provided to a child named in an open child protective cases that are necessary to ensure the child's safety and reduce risk of future abuse or maltreatment
Non-LDSS Custody-Relative/Resource Placement: (which must be selected in conjunction with Preventive or Protective)	This child is placed in the home of a relative or non-related person with or without a court order and the local social services district is providing supervision and/or services to enable the child to remain safely with the relative or resource person. The local social service district does not have custody of the placed child. This must be used in conjunction with a Program Choice of Preventive (either mandated or non-mandated) and/or Protective)

For example:

Preventive: Preventive Non-Mandated or Mandated and Protective
 Foster Care: Placement and Protective
 FSU: Preventive Mandated/Protective and if paroled to someone other than the caretaker
 Non-LDSS relative/Resource Placement.

Permanency Planning Goal (PPG): The PPG represents the most desirable and most realistic permanent living arrangement for the child (ren). Only one PPG may be set for each child to be chosen along with Program Choice above.

<i>Program Choice</i>	<i>PPG of Child</i>	Permitted Combinations
Preventive (non-mandated)	6. Prevent Placement 7. Prevent Return to Placement	Protective Placement or Non-LDSS Custody (if either is selected, PPG options 6 and 7 are not available)
Preventive (mandated)	6. Prevent Placement 7. Prevent Return to Placement	
Placement	1. Return to Parent (Parent) a. Parent b. Non-Parent Caregiver 2. Placement for Adoption 3. Referral for Legal Guardianship • Relative • Non-relative 4. Placement with a fit and willing Relative (Non Guardianship/Non-Custodian) 5. Place in another planned living arrangement • Discharge to Independent Living • Discharge to Independent Living/ • Unaccompanied Refugee Minor • Discharge to Adult Residential • Care	Protective Preventive (mandated) or Preventive (non-mandated) Placement or Non-LDSS Custody (if either is selected, PPG options 6 and 7 are not available) Protective Placement or Non-LDSS Custody (if either is selected, PPG options 6 and 7 are not available)
Protective	8. Protect Child	Preventive (mandated) or Preventive (non-mandated) Placement or Non-LDSS Custody (PPG option 8 is not available if combined with any other Program Choice)
Non-LDSS Custody-Relative/Resource Placement:	9a. reunite with Parent 9b. Legalize Living Arrangements with Relative/Resource. 9c. Permanent Living Arrangement (Non-Guardianship/Non-Custodian)	Preventive (mandated) or Preventive (non-mandated) Protective

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Referral to Family Preservation Program (FPP)

CPS will make a referral to FPP from an ICSC when the family is voluntarily willing to engage in an intensive 6 week behavior change program without the use of Family Court to reinforce it. FPP is considered a short term safety intervention that works with families to alleviate the threat of harm to the child (ren).

Required task:	Assess/describe/document:
1. Open Family Service Intake (FSI) and progress to Family Service Stage (FSS) within 24 hours of opening the FSI	• Merge all PID • Review and update all fields of the FSI • Complete the decision summary to progress to FSS • Determine the families eligibility for services • Complete the CS2921 to open the case in WMS/CCRS
Required task:	Assess/describe/document:
2. Make a referral for an ICSC	• Document the immediate safety concerns for the family and collaboratively with the family; identify what least restrictive interventions can be put in place without removing the child and/or taking the case to family court for intervention.
Required task:	Assess/describe/document:
3. Discuss/Choose FPP as an outcome from the ICSC Link to review: • FPP Protocol	• CPS should invite FPP to the table to discuss FPP if this is being discussed as a recommended outcome. • FPP is reality tested in the ICSC, the family voluntarily agrees to participate in the FPP program.
Required task:	Assess/describe/document:
4. Joint Home Visit	• Conducted between FPP/PD and the family within 1 business day of the referral to FPP.
Required task:	Assess/describe/document:
5. Continued collaboration	• Weekly updates between FPP and PD on the progress of the family both verbally or email and progress notes written in CNNX.

	• At minimum two weekly face to face visits with FPP and the family. • CPS PD continues to make face to face community visits to the family during this time. • If at any time there is disengagement, a family meeting and/or CSC is to occur to either re-engage or make changes to the outcome of the ICSC.
Required task:	Assess/describe/document:
6. Transition Meeting	• Held during the 6th week of intervention • Participants are FPP, PD, PPRS (If involved), the family and any other CBO's working with the family. • Service Continuum Summary Report will be discussed to include but not limited to: • Progress of the family; • Safety/Risk concerns at the onset of intervention and the level of Safety/Risk after 6 weeks; • Services provided • Referrals made (CBO and PPRS) • FPP to either extend their services with the family or close their case for which CPS PD will continue to work with the family. • FPP to un-assign CW role

CONNECTIONS Documentation Requirement:

- Document Service Continuum Summary Report, copy and paste into CNNX

Course of Action

- ✓ Ensure occurrence of the joint home visit.
- ✓ The CPS Investigation continues while FPP is working with the family.
- ✓ Ensure on going collaborative communication between FPP and CPS.

Referral for Court Ordered Supervision with the Family Service Unit

CPS is to make a recommendation of Court Ordered Supervision during an ICSC when a family refuses to engage in services that will provide the support needed to reduce the likelihood that the child (ren) will continue to be maltreated or repeat maltreated. It is also a recommendation for placement of children with resources that do not wish to become foster parents when there are imminent/impending safety concerns for the welfare of the child (ren) if they were to remain with their parents/legal guardians.

Link to review:

- [Family Service Unit](#)

The Family Service Unit model of practice is based on Solution Based Casework. Solution-Based Casework (SBC) is an evidence-based, family-centered model of child welfare assessment, case planning, and ongoing casework. It is a partnership approach to casework with families, service providers, and the courts that targets high-risk behaviors and organizes case planning around preventing repeat maltreatment.

Link to review:

- [Solution Based Casework](#)

Solution Based Casework anchors itself around three basic tenets:

- Full partnership with the family is a critical and vital goal for each and every case.
- The partnership for protection should focus on and follow the patterns of the everyday life of the family.
- Solutions should target the prevention skills needed to reduce the risk in those actual everyday life situations.

Building upon a family's strengths, case plans are developed specific to safety and risk concerns. These behaviorally-specific plans of action are done collaboratively with the family, service providers, and ACS. The plans target skill development for family members in order to change behavior in critical areas that can then be demonstrated, documented, and celebrated.

Solution-Based Casework is another enhancement to the foundational skills and supports that already exist in ACS practice. What SBC adds to practice are skill sets around therapeutic interviewing, family life-cycle development frameworks used in engaging families and the identification of specific desired outcomes in order to develop behaviorally measurable plans to prevent repeat maltreatment. Increased coordination and consensus is built among families, service providers and the family court system around what needs to be done to protect children and reduce risk.

SBC is an especially good fit with the Family Team Conferencing model, which promotes a partnership with families and ACS staff. SBC will also support and enhance the ACS Quality

Supervision model, which utilizes ongoing case consultation, coaching and mentoring for child welfare staff.

Required task:	Assess/describe/document:
1. Make a referral for an ICSC	Document the immediate safety concerns for the family and with the family identify what least restrictive interventions can be put in place without removing the child and/or taking the case to family court for intervention.
Required task:	Assess/describe/document:
2. Choose Court Ordered Supervision (FSU) as an outcome. Link to review: Family Service Unit Child Safety Alert #24: Guideline to Filing for Court Ordered Supervision	- Complete all sections of the 865D and file an Article 10 petition for Court Ordered Supervision - Filing must be the same day by 3pm if: - the outcome is a removal and/or a parole to a non-LDSS resource - Order of Protection is required - Any other safety intervention - Filing can be the next business day morning if none of the above factors apply. - Obtain court approval for Court Ordered Supervision
Required task:	Assess/describe/document:
3. Transfer PD Case to FSU. Link to review: FSU Case Transfer Policy	- All Open Investigations (INV) must be progressed off CNNX to the FSS stage - FASP must reflect appropriate PPG and Program Choice - Complete FSU Transfer Check List - Follow FSU Case Transfer Protocol
Required task:	Assess/describe/document:
4. Joint Home Visit	Conducted between PD and FSU within 7 days of Transfer Conference.
Required task:	Assess/describe/document:
5. Continued collaboration: Link to review: Guidelines for Court Settlements Family Court Appearance Chart Case Planning and Case	- Discuss any and all progress/updates to support the decisions needed for PD and/or FSU court settlements - Un-assign WM role at Fact Finding

Management Responsibilities for Children Discharged from care with "Court Ordered" Supervision •	
Required task:	Assess/describe/document:
6. When children are discharged to "Parole" Status from Foster Care. Link to review: • Case Planning and Case Management Responsibilities for children that are discharged from Care with Ongoing COS	• Notification to the Deputy Director of the receiving FSU Borough. • Follow the policy on transfer of cases from Foster Care to FSU.
Required task:	Assess/describe/document:
7. End of Court Ordered Supervision Link to Review: • End of Court Order Supervision Hearings Policy	• End of Court Ordered Supervision Hearing will be scheduled no fewer than 30 days prior to the end of the court ordered supervision date. • In all supervision cases in which a decision has been made not to request an extension of supervision, the FSU CPS is required to complete the 60 day end of order report and submit to FCLS Attorney at least 60 days prior to the End of Court Ordered Supervision date.
Required task:	Assess/describe/document:
8. 60 day end of order report	Within in this report, the FSU CPS must include the most recent information on the following: • The improvement or elimination of the acts or conditions found at the fact finding hearing that led to the family's involvement with ACS, including the improvement of conditions from DCP's assessment as well as key collaterals and service provides. • Information on adherence by all parties with the specific directives contained in the Dispositional, ACD, or Suspended Judgment court order; • Information regarding adherence to dispositional court orders and engagement

	in services by the parole resource, parent, child(ren) or adult who has child caring responsibility for the subject child(ren). • ACS contacts made with the caretaker who the court has released the child(ren) to, the parent, child(ren) and adult who has child caring responsibility for the subject child(ren). • Updated information from collateral resources (physicians, daycare and school personnel, parole officers, probation officers, mental health providers, therapist, etc.) where applicable; • Results of criminal clearances on Domestic Incident Reports (DIR's) (pulled within 30 days of writing this report); ○ FSU CPS must request an Investigative Clearance (IC) to consult to include criminal and DIR checks on all parole resources, respondent parents and adults who reside in the home that have caretaker responsibilities for the child(ren). When informed by the IC that a person is not eligible to be cleared in the E-Justice database due to criteria for accessing criminal record history databases, the CPS should include this explanation in their report and CNNX documentation. (Please refer to ACS Procedure Child Protective Units Access to Criminal History Reports for further information.) • Results of SCR clearances (pulled within 30 days of the writing of the report) on all parole resources, respondent parent and adults who reside in the home that have caretaker responsibilities for the child(ren). • Any additional information relevant to assess the well-being and safety of the child(ren). • All 60 day end of order reports are to be signed off on by the FSU CPS Supervisor II and Child Protective Manager. • FSU CPS must make contact with the FCLS Attorney at least 3 business days prior to the hearing to discuss any issues,
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	updates or changes on the case since the submission of the 60 day end of order report. • If it has been determined that a request for an extension of supervision is needed, the FSU CPS should immediately consult with the FCLS Attorney.
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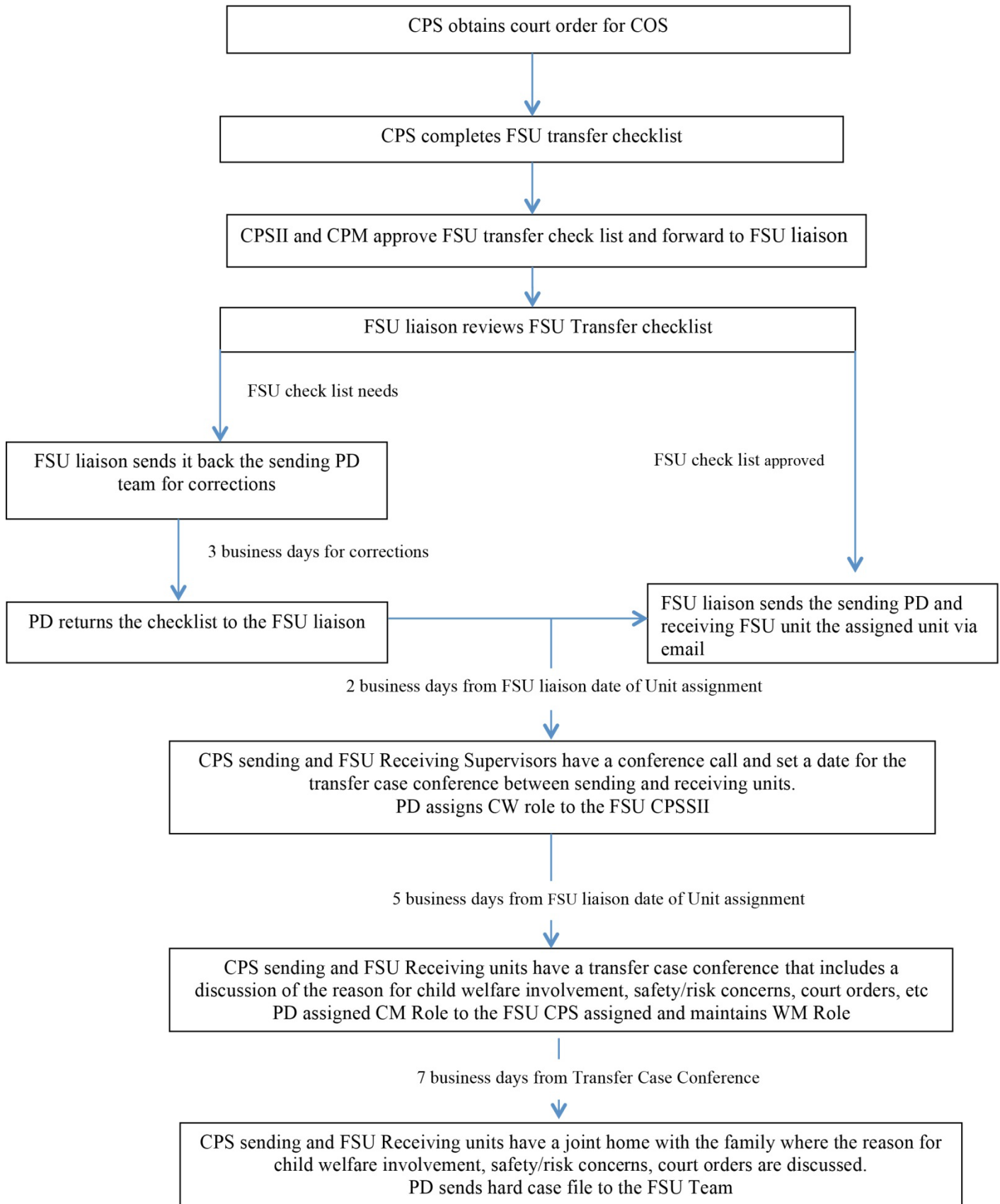
CONNECTIONS Documentation Requirement:

- Document FSU Case Transfer Checklist and copy/paste into CNNX
- CPS should document in CNNX the findings from the SCR clearance and IC consultant pulled within 30 days of the writing of the 60 day end of order report.

Course of Action

- ✓ Ensure transitional meeting occurs and is productive
- ✓ Copy and paste the FSU transfer check list into CNNX FSS progress notes.
- ✓ Ensure on going collaborative communication between FSU and CPS during the course of the case to support Disposition.
- ✓ Ensure that child (ren) discharged from foster care to the community on parole status are visited/supervised.

FSU Case Transfer Flow Chart



Referral to Homemaking Services

Homemaking Services are used temporarily when stress or crisis prevents primary caretakers from contributing to the developmental growth of their minor children or if parents or guardians possess poor parenting skills, are emotionally immature or incapable of coping with the care of their children without exposing these children to neglect or abuse. The homemaker provides services to the entire family. The ultimate goal of the inclusion of Homemaking services are to protect the child, prevent placement or support reunification when a child is discharged to their primary resource.

Stress can be defined as: physical and/or mental illness, rehabilitation, prevent placement of a child, death of a primary caretaker, caretaker requires training, caretaker preoccupied with care of ill child and alternative plan is being developed.

Crisis can be defined as: hospitalization, convalescence and complications of pregnancy.

Required task:	Assess/describe/document:
1. Make a referral Homemaking Services Link to review: • Family Home Care Policies • Home Care Forms	• A) Initial Request ○ M11Q (For all necessary family members) ○ Any additional supporting documents ○ CM-25 • Submit documents to ACS Preventive Services Home Care Liaison Original M11Qs must be submitted with the package within 10 days of the date of examination to complete the processing of the service request.
Required task:	Assess/describe/document:
2. Reauthorization and Changes	• Submit documents to ACS Preventive Services Home Care Liaison ○ M11Q (For all necessary family members) ○ CM-20X.W442C (Must be signed by Children's Services Staff) ○ CM-20A (If there is a request for change in hours), (Must be signed by Children's Services Staff) ○ CM-20E
Required task:	Assess/describe/document:
3. Overtime Sheet	• The CM-20L form should only be used with prior approval. The form should only be completed by Children's Services or Homemaker agency staff.

	(Must be signed by Children's Services Staff).
Required task:	Assess/describe/document:
4. Case Transfer for on-going Monitoring	• When PPRS is not assigned: case is transferred to FSU for on-going monitoring through the FSU Liaison • When PPRS is assigned: case is transferred to SSO with CP assigned to the assigned PPRS agency.

Referral to Home Attendant Services

ACS, in conjunction with the New York City Human Resources Administration offers Home Attendant Services to Medicaid eligible individuals who are incapacitated and who otherwise might be institutionalized. Home Attendant Services provide personal care to adults and children whose personal care tasks cannot be completely met by family due to medical needs. The ultimate goal of the inclusion of Home Attendant services are to protect the child, prevent placement or support reunification when a child is discharged to their primary resource.

Personal care is defined as: personal hygiene, dressing and feeding; nutritional and environmental support functions and health related tasks.

Required task:	Assess/describe/document:
1. Make a referral Home Attendant Services Link to review: • Family Home Care Policies • Home Care Forms	• Initial Request 1. M11Q (Required) 2. CM-25 • Submit documents to ACS Preventive Services Home Care Liaison. 1. Original M11Qs must be submitted with the package within 10 days of the date of examination.
Required task:	Assess/describe/document:
2. Reauthorization and Changes	• Submit documents to ACS Preventive Services Home Care Liaison 1. M11Q 2. M11A 3. M11S (Changes) 4. M11D (No Changes) 5. CM-20E (Must be signed by Children's Services Staff)
Required task:	Assess/describe/document:
3. Overtime Sheet	• Submit documents to ACS Preventive Services Home Care Liaison 1. M11V 2. CM-20A 3. CM-20E (Must be signed by Children's Services Staff)

Referral to Housekeeping Services

Housekeeping is a service that renders personal care services. Such services must be essential to the maintenance of the client's health and safety in his/her own home.

Personal Care services is defined as: environmental support functions

Required task:	Assess/describe/document:
1. Make a referral Housekeeping Services Link to review: • Family Home Care Policies • Home Care Forms	• Initial Request 1. M11Q (Required) 2. CM-25 • Submit documents to ACS Preventive Services Home Care Liaison. 1. Original M11Qs must be submitted with the package within 10 days of the date of examination.
Required task:	Assess/describe/document:
2. Reauthorization and Changes	• Submit documents to ACS Preventive Services Home Care Liaison 1. M11Q 2. M11A 3. M11S (Changes) 4. M11D (No Changes) 5. CM-20E (Must be signed by Children's Services Staff)
Required task:	Assess/describe/document:
3. Overtime Sheet	• Submit documents to ACS Preventive Services Home Care Liaison 1. M11V 2. CM-20A 3. CM-20E (Must be signed by Children's Services Staff)

Referral to Foster Care

Link to: [DCP Foster Care Policies and Procedures](#)

CPS must take all necessary measure to protect a child's life or health including when appropriate, the provision of emergency medical services, in home services and other safety interventions to ensure the health and safety of the child, and prevent or eliminate the need for foster care.

The law requires that reasonable efforts be made to prevent or eliminate the need for foster care placements, where appropriate.

There are 5 ways a child enters foster care system 1) by court order through a neglect or abuse petition, 2) by court order through Juvenile Delinquency Petition, 3) by court order through a PINS petition, 4) by a parent placing a child voluntarily and 5) because the child is destitute.

Emergency Removals:

CPS is authorized to conduct emergency removals without parental consent or court order only in circumstances of impending or imminent danger to the child's life or health, the parent/legal guardian is unwilling or unable to provide and follow through on a safety plan and when there is not enough time to apply for a court order.

On all CPS Emergency Removals, the ICSC must take place immediately if during business hours or at 9:30am the following business morning. If the recommendation to continue the removal is made, CPS must file a petition in court to request a Remand by 3pm the same day of the ICSC.

Link to review Policy: [Conducting Emergency Removals](#)

Court Ordered Removal:

Where temporary removal of a child from home is necessary to avoid imminent danger to the child's life or health, and the parent is absent, and if there is not enough time to file an Article 10 petition alleging abuse and neglect and for the court to hold a preliminary (1027) hearing but there is sufficient time to seek a court order prior to the removal without compromising the safety of the child CPS may seek an order from the court authorizing the removal. Inform the parent of the intent to apply for an order of removal. The CPS and supervisor must then make contact with FCLS to obtain a court order.

Examples of factors that may allow sufficient time to obtain a court order prior to removal:

- The child is temporarily safe because he or she is already staying with an appropriate resource for a time sufficient to allow ACS to obtain a court order.
- The child is temporarily safe because he or she is hospitalized or is in another safe location for a time sufficient to allow ACS to obtain a court order.
- The child is temporarily safe because the perpetrator of the harm or risk of harm to the child is in the custody of law enforcement authorities and has not access to the child for a time sufficient to allow ACS to obtain a court order.

Required task:	Assess/describe/document:
1. CPS assesses immediate and/or impending danger. Link to review: • Conducting Emergency Removals • IRT Manual Links to review: • Child Safety Alert #33: Nicholson Balancing Test: Balancing Imminent Danger and Emotional Risk of Harm during Removal Decisions	• CPS to consult with CPSSII, CPM and/or Deputy Director for guidance on the need for an emergency removal vs court ordered removal • CPS team to call IRT Coordinator for 911 assistance (Borough office for Day Hours/ECS for after hours) if an emergency removal is needed. • If the removal is imminent to the safety of the child and leaving CPS no time to arrange for 911 assistance through the IRT Coordinator; CPS should call 911 directly. • CPS must inform 911 that it is an emergency removal and you need immediate assistance due to the circumstances. • CPS must contact transportation, 646-935-7274, alerting them to the need for transportation on an emergency removal. • CPS to transport the child to the Borough Office.
Required task:	Assess/describe/document:
2. Medical (Serious Injury) and/or Child Advocacy Center Evaluations Use of Emergency Rooms and/or Child Advocacy Centers (CAC) Link to review: • Sex Abuse Protocol	• If CPS assesses the need for immediate medical attention/evaluation during the course of an investigation CPS is to ask the parent/legal guardian to accompany CPS to the Emergency Room and/or CAC. • If the parent/legal guardian is not in your presence at the time of the needed attention CPS is to ask the parent/legal guardian to meet CPS at the emergency room and/or CAC. • If the parent/legal guardian refuses immediate medical attention and CPS determines the continued need for it, CPS is to consult with their Supervisor to discuss the need for an emergency removal and continue on to the emergency room.
Required task:	Assess/describe/document:
3. Destitute Child: Link to review • Destitute Child Policy	• Make reasonable efforts to locate a parent/caretaker/legal guardian. • CPS to obtain an FCLS consult to determine if the child in question is destitute

	- Once taken into custody, CPS has 14 calendar days to file a destitute petition in court. - An ICSC is NOT required; CFS should be notified with the completed Child Section A and placed in the CES Ready Folder. - CPS to complete the 865D. - All Destitute Case filings will occur in Manhattan Family Court. - CPS will transfer the case as appropriate to the SSO and the foster care agency assigned. - CPS will maintain WM role until disposition and are required to attend all court proceedings.
Required task:	Assess/describe/document:
4. Voluntary Placement with Parent/Legal Guardian Link to review: Procedures for filing Court Petitions for the Approval of Voluntary Placement Agreement Instruments Voluntary Agreement Forms	- CPS to make a referral for an ICSC and have an outcome of a Voluntary Placement. - Obtain an FCLS consult - The parent does not sign the Voluntary Agreement Form until placement is located and the child (ren) will be placed that day to their placement destination (this can include ECS). - If the youth refuses to go with CPS, then the Voluntary Placement cannot be completed and an assessment must be made to return for a CSC. - Once placement has been located: - CPS to complete with the parent the Voluntary Placement Agreement
Required task:	Assess/describe/document:
5. Voluntary Placement with person Entrusted with the child. Link for: Voluntary Agreement with person entrusted with the care of the child	- CPS to make a referral for an ICSC and have an outcome of a Voluntary Placement. - Obtain an FCLS consult. - The entrustment agreement will not be signed until placement is found (including ECS). - The person cannot rescind an entrustment agreement and this should be made known to the person signing it. - If the youth refuses to go with CPS, then the Voluntary Placement cannot be completed and an assessment must be made to return for a CSC.

	- Once placement has been located: - CPS to complete Entrustment Agreement form
Required task:	Assess/describe/document:
6. Reducing Trauma during the removal process Link to review: Minimizing Harm to Children During the Removal and Placement Process Trauma Reduction Guidelines	- CPS is to ensure that all parents/guardians are appropriately notified of the reasons why the child(ren) are being removed - CPS should ask the child (ren) what items they would like to take with them: pictures, stuffed animals, etc. - CPS are to ensure that all medications that are required for the child(ren) are taken with them
Required task:	Assess/describe/document:
- Provide Notification of Temporary Removal to Parent/Guardian Link to review policy: Appropriate use of Notice of Removal Forms DCP 005 Notice of Temporary Removal	- CPS must complete the Emergency Removal Form DCP 005 - Provide a copy to the parent - A copy must also be maintained for the record.
Required task:	Assess/describe/document:
7. Provide Notification of Temporary Removal to Relatives and Grandparents of removed/placed child. Link to review: Notice of Rights.	CPS must provide relatives and all grandparents of children who have been removed (or voluntarily placed) from their parent/guardian the Notice of Rights.
Required task:	Assess/describe/document:
8. Provide Notification of Temporary Removal to school or other educational setting, Healthcare, Daycare or Community Setting. Link for form: CS 701C.	- CPS must provide the school or other educational setting, Healthcare, Daycare or Community Setting that is caring for the child at the time of the removal with the Emergency Removal Form CS 701C - A copy must also be maintained for the record. - When CPS removes a child from the above

	settings, CPS must leave the DCP 005 at the parent/legal guardians case address if no contact has been made since the removal was conducted
Required task:	Assess/describe/document:
9. Refer for an ICSC no later than 9:30am the following business Day CPS can make a request for an emergency ICSC at any point during normal business hours. CPS can also make a request for a planned ISCS which may or may not have to occur the same day.	• CPS to follow ICSC Procedures . • CPS to ensure that Correct ICSC outcome for Remand and Removal are clearly identified on the outcome form.

CONNECTIONS Documentation Requirement:

- CPS must document all reasonable efforts made to minimize the need for the removal of a child (ren).
- Once the child (ren) is removed, CPS will complete all required sections in the FSS pertaining to placement and permanency for example AFCARS.
- CPS will document the medical information in the Build 18.9 Health section.

Course of Action

- ✓ Make all reasonable effort to ensure the safety of the children while maintaining child(ren) safely in their homes preventing foster care placement.
- ✓ Obtain a court order for a removal when it is possible to do so without compromising the health or safety of the child.
- ✓ Ensure that all children on remand status that are in mental health settings are appropriately communicated to CFS.
- ✓ Make all possible efforts to reduce trauma children experience during a removal.

Exploring Kinship Resources

CPS is to make every effort to place children who are removed with kinship Resources, including relatives and other adults who already have a significant relationship with the child or family. This person could be a family member, community member, teacher, coach or priest. All caregivers must be willing to keep the child (ren) safe.

Kinship homes are eligible to receive an Expedited/Emergency Home study not only for children placed on Article X petition, but also for children who are voluntarily placed and for youth who come into care as a result of a Persons in Need of Supervision (PINS) or a Juvenile Delinquency (JD) Petition.

Placement and Domestic Violence:

In considering kinship foster care placement in cases involving domestic violence, a careful assessment must be conducted to ensure that such arrangements will not compromise the safety and well-being of the adult survivor or child, or interfere with permanency planning with the adult survivor (CIR Strategic Plan, May 2003). The need to have relevant information about using a relative for a placement is one more compelling reason why obtaining a careful and comprehensive family history, with a history regarding domestic violence, is so important during the assessment process. Individual family members must be interviewed separately during this assessment.

Children entering kinship placement must not be placed with any person who is currently a perpetrator of domestic violence. Some of the parents with whom we work come from families where significant problems related to domestic violence may affect the lives of family and extended family members. If an assessment reveals past domestic violence, a thorough assessment must be done to document that it is no longer present in the home. Children should not be placed where these circumstances exist.

Required task:	Assess/describe/document:
- Locating Kinship Placements Link to review: Kinship Care Policies Guidelines for placing with Kinship ACS Immigration and Language Guidelines for Child Welfare Staff (2nd edition)	- Interview all family members including the child (ren) to identify the adults whom the child(ren) has a significant relationship and who might be able to serve as a kinship placement resource. - Immigrants, including those who are undocumented, can be considered as kinship resources. - Relatives residing outside the United States may be considered as discharge resources for children in foster care, however, these relatives cannot be considered for foster parent status.

	• CPS is to make all efforts to locate a Kinship resource to foster the child(ren) being removed.
Required task:	Assess/describe/document:
1. If this involves a sibling group , CPS to make all efforts to place them together	Link to review: • Flexibility in sleeping arrangement requirements for sibling foster care placement
Required task:	Assess/describe/document:
2. If this is an Emergency Replacement of Children already in Foster Care CPS to review	Link to review: • Process of Emergency Replacement of Children in Foster Care
Required task:	Assess/describe/document:
3. If the child has special medical needs:	Link to review: • Assessment and Safety Planning for Children with Special Medical Needs
Required task:	Assess/describe/document:
4. If this child is a youth identified as sexually exploited:	Link to review: • Assessment and Safety Planning with Commercially Sexually Exploited (Sex Trafficked) Children
Required task:	Assess/describe/document:
5. Assessing Kinship Resources Link to review: • Guidelines for placing with Kinship	Before a child may be placed in an emergency certified or approved foster home: Children's Services must review its records to determine whether the potential caretaker(s) or eligible relative(s) has a prior history of abuse or maltreatment. • Children's Services must conduct SCR, ACRS+, NYC/WMS, and CCRS as well as DIR and Public Criminal History clearances of all members of the relative or non-relative household. If there is/was an SCR or ACS case, the case circumstances must be evaluated with and by the case manager. When the Division of Child Protection (DCP) has case management of the case, the CPM has final approval on the placement decision of the child in the home.

Required task:	Assess/describe/document:
6. In a situation where a prior indicated child welfare case is located when clearing a prospective Kinship Resource	• If the indicated report was from July 1, 1997 to present, the case should be reviewed on line in Connections to assess the situation. If the indicated report is prior to June 30, 1997, the physical case record must be reviewed and evaluated. • The child should not be placed in the home of the relative or friend until such record is reviewed. • Information should be obtained from staff and providers who previously worked with the family if available. • The current family functioning and household conditions should be reviewed to assess whether there have been changes since the prior report(s). • Review the level of seriousness of the past allegation(s) and any current concerns.
Required task:	Assess/describe/document:
7. Emergency Home Study for approval to become a foster parent for removed child(ren) Links to obtain: • Kinship Emergency Home Study • Emergency Home Study Tip Sheet	• CPS to complete the Emergency Home Study Package

CPS will transport all children to the Borough Office during daytime hours for medical clearance and placement.

Emergency Home Study Tip Sheet

Basic Requirements at Time of, or Before Placement:

1. Legal requirement: Article 3, 7, or 10; or Voluntary Placement, Section 384.
2. Relative **or** non-relative with significant past/present relationship with the child or the child's parent.
3. Relative or non-relative is 21 years of age or older.
4. Thorough assessment of the home – Document any Safety/Health Hazards.
5. Specifically document the sleeping arrangements for all members of the family and for any other household residents.
6. Clearances: WMS/CCRS, ACRS+, CNNX, NYC/WMS on all members of the household.
7. Parent(s) from whom child is being removed does/do not reside in the home.
8. Discuss the roles and responsibilities of being a Foster Parent and provide the kinship resource with a copy of: ACS Kinship Foster Care: A Helping Guide for Kinship Resources.
9. **No licensed foster home may have more than 6 foster children placed in it,(no more than 8 for siblings).**
10. The CPS Must:
 - Appropriately complete the Emergency Home Study Checklist (p. 11 of document).
 - Discuss the follow up to the Emergency Home Study that will be conducted by the foster care agency, including the role and authority of the foster care agency in supervising the placement.
11. The Kinship Foster Parent Must:
 - a. Agree to comply with all aspects of the foster care agency's full home study and all foster care regulations.

Evaluation Questions Which Must be Answered & Documented in Detail for Certification to Occur:

1. Describe the relationship of the foster parent(s) with the child(ren)'s parents.
2. If applicable, describe the care that the foster parent(s) provide to other children already in the home.
3. To what degree are the foster parents aware of the circumstances and conditions that led to the child (ren)'s need for foster care placement?
4. To what degree were the foster parents involved in helping and/or protecting the child from abuse or maltreatment in the past?
5. Are the foster parents able to protect the child(ren) placed in their home and do they understand the need to protect the child from abuse and maltreatment?
6. Describe the general condition of the home and the general condition of the kitchen, living room, bathroom(s) and bedroom(s).

Requirements Which May be Granted an Exception with ACS Approval:

1. Separate bed for each child over the age of 3 (Allowable exception prior to the completion of the full home study only).
2. No more than 3 persons occupying a bedroom.
3. No child 3 years or older sleeping in same room with adult of opposite sex.
4. Separate bedrooms for children of opposite sex over seven years of age.

Requirements Which May be Waived by the Agency:

1. No more than 2 infants under the age of 2, including children of the foster parent(s).
2. No boarders/lodgers (If there are boarders/lodgers, they must have an emergency clearance by the SCR).

Practice Tips for Conducting High Quality Emergency Home Studies:

1. All children referred must be in the identified kinship home.
2. There must be adequate space (in accordance with space per child regulations) for all of the children.
3. All checklists must be fully and accurately completed based upon what is observed. If you are completing a checklist, actually check what you see in front of you. All exceptions that are needed or requested must be clearly documented on the checklist.
4. If there is an agreement made between CPS and the kinship foster parent (i.e., promise to obtain smoke detector), it should be documented on the checklist with a specific date for compliance.
5. Every child requires a bed (or a crib for an infant) and if there is no bed/crib, the CPS, not the agency, will need to provide it. If there are no beds or insufficient beds, the CPS must provide documentation in the Comments section, along with the remediation plan and the sleeping arrangements for all household members.
6. The kinship family needs to have a landline, not just a cell phone. If they only have a cell phone, they need to be told that they need a landline, and this discussion needs to be documented in the Comments section.
7. Refer to Kinship Procedure (May 9, 2002) for additional details regarding Emergency
8. Home Study completion. Call your supervisor if you have any questions.

Placement to foster care

Link to review:

- [Placement Principles](#)

To thrive and grow into healthy, capable adults, children need a sense of belonging to a family who provides an unconditional commitment to them. When placement into foster care is required to ensure a child's safety, the family should be fully engaged in planning for services and the child's safe return home as soon as possible. An alternative safe and nurturing family for the child (ren) must be found as soon as possible.

Child Facilitation Specialists

The Child Facilitation Specialist (CFS) is responsible for evaluating all children and determining the most appropriate level of care, type of placement and services. CFS are located within every field office and at Pre Placement Services.

The Adoption and Safe Families Act:

Link to review:

- [Guide to ASFA \(Various Languages\)](#)

The Adoption and Safe Families Act (ASFA) emphasizes the need for expediting permanency for children and requires that the child's health and safety be the major considerations when seeking to reunify the family.

Interacting with Children during the Placement Process

Link to review:

- [Trauma Reduction Guidelines](#)

Placing children into protective custody can result in trauma to children. All parties involved in the process: CPS, CFS, nursing, child care and others must be sensitive to the needs of the child during this time. Each child will react differently to being placed into protective custody. Reactions to be expected can include:

- crying
- tantrums
- withdrawal
- refusal to speak
- extreme anxiety at being separated from CPS or other staff
- yelling, cursing
- attempting to AWOL

Staff must make every effort to ensure that children feel safe and respected during the placement process.

Required task:	Assess/describe/document:
1. Gather sufficient information about the children to be placed.	• Complete the Child Placement Referral Form, Section 'A' to refer child for

Links to review: • Guidelines for the Provision of Emergency and Inpatient Mental Health Services for Children in Foster Care and Child Protective System	placement and place in the CFS Ready Folder • CFS completes Child Placement Referral Form Section 'B' • CPS to ensure that MCHU is alerted when a child on remand status in a mental health hospital setting. • CPS to provide the Child Facilitation Specialist with the completed emergency home study.
Required task:	Assess/describe/document:
2. CPS to ensure that youth are signed in at the Children Center and nursery.	Links to review: • Securing Children's Personal Property at the Children's Center
Required task:	Assess/describe/document:
3. All children entering placement must be cleared by medical staff at field office or Children's Center nurseries. In special cases, children may be medically cleared at hospitals at which the child is known. CPS and Nursery or Hospital medical staff will confer concerning any special needs of the child. Nursery medical staff is also available for consultation on medical issues experienced by family members, or for general medical information. The Children's Center psychiatrist and psychologists should also be consulted in case applicable situations.	• Retain documentation of medical clearance for the case record.
Required task:	Assess/describe/document:
4. Accompanying a child to placement: When the child is brought to a foster home, the CPS should introduce the child and give the Child Placement Referral Form Section 'B', and the nurse triage form along with any needed medication, equipment or special care instructions to the	• Document the placement process and interaction with the resource parent(s) in a Progress Note: Include: • Child's reaction to the new home • Observe resource parent's interaction with the child • General observations of the foster home • Discussion with resource parent regarding any

resource parent. Ensure that the resource parent receives a copy of each child's individual CPR section 'B' and nurse triage form.) Provide any relevant facts about the child that will enable the resource parent to help adjust the child to his or her new setting. If transportation CPS is escorting the child to placement, that CPS must be equipped with the CPR section 'B' and any other information necessary to facilitate a discussion with the foster parents. Provide resource parent with a copy of the Kinship Foster Care Helping Guide. Inform the resource parent that planning conferences will be upcoming and suggest that they contact the foster care agency promptly for details.	critical health or mental health treatment (including medication and follow-up).
Required task:	Assess/describe/document:
5. CFS will recognize whether an infant qualifies for a "Babies Can't Wait" placement. These placements focus upon the attachment and permanency needs of infants who: • are under 18 months of age • do not have sibling(s) in care, or the sibling(s) have already been freed for adoption • do not have an available kinship resource Infants who meet these criteria are placed in a foster/adoptive home, while intensive concurrent planning toward reunification simultaneously occurs with the parent(s).	• CPS to alert CFS of any children that fall under the guidelines for "Babies Can't Wait."

Required task:	Assess/describe/document:
6. To make a request for <u>placement out of state</u> or for the return of a child to his/her parent out of state under trial discharge. Links to review: • Interstate Compact Packet	• CPS to complete the ICPC package DOCUMENTS REQUIRED: 1. Five (5) copies of ICPC Form 100A 2. Three (3) copies of all other materials, including: • Cover Letter • Financial and Medical Plan • Court Order – • Certification of Title IV-E Eligibility for Medicaid • Social History Summary • Social Security and Birth Certificate • ICPC 100B form - If the child is already present in the receiving state • All requests to place a child out of state must be made to: Interstate Compact on the Placement of Children New York State Office of Children and Family Services Room 323 North Building 52 Washington Street Rensselaer, NY 1214
CONNECTIONS Documentation Requirement for Placement:	
• All information pertaining to the child (ren)'s medical, mental health and educational needs in the FSS. • All demographic information about the foster care placement • ICPC information has been submitted, contact information, follow- up and outcome of ICPC submission. • Any and all information pertaining to the behavior of child (ren) during the removal process and what interventions were used to reduce trauma.	

**New York City Administration for Children's Services
Division of Child Protection**

Trauma Reduction Guidelines

Goal: Removing a child from home and family is a traumatic experience for both children and parents. We can't eliminate this trauma, but we can commit ourselves to do everything we can to minimize it. We do this to not only to give the children and parents whose lives we enter the respect and compassion they deserve, but also because minimizing trauma will make it much easier to begin the work needed to provide a child with a safe stable home. The following are some of the principles that work toward reducing trauma during placement.

Families must know why the removal is taking place and what to expect.

- Parents must be fully informed about the reasons why their child is being removed from their home, where their child is being taken and when they will be able to speak with their child.
- Parents must be fully informed about what their rights and responsibilities are with regard to the Family Court proceedings and the Child Safety Conferences and encouraged to actively participate.

Children must know why they have been removed from their home and what to expect.

- Children must receive an explanation of why they have been removed from their home, where they are going, and when they can speak with their parents.
- Children must be assured that the removal is not their fault and the separation is not a punishment for anything that happened.
- Children must receive a step by step description of what will happen during the placement process, including who the children will encounter, what the people do, and how that relates to the children.

Children need consistency during the placement process.

- Children must have continuous contact with a trusted adult from the time the removal occurs until they are introduced to their new caretakers at the foster home or other placement.
- Limit the number of different strangers children talk to, places they go, and information requested
- Foster parents or other new caretakers must be provided with enough information about the children to care for them and help the children adjust to their new surroundings.

Children deserve compassion during the placement process.

- Children must be spoken to in a soothing and calm voice.
- Children must be cared for and comforted in a sensitive age-appropriate manner.
- Children should be encouraged to ask questions and must have their fears and concerns about placement sincerely listened to and acknowledged.

Children need to maintain a connection with their families.

- Children must be encouraged to bring both necessary and sentimental belongings with them at the time of the removal.
- Children must be given opportunities to contact their parents during the placement process.
- Children deserve to be placed with their siblings and kin (see Placement Principles.)

Actions: The steps outlined below describe steps to be followed to reduce the trauma to children and parents during an emergency removal. Due to the volatile nature of an emergency removal, it may not be feasible to follow all steps, but every effort must be made to follow these requirements. In severe cases, depending on the nature of the abuse or neglect experienced by the children, certain statements or outlined actions in these guidelines may need to be adjusted to be appropriate for some case circumstances.

Prepare Parent/Caretaker

Requirements during an Emergency Removal

- Identify the role of the caseworker in assisting the family.
- Explain clearly to the parents/caretakers the decision for removal including unsafe condition(s) in the child's environment.
- Inform the parents or caretakers they will have telephone contact and face to face visits with their children during the separation.
- Help identify relatives for kinship.
- Provide the parents or caretakers with information about ACS (ACS Parent's Handbook.)
- Provide the parents or caretakers with information about the child welfare laws (Parent's Guide to NYS Child Welfare Law.)
- Provide the parents or caretakers with information about the Family Court Process (Notice of Temporary Removal and Right to Hearing DCP 005.)
- Provide the parents or caretakers with information about the child safety conference (Initial Child Safety Conference and Follow- Up Conference)
- Advise the parent/caretaker that follow-up and contact from an ACS caseworker and foster care workers will occur. Give the parents your phone number so they can contact you with additional questions or concerns.
- Advise the family to expect a call by the following morning regarding the name and telephone number of the foster care agency.
- Identify an emergency contact number for the family if they do not have a phone.
- Obtain medical information, medications, change of clothing, and any other items needed to maintain the child's connection to the home and family. Explain to the parents or caretakers that these belongings are needed for their children's comfort.

Under Favorable Circumstances

- Explain to the family ACS' role and that ACS wants to work toward reunification and help fix or minimize the unsafe conditions.
- Discuss the services available through ACS and how they can help the family.
- Help identify relatives for kinship and explain the kinship foster care process.

- Discuss the child welfare laws and the Family Court Process with the parents or caretakers. Emphasize to the parents or caretakers that their involvement is very important and they have opportunities to be heard in court and plan with ACS in the next couple days.
- Discuss the child safety conference with the parents or caretakers.
- Help to identify people that the parents or caretakers feel would provide support to them during the separation and child safety conference.
- Discuss the family's social interactions and relationships i.e. religious affiliations, community organizations, friends, and relatives.
- Obtain from the parent/caretaker any additional information that will be helpful in allowing the child to have an easier adjustment to a foster home i.e. special diet, favorite foods, and recreational activities.

Prepare Children

Requirements during an Emergency Removal

- Use a soothing voice and calm manner to put the children at ease.
- Use vocabulary that is easy for the children to understand and define words, such as kinship or placement in simple terms.
- Identify the role of the caseworker in assisting the family.
- Help the children understand what is happening and why they are leaving the home including helping the children understand the reason for placement.
- Assure the children that they can contact their family during the separation.
- Encourage children to bring personal belongings that can be helpful during the transition into care, i.e. school books, favorite toys, photographs.
- Offer the child a duffel bag if they do not have appropriate luggage.

Requirements after Removal

- Make sure the children understand that the emergency placement is not their fault and is not a punishment for anything.
- Sincerely listen to the children's fears and concerns about placement and address them.
- Encourage the children to ask questions to alleviate anxiety.
- Help the children understand that foster care is temporary and, if appropriate, that ACS wants to work toward reunification. Also tell the children that ACS will try to keep siblings together.
- Describe step by step the events that will follow throughout the placement process.
- Identify the names of staff the children will encounter, their job functions, and possible questions that staff may ask the children.
- Prepare the child for separation from CPS; after foster care placement arrangements have been secured.

Transition to Other ACS Service Professionals

Requirements during Transition

- The worker that removed the children for emergency placement is the adult responsible for the children and whenever possible, must be continuously in the presence of those children until they are placed.
- Minimize the number of repetitive interviews and examinations by various professionals.
- Help siblings to remain together during the pre-placement process. Be sure to notify siblings when they will need to be seen individually, why, and how long the separation will last.
- Give the children information about who will be seeing them, what that person does, why the children are seeing this person, what the child can expect to happen, and about how long it will be before the children see you again and how you can be reached, if you need to leave them in the care of another staff member.
- Give children an opportunity to speak with family.

Placement

Requirements

- All staff involved with the case must share all relevant information about the children and the children's placement.
- Once a placement has been arranged, call the parents or caretakers with the placement information and contact numbers.
- Tell the child about the placement.
- Tell siblings if they will be separated and why. Children may experience additional trauma by being separated. Provide separated siblings with pictures of each other and the phone numbers of their placements. Tell the children that ACS will make every effort to reunify the children as soon as possible and the children will be able to visit each other during the separation.
- Transport the child to the foster care setting. (Whenever possible, the worker who made the emergency placement should travel with the children to the foster home to provide the children with continuity.)
- If the worker is unable to travel to the foster home: explain to the children why the worker cannot go with them, and tell them who they will be meeting, where they will be going, and next steps.
- Tell the children approximately when the worker will see them again or say goodbye if the worker will not.
- Take time to help the children adjust to the new worker who will be transporting them to the foster home.
- Give the children your phone number in case they need to call you.

Foster Care

Requirements

- Provide the children with names of the foster care parents or facility staff.

- Discuss location of the placement with the children in relation to the children's own school and community.
- Remind the children of what to expect while in care, including the need for a medical examination and interviews with foster care agency workers.
- Remind the children that they can have phone contact and face to face visits with their parents during the separation.
- Introduce the child to the foster family or other caretakers.
- Help child settle prior to departing.
- Discuss with the foster parents or other caretakers information about the children that will help the foster parent care for the children, and assist the foster parents in helping the children adjust to their new settings.

Next Day Follow-up

In order to ensure continuity and minimize the trauma experienced by children during the removal/placement process, the CPS worker will contact the family, the child, the foster home and the foster care agency to provide and relay information and to ascertain the status of the acclimation of the child to the foster home environment. In severe cases, depending on the nature of the abuse or neglect experienced by the children, certain statements or outlined actions in these guidelines may need to be adjusted to be appropriate for some case circumstances. The following steps should be used in this process.

- ECS: Document all Day CPS contacts and interventions on ECS History and complete ECS face sheet. Fax to appropriate Field Office immediately.
- Follow casework protocols and procedures.
- Assess case for appropriateness of contact with family and child.
- Contact foster home; obtain the status of child's acclimation to foster home environment.
- Speak with children - listen to concerns and answer any questions, provide reassurance and explain the next steps of placement process.
- Contact biological parents or caretakers and provide feedback and relay any communications or requests from the children. Be sure to discuss any information that was not possible to convey at the time of removal such as the name of the foster care agency and phone number.
- It should be reinforced to the parents or caretakers that their involvement in the process is important and they have opportunities to be heard in court and to plan with ACS in the next couple days. Re-explain process of placement, family court, and the child safety conferences.
- Explain to parents or caretakers that field office staff will continue with the investigation and services.
- Contact the foster care agency - relay any information and or requests from the child, and or biological parent or caretaker.
- Re-contact, if necessary, the foster home to convey additional information to child or foster parent.
- Field Office: Complete all necessary documentation.

Required task:	Assess/describe/document:
1. Transfer Case Planning to the Foster Care Agency	• Once a Foster Care Agency is assigned, CPS to ensure that a CW role is provided

Links to review: • Transition to Foster Care • Documentation Guidelines for Transition Meetings for Children Entering Foster Care Links to review • Timely Submissions of 2921 for Children on Remand status at the Children's Center or Hospitalization Status	to the Foster Care Worker assigned. • CPS or FCA to inform the parent/caretaker of the name of the foster care agency assigned as contact information. • CPS to conduct a Transitional Meeting within 2 business days of legal foster care placement • Timely notification to SSO when children are on remand status • In 2 business days complete the 2921 • CPS to maintain WM role in the FSS stage until Family Court proceedings reach the dispositional stage.
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CONNECTIONS Documentation Requirement for transition into Foster Care:	
• Outcome of the Transitional meeting • Date of the parent to parent meeting and 1st visit • Any and all medical/mental health information for child (ren) being discharged from a hospital setting. • Complete placement package within 2 business days after the courts grant the removal/remand	

Closing a Case

Cases may only be closed to CPS when there are no longer any protective concerns, or there is no legal basis for providing oversight. Children are assessed to be safe despite the withdrawal of controlling interventions that may have protected the children and the risk of future abuse/maltreatment/

Once child protective concerns are resolved, or there is no court-ordered services and Family Court intervention is not warranted, the case may be closed to the SCR. The documentation of closing must explain why the case is being closed. The family should be engaged in the case closing decision

Required Task	Asses/Describe/Document
Unfounded Cases: - Evaluate whether additional services are needed. - If not, proceed to close the case. - If yes, refer the case either to a Community Based Organization and/or PPRS.	Summarize all cases activities in CNNX as a closing summary note; - Reason why additional services are not needed - Reasons for services and kind of services referred to if referring the case. - Families with <u>High Rap Scores</u> must be engaged to take services, documentation of CPS efforts must be made in CNNX. - Families with <u>Very High RAP</u> score that are not referred for services require the approval of a CPM for closure.
Required Task	Asses/Describe/Document
Indicated Cases: For cases that are indicated and closed at the same time: follow the Risk Assessment Profile Guidelines	Summarize all cases activities in CNNX as a closing summary note; - Significant case events that have occurred and interventions that have been provided - Reasons why the case is being closed although there have been substantiated allegations of abuse/maltreatment. - What is the protective capacity/vulnerability of the child and a clear rationale for closing the a case when the risk level of assessed

	to be of an intermediate of higher level. • Separate Closing Summary progress note from the CPM with the reasons for approving the closure of an indicated case with a Very High Risk RAP.
Required Task	Asses/Describe/Document
Home Visits for closure: For indicate or unfounded: • Conduct at least two successful home visits before closing a case • Conduct a home visit within 7 days of closing the case. Home Visits for transfer: All children must be seen 14 days prior to any case transfer. The following are specific meetings that must take place in order for a transfer to be completed. • PPRS: Conduct a Joint Home Visit prior to transferring a case to PPRS; all families/children must be seen 14 days prior to transferring CM to SSO. • Foster Care: Conduct a Transitional Meeting prior to transferring ato Foster Care Agency/SSO. • FSU: Conduct a home visit within 14 days prior to transferring the case to FSU.	Summarize the following events in CNNX: • Discussion with the family the safety/risk/permanency and well-being concerns. • Discussion of behavioral changes that increase the protective capacity of the parent. • Discuss with the family the closure of the case and their reaction • Discuss with the family the availability of resources both CBO and PPRS. • Conduct case management activities until closing or transferring of the case. • Inform service providers of the closure/transfer of the case. • Provide Mandated Reporter letters of outcomes • Follow referral to PPRS/Foster Care/FSU Case Transfer policies.
Required Task	Asses/Describe/Document
All prior records, both closed and opened must be retrieved and reviewed prior to closing the case.	Summarize the following events in CNNX: • When records are requested and received.

	Result of review of prior history, describe if the prior records support the closure of the case.)
Required Task	Asses/Describe/Document

Supervision

Supervision requires a skill that creates an environment for achievement of agency goals and mission to inform practice. This practice supports the development of the frontline assessment skills leading to positive changes and strengthening of families.

SUPERVISION

Supervision is coaching, supporting and assisting staff to make accurate, comprehensive assessments and to provide appropriate delivery of services to families; supervisors have a direct impact on the quality of life outcomes for children and families their staff come into contact with. Supervision requires that all staff perform their respective duties and responsibilities diligently and responsibly, therefore, adhering to quality case practice. Supervisors are charged with ensuring appropriate practice is in place to achieve safety and timely permanency of child well-being. They are obligated to remain focused on helping staff to achieve positive outcomes for children.

Supervision Indicators:

- Ensures that staff makes accurate safety decisions by articulating clear expectations, monitoring key success variables and providing effective feedback.
- Coaches CPS to achieve ACS outcomes by helping them make accurate decisions at each decision point and encouraging them to involve families in decision making throughout the case process.
- Assist staff in establishing concurrent plans and achieving timely permanency for children and adolescents in accordance with ASFA.
- Reviews cases and holds supervision meetings with staff members to monitor and assess the provision of services and case work contacts and to make sure that each of the outcomes are being achieved.
- Monitor family service planning to make sure services match assessments
- Coaches staff when assessing families to consider primary needs, attitudes, biases, life experiences, change readiness and strengths in order to identify how these factors influence or sustain the family's maltreatment.
- Holds unit meetings to introduce or debrief new policies, programs or initiatives that impact the division.
- Shares data on the units collective efforts toward meeting or exceeding program outcomes
- Monitors staff performance regarding job standards and their success at achieving the child welfare outcomes

Formal one on one supervision, as well as regularly scheduled group meetings are critical components of quality supervision and are required for all levels of the Child Protective Services Division. Supervision meetings must be structured to provide administrative, supportive and educative supervision to staff and is done in three forms: individual, group and during daily interaction.

Individual: bi-weekly supervision meetings with each unit member. The supervision session should be used to address workloads, case specific issues not addressed in daily interaction, reflective listening and straight talk with staff, discuss performance using observations, reviews and data, identify areas for improvement and if necessary begin the development of a plan for training, performance improvement and skill development.

[Employee Critical Incident Information](#)

Group: bi-weekly meetings are to be held with unit members to discuss new policies, initiatives and changes to agency practice; debrief with staff to determine how to integrate learning into practice change. Identify training needs for the team that will strengthen practice, critical thinking skills and decision making.

Daily interaction: meetings with unit members to trouble shoot case issues as they are presented, promote critical thinking by using spot feedback, reflective listening, observation and guidance on problematic case/staff situations.

Kadushin Supervision Model

Link to review:

- [Kadushin Supervision Model](#)
- [NYC Supervision Guidelines](#)

The Kadushin model of supervision is a social work supervision model commonly used in social service settings. The model identifies **Administrative, Educational and Supportive Supervision** as critical elements to help staff achieve organizational objectives.

Administrative Supervision

Administrative Supervision is the correct, effective and appropriate implementation of agency policies and procedures. Administrative Supervision promotes and maintains practice that is efficient and consistent with administration policies.

Tasks of Administrative Supervision relative to our Child Protection daily functions can include:

- Operationalizing the vision of the agency and unit
- Management of Workload
- Assessment of unit and individual staff performance
- Analyzing data to monitor unit and individual staffer's performance
- Monitoring, reviewing, and evaluating of CPS work
- Communication/Advocacy between CPS and upper management

Administrative Policies

Educational Supervision

Educational supervision is the development of each individual staff member and to assist CPS in completing the required tasks and develop his/her full potential. The primary goal of educational supervision is to increase CPS's skill to meet the organizational goals.

Tasks of Educational Supervision relative to our Child Protection daily functions can include:

- Debriefing with staff on new policies, procedures or agency outcomes
- Providing technical, hands on teaching relative to job functions
- Creating a plan for training, performance improvement and skill development
- Reinforcement of the appropriate use of resources (e.g. clinical/investigative consultants)

Supportive Supervision

Supportive Supervision builds CPS morale and job satisfaction

Tasks of Supportive Supervision relative to our Child Protection daily functions can include;

- Identify and discuss areas of discomfort in carrying out job responsibilities
- Recognition of strengths and areas of success
- Advocate and collaborate with other divisions on behalf of CPS
- Assist and guide CPS in time management
- Addressing the need for self-care, referrals to Employee Assistance Program (EAP)

Required task:	Assess/describe/document:
3. Supervision meetings *Hold bi-weekly, monthly and as needed supervision meetings with CPS staff	• Document all supervisory sessions by completing the supervisory session form CS-583 • Submit CS-583 form in journal of outlook

Supervisor's checklist to Support preparing CPS for Supervision

- CPS should be prepared to have meaningful discussion of cases on their workload, who is the family, what are their strengths, needs, or challenges.
- Share thoughts as to what are the proposed next steps for the family, given your insight on who the family is.
- CPS should be able to describe who their families are, so as to provide the background needed for supervisors to give appropriate guidance and support for the work that is needed.
- CPS should be prepared with agenda items that they would like to have discussed during supervision.
- CPS should expect to receive critiques and feedback on tasks needed to improve and provide support for continued work with the families.

Supervisory Functions

Supervisors offer a broader, more experienced view of the case, including history and context, which is particularly important if there has been turnover in the case. Supervisors should be responsible in providing support and being knowledgeable in the decision making process especially in cases that have ambiguous situations.

- Supervisors must make timely assignment of cases to workers in CNNX and ACRS+. As part of their supervisory role they should always ensure that follow up and guidance are given with due dates. In addition, Supervisors must follow up and check in to make sure staff is in compliance with the due dates.
- Assign staff to cover workloads of other caseworkers who are out of work for any length of time.
- Arrange for new caseworker staff to be trained to meet the program requirements for supporting families in keeping their children safe and receiving the right services.
- Supervisors ensure CPS completes timely visits and document face-to-face contacts appropriately.

- Provide quality oversight and guidance for handling each case, especially for key case milestones. For example, at the 45- day conference, a full assessment, including the family genogram and crisis plan must be completed for a thorough discussion and inclusion in the Family Assessment Service Plan (FASP).
- Assign responsibility for and oversee updating of WMS/CCRS, especially court updates and timely case assignments to or un-assignments from provider agencies, when appropriate.
- Supervisory consultation and ongoing case practice direction is expected on all cases.
- Supervisors are expected to ensure that face to face contacts, home visits, and service provider contacts are made within required time frames.
- Bi-weekly supervisory caseload conferences are required to ensure quality service provision to the families and children.
- Bi-weekly, as well as ongoing supervision between supervisors and CPM is a necessary function to obtain feedback and support on challenging issues as well as concerns to enhance the practice on cases in the unit. For example, a case situation requires intensive case planning and oversight, domestic violence, substance abuse, cultural background or diversity issues that could affect case planning, placement appears necessary and or court order is violated.
- Supervisors must meet bi-weekly for individual and group supervision with Child Protective Managers (CPM).
- Supervisors are expected to consult with CPM whenever a family requires intensive case planning, and oversight.

Casework Supervision

Initial Casework Supervision

Supervisors must provide case specific guidance on what information is needed during the CPS initial contact with the family. This guidance is based on the reported safety concerns and any prior history found during clearances.

Is there any critical action that the CPS needs to take to ensure safety given the information available at this time? What are the **“must”** interviews, and **“must”** information that the CPS needs to obtain during the initial contacts?

If CPS is in the field, the Supervisor will provide guidance over the phone regarding case initiation. Documentation of this discussion must be captured in Connections and should reflect the discussion was held with CPS via phone.

Required task:	Assess/describe/document:
Pre-Conference Summary	• What key safety and risk concerns were reported • Who is in the family • What critical information is known from source or relevant parties to the case • Who are the relevant collaterals that should be contacted?
Required task:	Assess/describe/document:
Prior History Review	• What red flags were identified • What are the repeated themes, (DV, substance use, etc.) • History of similar patterns of allegations • History of unfounded or indicated reports. • What information was retrieved from the DIR's, Criminal Checks, HSS Connect
Required task:	Assess/describe/document:
Patterns /Themes Identified	• An assessment of repeated domestic violence, substance use, mental illness, educational neglect, etc. • Multiple arrests related to domestic violence, substance use, etc.
Required task:	Assess/describe/document:
Identifying Presenting Problems	• What is the threat of harm to the child • What actions impact child safety in the home • What are the everyday struggles for the family • Which family members are contributing to the presenting problem
Required task:	Assess/describe/document:
Assess Child's Vulnerability	• Child's age

	• Child's developmental needs • Mental health, medical, mental health needs • Availability of reliable and useful resources for child
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CONNECTIONS Documentation Requirement:

- Guidance given to CPS prior to initiating the investigation (whether the directives were given in person or via phone)
- Key points and themes of the prior history and its relation to current allegations

Links to Review for supervision:

- [Critical Thinking and Decision Making: Supervising Safety and Risk in CPS Investigations](#)
- [Administrative Policies](#)
- [Case Transfer Policies](#)
- [Policy Dissemination](#)
- [Case Management Changes with CNNX Build 18](#)

48-Hour Review- Supervisors must provide guidance on what activities from the initial supervisory guidance were not completed as well as the additional actions the CPS needs to complete before the 5 day review. What key information and actions are needed to inform the safety decision? Supervisors must determine and document whether the initial information obtained and actions by the CPS support the safety of the child. Supervisors must provide a short summary of what has been obtained or confirmed at this point in the investigation. The Supervisor must synthesize the information gathered from the prior history and the CPS's initial contact and document such in Connections.

5th Day Review and 7-day Safety Assessment Approval - Supervisory guidance should focus on practice gaps, safety and immediate actions needed to ensure and support the safety of the child. Supervisors should identify the practice and documentation gaps and set timeframes for CPS to complete follow ups. Supervisory review must determine if the documented information supports the safety decision. If the documentation does not support the safety decision, what specific and timely actions does the CPS need to take? Supervisory guidance should identify next steps and actions needed to move the case to the 25th day. If the child is safe, what risk factors need to be addressed to reduce repeat maltreatment?

Required task:	Assess/describe/document:
1.Supervisory conference with CPS	• Describe and document the 48 hour face to face supervisory conference • Document key points from case history, current SCR, clearances and information obtained during initial interviews completed by CPS

Required task:	Assess/describe/document:
2. Case specific guidance	• Describe and document identified gaps in assessment that need to be addressed • Provide guidance on how to interview particular persons listed on the case and how to obtain the additional information needed
3. Support Safety Assessment Factors and Decision	• Describe and document supportive document for the safety factors selected and safety interventions implemented. • Identify the elements of safety

25th Day Review—Supervision should focus on ensuring that all actions needed to support the safety of the children are completed or in progress. There should be a discussion of the information gathered and the identified service needs of the family. Supervisors must discuss the RAP risk rating and how this informs the decision to implement risk reduction services for the child. If there are supervisory overrides (risk rating low and services still required or risk rating high and services not required), supervisors should discuss this with the CPS and document the reasons for the override. Supervisors should verify that the CPS case documentation is current and accurate.

Required task:	Assess/describe/document:
1. Supervisory conference with CPS to be conducted 25 days from the initial intake.	• Document and describe the face to face supervisory conference with CPS
Required task:	Assess/describe/document:
2. Provide case specific guidance	• Information from prior history and 7 day safety assessment • Interviews of all family members, collateral contacts and other sources • Safety Interventions Implemented and their outcomes • Clinical Consultation Outcomes • Investigative Consultation Outcomes • RAP Score (if completed) • SCI Instrument (if applicable) • Family strengths and capacity for change • Goals that may have been set and achieved • How events in the family have impacted the individuals, family and parental protective capacity of the caretaker(s). • Provide explanation if services were in place and service outcomes were inconsistent. • Give case specific guidance for the

	progress of the Investigation to either closure and/or referral for services that address the gaps identified for further assessment and investigation.
Required Task:	Assess/describe/document:
3. Synthesis of identified the issues and gaps in assessment and/or information that needed to be addressed, including resolving any inconsistencies in the accounts obtained.	• CPS incorporated most/all themes and patterns from the family's history (prior SCR, DIR, criminal background) into the ongoing assessment of safety and risk. • If there were follow-up actions needed to further assess or address issues from the family's history, the CPS took appropriate action. • CPS completed the initial interviews with all caretakers and children who were not interviewed during the first 7 days of the investigation and assessed/re-assessed each caretaker's interaction with the child (ren). • In IRT enhanced cases, to assess the safety of the child (ren), the CPS conducted a joint interview at the CAC or some other location with law enforcement • If any child was not developing at an age appropriate level including motor, verbal skills and cognitive development, the CPS took actions to address the issues. • Depending on case circumstances, the CPS contacted key collaterals (e.g. the source, service provider, neighbor, school, personnel, extended family) to further corroborate the accounts provided by family members, and/or to obtain clarifying information about the family functioning and parent protective capacity. • The CPS utilized external or in-house resources to assist in the assessment of caretaker(s), child(ren), or functioning of other family members (e.g. IC, DV, MH, CASACs) • The CPS sufficiently addressed inconsistencies in accounts provided by

	the family members, source, collaterals, and /or among family members prior to investigation stage closing • All safe sleep issues have been adequately addressed at this point. • The CPS assessed whether the mental and healthcare needs and routine and special developmental/ health needs of each child are being met. (Consult with Medical Consultant) • The CPS took action to address the child (ren)'s educational needs in cases where the child (ren) is excessively absent from school and/or academically delayed. • The CPS was sensitive to the family's self-identified culture. • The CPS asked the appropriate service providers about the family or family member's attendance. • If the individual/family is not attending, or attending sporadically, the CPS took appropriate follow-up action. • If the individual/family member is attending, the CPS discussed the progress in changing the behaviors and conditions that require change. • A Family Meeting was held when needed to address (increasing) risk and the behaviors that increase risk and need to change are clearly identified. • The Family Meeting included family members non-family members involved with the family and youth and the support to be provided to mitigate risk involved with the family and youth. • The family was involved in development of the service plan. • There is agreement between the CPS team and the family members about the behaviors and conditions that must change to keep the child (ren) safe/and or reduce future risk. • Sufficient information was gathered to identify the appropriate risk elements and to assess risk to all the children in the household and is documented in the
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	RAP. Documentation supports the risk rating. The conditions that increase risk and need to change are clearly identified in the Risk Assessment Protocol and services identified to mitigate risk are appropriate to address the specific behaviors and conditions that put children at risk with appropriate referral made.
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45th to 55th day Review- Supervision should focus on concluding the investigation. Determine if the evidence supports the determination to indicate or unfound (link/nexus between the actions by the parents and the child being harmed or placed at risk of harm). Determine if there are changes to safety and risk to the child and are there any additional actions needed to be addressed by the CPS. If all risk concerns are addressed, give specific steps and timeframes for case closing/transfer.

Required task:	Assess/describe/document:
1.Review of case documentation for closure or transfer of services	• Ensure that all directives are addressed and any outstanding safety issues were completed. • Synthesis of all information throughout the life of the case
Required task:	Assess/describe/document:
2. Provide case specific guidance on case closure or case transfer	• Provide explanation if services were in place and service outcomes were inconsistent. • Give case specific guidance for the progress of the Investigation to either closure and/or referral for services that address the gaps identified for further assessment and investigation • Provide supportive documentation for the safety factors selected and safety interventions implemented.

Utilizing Appropriate Office Resources

Supervisors should implement processes to inform and remind staff of the need to effectively use the identified resources within the Borough Offices.

The Clinical Consultants, CASAC's, Medical and Investigative Consultants should provide monthly data to Borough Leadership capturing service utilization by Zone, and CPM area. This data will inform patterns of usage, and show areas or units where training will improve utilization.

Assessment of Safety Concerns

Supervisors should develop staff's capacity to use the information obtained to assess the Threat of Harm to the child, the Child's Vulnerability (how vulnerable is the child, eg. age, Special Needs) and the Protective Capacity of the parents and other family members.

- Staff should also develop capacity in identifying safety interventions that are Immediate, Specific (targeted to the safety concerns) and Concrete (measurable) to reduce or remove the safety concerns.

Contemporaneous Documentation throughout the Life of the Case

Links to review:

- [Documentation Guidelines](#)

Supervisors should implement consistent use of the Open Case Inquiry (OCI) print out generated by CPS to support and guide timely completion of CPS activities including required contacts and case documentation. In addition, handwritten case activities should be recorded in the DCP notebooks. During supervision meetings, Supervisors should review the contemporaneous notes in the CPS issued notebook. These notes should also be captured into Connections (CNNX) in a timely manner.

Managerial oversight provides the CPS staff with support and assists with identifying gaps to support the safety assessment obtained. The managers' oversight should help staff to identify additional resources to assist with the assessment of the recognized needs of the family. Managers should reinforce what key information is needed in cases in addition to the critical contacts and documentation that is needed prior to the approval of the safety assessment or case closure.

CPM's must have discussions around necessary next steps or actions needed, especially when children are not assessed in a timely manner. Managerial oversight should be occurring at all stages of the case review process, and must offer guidance to address practice gaps, use of resources, and on our goals and outcomes. Managers are responsible for completing the tasks listed in the prior section entitled *Case Supervision Section* as well as the tasks below.

Managerial Oversight

Managers are directly responsible for the review of all High Priority 1 and 13 coded cases. CPM's are also responsible for completing Managerial Random Reviews of 12 cases per month. Nine cases are selected by the Automated Case Recording System (ACRS). The cases selected for review are high priority 2-12 or non- high priority cases. CPM's can also select three (3) cases for review during the week that cases are not selected by ACRS.

Required Task:	Assess/describe/document:
1. Review of Cases coded High Priority 1 and 13 Reviews should be completed at: • Day 5 • Day 30 • Day 55	• Information from prior history and 7 day safety assessment • Interviews of all family members, collateral contacts and other sources • Safety Interventions Implemented and their outcomes • Clinical Consultation Outcomes • Investigative Consultation Outcomes • RAP Score (if completed) • SCI Instrument (if applicable) • Family strengths and capacity for change • Goals that may have been set and achieved • How events in the family have impacted the individuals, family and parental protective capacity of the caretaker(s). • Provide explanation if services were in place and service outcomes were inconsistent. • Give case specific guidance for the progress of the Investigation to either closure and/or referral for services that address the gaps identified for further assessment and investigation.

Required task:	Assess/describe/document:
2.Complete Random Case Review	• Complete the random review instrument • Discuss with Supervisors the outcome of the review • Document the outcome of the meeting/discussion with the Supervisors and guidance to close any case specific practice gaps • All reviews are to be stored in the Random Review folder of the S drive
Required Task:	Assess/describe/document:
3.Use of data to track events, identify trends, areas of strength or challenges for improvement	• During supervision sessions with Supervisors, review data to address issues within the unit or managerial area • Review of the following reports and documentation as needed to support case reviews or supervision with Unit Supervisor From Connections (CNNX) ○ Overdue incomplete investigations - IC Cohort Table - Investigation Start Dates ○ Indication Rate report ○ 7 Day Safety Assessment Report From ACRS+ ○ Zone Level Pending Rates - ACRS ○ Caseload Dispersion – ACRS ○ Caseload Reports – Unit- ACRS ○ 24/48 Hour Face-to-Face Contact Summary and detail report- ACRS Other Reports ○ FASP Reports ○ FSS no WMS ○ Multiple Person Identification Report (MPR) ○ Management Report Logs

Links to review:

- [ECS No Contact Priority Log Tracking](#)
- [Guidelines for Managerial Decisions on Deferral of Immediate Face to Face Contacts](#)
- [Guidelines for Family Court Settlements](#)
- [MPI Management Performance Indicators Curriculum](#)
- [Managerial Random Review](#)

Quality Assurance

The items in the following section support and guide the quality practice in child welfare investigations.

Links to Review:

- [Quality Assurance Subfolder](#)
 - [Quality Assurance Zone Case Review](#)
 - [DCP Quality Practice Case Review Guidelines](#)
 - [Zone Metrics Training and Worksheets](#)

Links to review:

- [Child Safety Alerts](#)
- [Child Parent Templates \(Day Operations, ECS and OSI\)](#)
- [ACS Principles \(Placement, Permanency, Substance Abuse, Mental Health, Domestic Violence, etc.\)](#)

[i] See Social Services Law §§ 424 and 427-a; 18 NYCRR §§ 428.1, 428.6, 432.2, and 443.3; [ACS Foster Care Quality Assurance Standards, p. 43](#); and [ACS Preventive Services Quality Assurance Standards and Indicators, p, 19](#).

[ii] See [86-OCFS-ADM-12 Policy on Fire Safety and Fire Prevention in Foster Homes](#).

[iii] See [10-OCFS-ADM-02 Standards of Installation and Maintenance of Carbon Monoxide Detectors and Amanda's Law](#).

[iv] See [OCFS Pub. 5079 Safety Factors](#).